

GOVERNMENT OF ALBERTA



# Health

## Annual Report 2024-25

*Alberta* 

Health, Government of Alberta | Health Annual Report 2024–2025

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## Preface

The Public Accounts of Alberta are prepared in accordance with the *Financial Administration Act* and the *Sustainable Fiscal Planning and Reporting Act*. The Public Accounts consist of the annual report of the Government of Alberta and the annual reports of each ministry.

On October 16, 2024, the government announced new ministry structures. As such, the responsibilities for Continuing Care, previously held by the Ministry of Health were transferred to the Ministry of Seniors, Community and Social Services. The 2024-25 Annual Report reflects the 2024-27 Ministry business plan, the Government of Alberta Strategic Plan, as well as the ministry's activities and accomplishments during the 2024-25 fiscal year, which ended on March 31, 2025.

The Annual Report of the Government of Alberta contains *Budget 2024* Key Results, the audited Consolidated Financial Statements and Performance Results, which is a comparison of actual performance results to desired results set out in the government's strategic plan.

This annual report of the Ministry of Health contains the Minister's Accountability Statement, the ministry's Financial Information and Results Analysis, which is a comparison of actual performance results to desired results set out in the Ministry Business Plan. This ministry annual report also includes:

- the financial statements of entities making up the ministry including Alberta Health Services (AHS) and the Health Quality Council of Alberta (HQCA) for which the minister is responsible; and
- other financial information as required by the *Financial Administration Act* and *Sustainable Fiscal Planning and Reporting Act*, as separate reports, to the extent that the ministry has anything to report.

All Ministry annual reports should be considered along with the Government of Alberta Annual Report to provide a complete overview of government's commitment to openness, accountability, and fiscal transparency.

## Ministers' Accountability Statement

The ministry's annual report for the year ended March 31, 2025, was prepared under my direction in accordance with the Sustainable Fiscal Planning and Reporting Act and the government's accounting policies. All the government's policy decisions as at June 3, 2025, with material economic or fiscal implications of which I am aware have been considered in the preparation of this report.

[Originally signed by]  
Honourable Adriana LaGrange  
Minister of Primary and Preventative Health Services

[Originally signed by]  
Honourable Matt Jones  
Minister of Hospital and Surgical Health Services

## Message from the Ministers



Since our refocusing initiative was first introduced, we have made tremendous progress on implementing a reliable, accountable health care system that puts patients first while empowering health care workers.

We know how important it is to get this work right. That's why we've focused on engaging with the people who matter most: Albertans. During our first round of public engagement, we heard from almost 30,000 people through in-person and virtual town halls and using online tools. We launched a second round of engagement in January 2025 to share information and receive additional feedback, which has helped shape our path forward.



Our 2024-25 annual report reflects the progress that has been made to date, including the introduction of four provincial health agencies that are responsible for overseeing key parts of the health care system: Primary Care Alberta, Acute Care Alberta, Assisted Living Alberta and Recovery Alberta. Each of these provincial health agencies reports to a minister, improving accountability and access to services across the province.

In addition to our refocusing work, the 2024-25 annual report highlights significant work that was done over the past year to improve health outcomes for Albertans.

Specific to acute care, we added 83 emergency department physicians and 157 more emergency department nurses in 2024-25, and we completed a record number of 318,601 surgeries, surpassing the target of 310,000. We also opened the \$1.4 billion Arthur J.E. Child Comprehensive Cancer Centre, a state-of-the-art facility that offers 160 inpatient beds, chemotherapy chairs, exam rooms, radiation vaults, outpatient services and clinical trial space to support Albertans living with cancer.

To support improved access to primary care services, we introduced a new Primary Care Physician Compensation Model that was developed in collaboration with the Alberta Medical Association. This model encourages physicians to grow their patient panels while rewarding high-volume, high-quality care with incentives. We also introduced a Nurse Practitioner Primary Care Program to support nurse practitioners so they can practise comprehensive patient care autonomously and operate their own practices, or practise autonomously in existing primary care settings, and we launched the Rural and Remote Family Medicine Resident Physician Bursary Program to support family medicine residents who commit to practising in rural or remote areas.

In total, *Budget 2024* provided \$26.2 billion in operating expenses to prioritize the delivery of high-quality, reliable health services across the province. *Budget 2024* also provided \$4.5 billion over three years to support capital projects like the redevelopment of the Red Deer Regional Hospital Centre, plans for a stand-alone Stollery Children's Hospital in Edmonton, and the replacement of the Beaverlodge Health Centre.

The 2024-25 annual report sets the foundation for what lies ahead: a better, refocused health care system that will ensure every Albertan, regardless of which part of the province they call home, can access the services they need close to come.

[Originally signed by]  
Honourable Adriana LaGrange  
Minister of Primary and Preventative Health Services

[Originally signed by]  
Honourable Matt Jones  
Minister of Hospital and Surgical Health Services

## Management's Responsibility for Reporting

The Ministry of Health includes Alberta Health, AHS, Primary Care Alberta, and the Health Quality Council of Alberta.

The executives of the individual entities within the ministry have the primary responsibility and accountability for the respective entities. Collectively, the executives ensure the ministry complies with all relevant legislation, regulations and policies.

Ministry business plans, annual reports, performance results and the supporting management information are integral to the government's fiscal and strategic plan, annual report, quarterly reports and other financial and performance reporting.

Responsibility for the integrity and objectivity of the accompanying ministry financial information and performance results for the ministry rests with the Minister of Health. Under the direction of the minister, I oversee the preparation of the ministry's annual report, which includes the financial information, performance results on all objectives and initiatives identified in the Ministry Business Plan, and performance results for all ministry-supported commitments that were included in the 2024-27 Government of Alberta Strategic Plan. The financial information and performance results, out of necessity, include amounts that are based on estimates and judgments. The financial information is prepared using the government's stated accounting policies, which are based on Canadian public sector accounting standards. The performance measures are prepared in accordance with the following criteria:

- Reliable – information used in applying performance measure methodologies agrees with the underlying source data for the current and prior years' results.
- Understandable – the performance measure methodologies and results are presented clearly.
- Comparable – the methodologies for performance measure preparation are applied consistently for the current and prior years' results.
- Complete – outcomes, performance measures and related targets match those included in the ministry's Budget 2024.

As deputy minister, in addition to program responsibilities, I am responsible for the ministry's financial administration and reporting functions. The ministry maintains systems of financial management and internal control which considers costs, benefits and risks that are designed to:

- provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded to maintain accountability of public money;
- provide information to manage and report on performance;
- safeguard the assets and properties of the province under ministry administration;
- provide Executive Council, the President of Treasury Board and Minister of Finance, and the Minister of Health the information needed to fulfil their responsibilities; and
- facilitate preparation of ministry business plans and annual reports required under the Sustainable Fiscal Planning and Reporting Act.

In fulfilling my responsibilities, I have relied, as necessary, on the executives of the individual entities within the ministry.

[Originally signed by]  
Rae-Ann Lajeunesse  
Deputy Minister of Primary and Preventative Health Services  
June 3, 2025

[Originally signed by]  
Bryce Stewart  
Deputy Minister of Hospital and Surgical Health Services  
June 3, 2025

# Results Analysis

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## Ministry Overview

### Organizational Structure

The Government of Alberta is refocusing Alberta's health care system to improve health outcomes for Albertans and empower health care workers to deliver quality care across the province. A refocused health care system will provide Albertans with the necessary care when and where they need it, enhance government's ability to provide system-wide oversight, set system priorities, and ensure accountability for those priorities on behalf of Albertans. As part of this refocused approach, the ministry continues to support Albertans' health and well-being throughout their lives by protecting public health and promoting wellness; coordinating and delivering safe, person-centered, quality health care services; planning capital infrastructure; supporting innovative information management and technologies; regulating health care; and funding the health system.

In fall 2024, amendments to the *Provincial Health Agencies Act* provided the Ministry of Health with a clear mandate to transform Alberta's health care delivery and governance. This new, outcome-focused approach replaces the single regional health authority model with a more integrated system comprised of four provincial health agencies (PHAs), each focused on a key sector of care: Primary Care Alberta (PCA), operational since February 1, 2025; Acute Care Alberta (ACA), operational as of April 1, 2025; Assisted Living Alberta (ALA), established as a legal entity on April 1, 2025; and Recovery Alberta (RA), which began operations on September 1, 2024. These agencies will work in coordination under the ministry's oversight to improve health outcomes and ensure greater accountability across the system.

The Ministry of Health currently consists of Alberta Health, ACA, PCA, and the HQCA. The Ministry of Mental Health and Addiction oversees RA, and the Ministry of Seniors, Community and Social Services oversees ALA. The alignment of PHAs within the health system is depicted in the chart below. AHS will continue to have a role in the refocused health care system by concentrating on the delivery of acute care services. To promote continued alignment within the system, the province established an Integration Council, which all four PHAs will be a part of. The goal of the Integration Council is to ensure all ministries, PHAs, and service delivery providers work together to deliver better health outcomes for Albertans.

The Ministry of Health relies on collaboration with a range of health professionals; partners and organizations in the health and social sectors; academic and research institutions; First Nations, Metis Settlements, and the Métis Nation of Alberta; and other orders of government to deliver health services. The Minister of Health is supported by parliamentary secretaries for rural health and health workforce engagement.



Note: Dotted line represents the integration and alignment of the four provincial health agencies within the Health System.

Alberta Health establishes the Government of Alberta's strategic direction for health. The department advises the government on health policy, legislation and standards, and public health concerns; monitors and reports on health system performance; sets policies and priorities for the electronic/digital health environment; provides oversight; and ensures accountability across the health system.

ACA, the PHA for acute care, will oversee the governance and coordination of acute care services across Alberta by working directly with acute care service providers to reduce wait times for emergency departments (EDs) and surgeries, lower emergency medical services (EMS) response times, and enhance the overall care experience. The goal is to deliver timelier, more coordinated acute care services across the province. Alberta acute care providers include AHS, Covenant Health, the Lamont Health Care Centre and chartered surgical facilities.

PCA, the PHA responsible for the primary care health services sector, coordinates primary health care services and contributes to the transparent oversight of the health system at the provincial level. PCA will focus on ensuring every Albertan is attached to a family physician or a nurse practitioner (NP), providing timely access to high-quality primary care services, and supporting an integrated team of health professionals to provide comprehensive primary care with appropriate access to patient health information.

The HQCA works collaboratively with health system partners on a variety of topics and initiatives to improve patient safety, person-centered care, and overall health service quality across the province. The HQCA's role will expand to support Alberta Health and the PHAs to set performance standards and performance indicators and to support audit and compliance functions.

The Office of the Alberta Health Advocates reports to the Minister of Health to improve patient care and the health system. The office supports Albertans in navigating the health system and accessing the appropriate resolution services to address health care concerns, including those specific to the *Alberta Health Act*, seniors, their families, and service providers.

## Operational Overview

### Department of Health

With direction from the minister, the deputy minister is responsible for the daily operations of the Department of Health, which is structured as follows:

**Deputy Minister's Office** – provides leadership to the health system to ensure the delivery of quality health services, drives innovation, and builds and maintains collaborative relationships across government ministries, AHS, the HQCA, and the PHAs. The office provides policy coordination and issues management for the minister and leadership in priority setting, decision-making, and operations, as well as ministry correspondence services.

**Acute Care Division** – leads the province's renewed focus on acute care services, including developing standards, monitoring and evaluating the acute care and Emergency Health Services (EHS) performance, and working with partner divisions to oversee the delivery of hospital care, urgent care, cancer care, provincial program organ and tissue donation, and transplantation services clinical operations, surgeries, and EHS. The division also supports strategic policy for acute care, working with partners to develop solutions for the acute care and EHS sectors, oversees the governance and legislative compliance of EHS delivery and is currently supporting the implementation of the new Alberta EMS Standing Committee (AESC).

**Finance and Capital Planning Division** – develops and manages the ministry's budget and oversees financial activities within the Department of Health. The division provides financial advice, prepares annual financial statements, and leads corporate planning and reporting. The Finance and Capital Planning Division (FCP) is responsible for AHS accountability, capital planning and coordination of infrastructure projects with Alberta Infrastructure and AHS. The division also leads integrated health system planning by developing system planning policies to support a more coordinated and outcome-focused provincial health care system. Additionally, FCP manages the department's grant approval process, provides corporate administrative and contracting services, and oversees registration, designation and acute care bed survey processes and reporting.

**Health Information Systems Division** – provides leadership and strategic direction for the development and implementation of policy and standards for the provincial eHealth environment, and the Department of Health. The division is responsible for Alberta Netcare (electronic health record), Community Information Integration and Central Patient Attachment Registry, Connect Care (clinical information system), and MyHealth Records (personal health record), including health information policy and advice, stakeholder engagement, strategic planning and project and system delivery and operations.

**Health Workforce Division** – develops and implements health workforce and system policies related to compensation for insured health care providers and the self-regulation of health professions. Working in collaboration with stakeholders, the division oversees the implementation of key health workforce planning and priorities, such as the health workforce strategy, and provides health workforce planning support, enabling a fiscally sustainable and effective health workforce to support Albertans' needs. Together, these initiatives support the government's commitment to build a health care system that is sustainable and serves the needs of Albertans.

**Indigenous Health Division** – leads government efforts to improve the health and well-being of First Nations, Métis, and Inuit peoples in Alberta through culturally safe health care service delivery. The division facilitates this by including Indigenous partners' perspectives and reflecting patients' unique health needs across all health program and policy planning. The division leads key mandate priorities, such as strengthening Alberta Health's capacity to develop effective health policy and conduct strategic planning, building respectful and trusting relationships, advancing the

implementation of the Indigenous Modernizing Alberta's Primary Health Care System (MAPS) initiative, and working collaboratively with First Nations, Métis, and Inuit partners, and health system providers to build a culturally safe health care system.

***Pharmaceutical and Supplementary Benefits Division*** – oversees and provides governance to the Alberta Health Care Insurance Plan (AHCIP) and all government-sponsored supplementary health benefits programs (i.e., Coverage for Seniors, Non-Group Coverage, Assured Income for the Severely Handicapped, Income Support, Alberta Adult/Child Health Benefit programs, Ukrainian Evacuee Health Benefit, Alberta Aids to Daily Living, and Dental and Optical Assistance for Seniors) that provide Albertans with pharmaceutical, optical, dental, and other medical supports (wheelchairs, prosthetics, oxygen, medical/surgical supplies, etc.). The division is also responsible for Alberta's participation in the National Blood Program and advising the minister in their role as a corporate member of Canadian Blood Services.

***Primary Care and Rural Health Division*** – responsible for developing policy and performance oversight of primary care, including new primary care agency (Primary Care Alberta) and models (e.g., Regional Primary Health Care Networks) and developing related accountability mechanisms, such as standards, guidelines, and legislative tools. The division is also responsible for developing and implementing key policy initiatives aligned with the government's renewed focus on primary care, such as the implementation of the MAPS initiative, supporting the administration and governance of Primary Care Networks (PCNs), and strengthening relationships with key stakeholders. The division will oversee primary and community health; chronic disease management; and the administration and governance of PCNs to meet the needs of Albertans. The division collaborates with stakeholders, such as physicians, NPs, registered nurses, licensed practical nurses, health professions' regulatory colleges, and health system leaders from academia and the community to support the provision of quality primary health care services to Albertans. The mandate of the division is achieved through evidence-informed and value-oriented initiatives, promotion of the patient medical home initiative through PCNs and their governance structure, performance monitoring and evaluation, and collaboration with health professions' regulatory colleges to support patient safety. The division is also responsible for the development of rural health-related policy and is responsible for ensuring that rural and remote perspectives are considered and incorporated in Alberta Health policies.

***Public Health Division*** – provides strategic direction and leadership on emerging public health risks, communicable diseases, immunization, compliance monitoring, environmental public health, newborn screening, health promotion, and emergency preparedness, response and recovery, through the assessment, development and implementation of provincial policies, regulations, strategies, and standards. The division carries out these functions to support innovation and engagement with Albertans towards wellness, health promotion, and injury and disease prevention. To support health system quality, the division collaborates with partners to perform compliance and monitoring activities, and enforcement of the acts, regulations and standards administered by the division in the areas of infection prevention and control oversight, and protection for persons receiving certain care or support services.

***Strategic Policy and Performance Division*** – responsible for providing leadership for health care research and innovation, including monitoring, assessing, and improving health system performance. The division is the primary source for Alberta's overall health system data standards and analytics, evaluating the health system's performance to support evidence-based policy decisions for all health sectors. The division is also responsible for the *Freedom of Information and Protection of Privacy Act* (FOIP), legislative planning, strategic policy coordination, governance of over 45 health public agencies and over 70 health foundations (including 450 member opportunities), and all Cabinet-related materials, as well as intergovernmental relations. The

division works with other divisions to develop performance and reporting standards for the health care system.

***System Refocusing Division*** – responsible for implementing the new direction for Alberta's health care system to improve health outcomes for Albertans and empower health care workers to deliver quality care across the province. This includes supporting the transition to a unified provincial health care system by helping establish governance structures and operational processes for the newly created sector-based PHAs for acute care, continuing care, primary care, and mental health and addiction. The division also supports ministry divisions, AHS, and system partners in aligning with the new direction, and engages with health care workers and members of the public to understand the current system needs and support for a smooth transition. In addition, the division provides secretariat support to the Integration Council, which is tasked with guiding and monitoring the implementation of the integrated health system and ensuring coordination across all PHAs.

***Office of the Chief Medical Officer of Health*** – monitors and reports on the health of Albertans and advises on actions to protect and promote the health of the public under authority of the *Public Health Act*. This includes legislated responsibilities related to disease surveillance, communicable disease outbreaks, infection prevention and control measures, health risk assessments, and states of public health emergencies. The Office of the Chief Medical Officer of Health provides strategic leadership, oversight, support and clinical expertise on issues of public health importance to Albertans. The office works closely with diverse partners within and beyond the health system, including the Department of Health's Public Health Division, Primary Care and Rural Health Division, Medical Officers of Health at AHS, and Indigenous Services Canada to facilitate policies, processes and programs to prevent chronic diseases, control the spread of communicable diseases, support health surveillance, and strengthen the public health system in Alberta.

***Communications and Public Engagement [CPE]-Health*** – provides Albertans and health system partners with information about ministry policies, programs, and initiatives. The CPE-Health team works with department staff to develop and implement communication plans and offers communications support, such as media relations, issues management, writing and editing services, product development, and online communications services. CPE-Health also works closely with AHS and other reporting entities to coordinate ministry communications.

***Health Law*** – through Alberta Justice, a team of lawyers support all aspects of the Department of Health's activities, ranging from grant agreements and contracting and procurement to developing and interpreting legislation and general legal advice to the ministry.

***Human Resources*** – through the Public Service Commission, Human Resources is dedicated to delivering programs, supporting initiatives, and providing human resource expertise and services that attract, retain, and engage the department's workforce. The branch works in partnership with managers and employees to build and sustain workforce capacity to achieve business goals and create an environment where employees are respected, valued, engaged and resilient.

***Procurement and System Optimization Secretariat*** – ensures procurement frameworks are developed and maintained to support health care contract management related to services and commodities, including governance, risk management, performance management, compliance, and public accountability. The secretariat supports system-wide optimization (i.e., cross-sector optimization projects, etc.) and leads development of standing offers for system-wide ancillary services to support system-wide optimization, where appropriate. The secretariat establishes spending and urgency thresholds to support local procurement and decision-making. The overall goal is to provide a system-wide procurement oversight function to ensure efficient, open, and transparent procurement processes are maintained for safeguarding public funds, supporting

Alberta's health service delivery partners, while adapting to local needs where required and where it makes sense

## Key Highlights

The Ministry of Health focused on accomplishing [number of outcomes] identified in the 2023-26 Ministry Business Plan:

- Enhanced timely access to an effective, safe and responsive publicly funded health care system that provides Albertans with the necessary care when and where they need it.
- A modernized and refocused health care system that is fiscally sustainable and delivers the highest-quality patient care to all Albertans, including indigenous peoples, regardless of where they reside in the province.
- The health and well-being of all Albertans is protected, supported and improved, and health inequities among underserved populations, including indigenous peoples, are reduced.

Key highlights and results achieved by the Ministry of Health in 2024-25 include:

|                                                                         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|-------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Reducing Surgical Wait Times through Alberta Surgical Initiative</b> | <p>Alberta Health continued to reduce surgical wait times in 2024-25 through initiatives under the Alberta Surgical Initiative (ASI), including centralized referrals and expanded use of CSFs. By March 31, 2025, 318,601 surgeries were completed, exceeding the target of 310,000 and up 4.6 per cent from the previous year. About 74,928 patients remained on the wait list, with 57.8 per cent within clinically recommended timelines.</p> <p>CSFs performed 64,500 surgeries (20.2 per cent of all scheduled procedures). Rural surgical volumes rose 4.4 per cent, and cancer surgeries increased 9.4 per cent, with 55 per cent completed within benchmark timelines.</p> |
| <b>EHS Improvements</b>                                                 | <p>In 2024-25, Alberta invested \$704 million in Emergency Health Service (EHS). EMS Return to Service initiatives reduced average hospital handover times. The replacement of end-of-life EMS vehicles was funded through the EMS Vehicle Capital Grant Program, with a total of \$15 million in 2024-25 toward a \$35-million, three-year investment in this program. This funding supports the purchase of new EHS vehicles and provides additional equipment to meet the needs of Albertans in both urban and rural areas of the province.</p>                                                                                                                                  |
| <b>Access to Cancer Care and Diagnostics</b>                            | <p>The Arthur J.E. Child Comprehensive Cancer Centre opened in Calgary on October 28, 2024. The \$1.4-billion facility includes 160 inpatient beds and 90 chemotherapy spaces. It will add a new clinical trial space in late 2025 and expand based on patient and clinical needs.</p>                                                                                                                                                                                                                                                                                                                                                                                              |
| <b>Investment in Rural Health Workforce</b>                             | <p>To address rural health care needs, Alberta invested over \$7.06 million in 2024-25 through the Rural Health Professions Action Plan (RhPAP). An additional \$2 million supported Indigenous second birth attendants. As of March 31, 2025, 53 NPs began providing care across zones, including four NPs on First Nation communities or Metis Settlements. Other investments in rural health workforce included: continued support for rural physician education programs; financial incentives for rural physicians, such as the Rural Remote Northern Program; AHS' targeted recruitment of</p>                                                                                |

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|---------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                             | <p>internationally educated nurses; the launch of the Rural and Remote Family Medicine Resident Physician Bursary Pilot Program; and continued implementation of the Alberta Medical Association (AMA) Agreement. The Indigenous Wellness Program Alternative Relationship Plan was also established in 2014 to overcome geographical, economic, political, cultural, and socio-historical barriers faced by First Nations, Métis, and Inuit patients, and continues to expand to support delivery to an increasing number of First Nations, Métis and Inuit communities. In 2024-25, 43 family physicians worked under this plan across Alberta.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
| <b>Strengthening Primary Health Care</b>    | <p>In 2024-25, the Government of Alberta continued to implement the Modernizing Alberta's Primary Health Care System (MAPS) initiative that was launched in September 2022 to address urgent system pressures and improve access to care. Following the release of MAPS final reports in October 2023, the MAPS Two-Year Implementation Plan was released in April 2024, which outlined a series of actions to strengthen and stabilize primary health care in Alberta. In June 2024, an interactive dashboard publicly reported the progress of the actions.</p> <p><i>Budget 2024</i> invested \$475 million to support the continued implementation of the MAPS initiative, including: \$100 million to improve access to family physicians; \$10 million for primary health care initiatives in Indigenous communities; \$17 million to further develop a compensation model for nurse practitioners; \$24.3 million to support panel management and practice improvement activities; and \$30 million to address the needs of underserved populations. The funding is dedicated to implementing the MAPS recommendations aimed at strengthening primary health care across the province.</p> |
| <b>Health System Refocusing Initiatives</b> | <p>In 2024-25, the Government of Alberta advanced transformation of the health system by establishing three sector-based PHAs for acute care, mental health and addiction, and primary care – while work was underway to establish established four Provincial Health Agencies—ACA, ALA, PCA, and RA—to deliver more specialized and integrated care across Alberta, with phased launches between 2024 and 2025. These agencies form the core of a unified provincial health care framework designed to improve accountability, integration, and patient-centred care.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Digital Health Modernization</b>         | <p>In 2024-25, the Government of Alberta continued to enhance digital access to health information as part of its commitment to a person-centred health care model. Progress included expanding the information available through the MyHealth Records (MHR) portal and integrating community primary care data into Alberta's Electronic Health Record, Alberta Netcare, Community Information Integration and Central Patient Attachment Registry, and Connect Care to support informed clinic decision-making. By March 31, 2025, there were over 1.9 million MHRs users, with an average of 32,000 daily log ins. In addition, over 2,100 physician offices and 1,700 community pharmacies were using Alberta Netcare. Connect Care</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                       |

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|-----------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                           | supports approximately 125,000 staff, physicians, and clinicians across AHS, its affiliates, and operating provincial health agencies across Alberta.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
| <b>Diagnostic Imaging Enhancements</b>                    | Alberta allocated \$15 million in 2024-25 for additional CT and MRI scans, targeting 41,000 more CT and 12,000 more MRI scans. While the proportion of patients seen within provincial wait time targets remained stable for CT scans and declined marginally for MRIs, median wait times improved, including for community lab services.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <b>Advancing Public and Population Health</b>             | In 2024-25, Alberta Health invested \$148 million to safeguard public and population health through initiatives focused on disease surveillance, immunization, chronic disease prevention, environmental health, food safety, and injury prevention. The fall outreach immunization campaign was launched on October 1, 2024, and by March 31, 2025, 21 per cent of Albertans had received an influenza flu vaccine and 13.6 per cent a COVID-19 vaccine. Ongoing efforts targeted chronic conditions like diabetes and cancer, as well as communicable diseases, including measles, shigellosis, Mpox, sexually transmitted and blood-borne infections, and respiratory viruses, such as influenza and respiratory syncytial virus (RSV). In December 2024, Alberta joined the National Strategy for Drugs for Rare Diseases, securing \$54 million per year over three years to expand access to cutting-edge therapies and improve rare disease diagnostics.                                                                                                                                                                                                                                      |
| <b>Improving Health Equity for Indigenous Communities</b> | In 2024-25, Alberta Health advanced its commitment to health equity by expanding access to culturally safe, community-based care for First Nations, Métis and Inuit peoples guided by the recommendations of the MAPS initiative. The Indigenous Primary Health Care Innovation Fund provided \$20 million in funding over two years to support Indigenous-led programs. Eighteen grant recipients will receive \$7.6 million in funding over three years through the Indigenous Patient Navigator Grant Program to help improve access to primary care services closer to home. The Government of Canada contributed an additional \$6.3 million over three years to launch a second round of navigator funding, leading to the creation of the Bilateral Indigenous Patient Navigator Grant Program. <i>Budget 2024</i> committed approximately \$2 million over three years, of which \$333,000 was provided to two Indigenous post-secondary institutions to develop and deliver First Nations-infused health care aide training. <i>Budget 2024</i> also invested \$1.4 million to an Indigenous organization - Kihew Awasis Wakimik Cultural Society, to offer second birth attendant program. |

The Ministry of Health remains committed to regulatory approaches and program delivery that reduces unnecessary government oversight and emphasizes outcomes, to improve access to government services, attract investment, support innovation and competitiveness, and grow Alberta businesses.

## Discussion and Analysis of Results

### Actions that support the priorities of the Government of Alberta Strategic Plan

#### Key Priority 2: Investing in Albertans

Ensuring an accessible, responsive, and refocused health care system

Refocusing the health care system to improve access and patient outcomes while empowering Alberta's front-line health care workers. This refocusing work will create a unified health system with provincial sector-based health agencies that centre around four dedicated organizations: primary care, acute care, continuing care, and mental health and addiction. Alberta Health Services will continue to deliver acute care services, and an Integration Council will ensure the system is in alignment to better deliver health outcomes.

Detailed reporting found on pages 32, 33, 35, 37 (Key Objectives 1.3, 1.4, 2.1, & 2.2)

Improving access for underserved populations including First Nations, Métis, and Inuit peoples to quality and culturally appropriate health services that support improved health outcomes by promoting and exploring novel, innovative, and creative strategies.

Detailed reporting found on pages 40, 46, 50, 51 (Key Objectives 2.4, 3.1, 3.3, & 3.4)

Improving access to family doctors and health professionals. Government is working with the Alberta Medical Association to implement new payment models for family physicians while finding ways to reduce their administrative workload and allocating \$57 million over three years to help doctors and nurse practitioners working in primary care manage costs of increased patient caseload. An additional \$200 million over two years will improve access to family physicians and help ensure primary health care is available for every Albertan when and where they need it. Government is also emphasizing alternative models of care, such as access to team-based care and integration between primary health care and community care.

Detailed reporting found on pages 29, 33 (Key Objectives 1.2 & 1.4)

Improving Emergency Medical Services system response times, reducing emergency department and surgical wait times, and resolving laboratory and diagnostic services delays to ensure patients are receiving the appropriate level of care as well as empowering front-line workers to better deliver health care services.

Detailed reporting found on page 20 (Key Objective 1.1)

Strengthening and modernizing continuing care, primary health care, and digital health systems to increase choice, equitable and timely access, improving the quality of care for Albertans.

Detailed reporting found on pages 20, 32, 39, 40, 42 (Key Objectives 1.1, 1.3, 2.3, 2.4, & 2.5)

Addressing health care staffing challenges by investing \$164 million in 2024-25 towards initiatives to train, recruit, and retain more health care professionals for Alberta. This work is supported by Alberta's Health Workforce Strategy and the Rural Health Professions Action Plan, addressing specific challenges in remote and rural communities, as well as creating additional seats for physician education at Alberta's medical schools.

Detailed reporting found on page 29 (Key Objective 1.2)

Addressing the province's critical nursing shortage by making it easier for credentialed nurses from outside of Alberta/Canada to bring their skills to Alberta patients. Government is attracting internationally educated nurses by streamlining immigration processes for health care professionals and by addressing barriers they face when coming to Canada, including navigating regulatory requirements, assessment, and licensing processes, and having access to bridging education and clinical placements if needed.

Detailed reporting found on pages 29, 33 (Key Objectives 1.2 & 1.4)

Increasing access to maternal care by investing over \$10 million over the next three years to develop and implement Alberta's Midwifery Strategy, reducing pressure on hospitals and improving access to maternity care in rural and remote areas.

Detailed reporting found on pages 46, 51 (Key Objectives 3.1 & 3.4)

Investing \$4.5 billion over three years in capital funding to maintain or expand health care facilities across the province. This includes funding for the redevelopment and expansion of the Red Deer Regional Hospital Centre, which will increase critical services and add capacity to one of the largest hospitals in central Alberta.

Detailed reporting found on pages 46, 51 (Key Objectives 3.1 & 3.4)

Forming lasting and meaningful partnerships with Indigenous communities:

Working with the federal government and First Nations, Métis and Inuit communities to improve access for Indigenous Peoples no matter where they reside in the province. Working with the federal government, and First Nations, Métis and Inuit communities to improve access to key services such as education and healthcare for Indigenous peoples, no matter where they reside in the province.

Detailed reporting found on pages 39, 40, 50 (Key Objectives 2.3, 2.4, & 3.3)

**Outcome One: Enhanced timely access to an effective, safe and responsive publicly funded health care system that provides Albertans with the necessary care when and where they need it**

Alberta's government continues to make health care more accessible and responsive by building health system capacity to support Albertans' health and well-being throughout their lives, ensuring an integrated and high-quality health care delivery that provides the highest levels of service to Albertans and timely access to the most appropriate care in the most appropriate setting.

**Key Objectives****1.1 Increased access to health care services for Albertans by improving the Emergency Medical Services system, reducing surgical wait times, decreasing emergency department wait times, reducing laboratory and diagnostic services delays, and improve primary health care across the province**

A key priority for the Alberta government in 2024-25 was to continue strengthening the health care system, making it more effective and accessible. Alberta Health remained focused on prioritizing health system capacity to ensure a high-quality health system that meets patients' needs and provides access to the most appropriate care in the most suitable setting. This included building surgical, and intensive care unit (ICU) capacities, as well as strengthening the health workforce. Several initiatives were implemented to expand the capacity of Alberta's publicly funded health care system and minimize disruptions to patient care.

The Government of Alberta remained committed to increasing surgical capacity to keep pace with demand and reduce the length of time Albertans waited for scheduled surgeries. Efforts continued to improve the patient journey by streamlining care coordination and surgical pathways, supporting family physicians with earlier specialist advice before consultations, and centralizing referrals to match patients with the right surgeon faster. Alberta Health addressed urgent health care needs while working towards a more resilient and efficient health care system for Albertans with four key goals:

- Decreasing emergency department (ED) wait times
- Improving emergency medical services (EMS) response times
- Reducing wait times for surgeries
- Empowering front-line workers to deliver health care

The ministry implemented the Alberta Surgical Initiative (ASI), a surgical access improvement plan with a patient-first focus to reduce surgery wait lists, wait times, and strengthen surgical services in Alberta. This ensures that Albertans have surgeries within clinically recommended wait times. As part of the ASI, Alberta Health worked with Alberta Health Services (AHS) to improve and standardize the entire surgical journey by:

- Prioritizing surgeries and allocating operating room time according to the greatest need
- Streamlining referrals from primary care to specialists
- Increasing surgeries at underutilized operating rooms (mainly in rural areas)
- Providing lower-risk, routine surgeries through accredited chartered surgical facilities (CSFs) to help provide publicly funded insured services and extend existing capacity in hospitals

As part of ASI, the second operating room at the Rocky Mountain House Health Centre became operational in April 2024. This project supports the government's commitment to improve access to timely surgical care for Albertans, particularly in rural communities.

The expansion project added 4,736 square feet of new space, including:

- A new operating room
- Six additional recovery beds
- A patient support area
- An upgraded medical device reprocessing department

This investment enables the facility to provide approximately 120 surgeries per month, significantly enhancing rural surgical capacity and reducing the need for patients to travel long distances for care.

### ED Wait Times, EHS Performance, and Workforce Initiatives

The Government of Alberta implemented several initiatives in 2024-25 to reduce ED wait times by increasing front-line staff capacity and improving patient flow. As of March 2025, there were 3,888 AHS nurses (including registered nurses, licensed practical nurses, and health care aides) working in EDs —157 more than the previous year. Between April 1, 2024, and March 31, 2025, 83 ED physicians were recruited, including:

- Physicians with a specialty designation
- Family physicians with additional emergency medicine certification
- Family physicians working in EDs without added credentials (Note: Covenant Health physicians are not included in these totals.)

#### Performance Measure 1.a:

**Emergency department wait times: 90th percentile time to initial physician assessment in the 16 largest sites (in hours)**

|                                                                                                                        | 2021-22 | 2022-23 | 2023-24 | 2024-25 Target        | 2024-25 Actual |
|------------------------------------------------------------------------------------------------------------------------|---------|---------|---------|-----------------------|----------------|
| <b>ED wait times: 90th percentile time to initial physician assessment in the 16 largest sites (hours)<sup>1</sup></b> | 4.5*    | 6.2*    | 6.7     | Below 2023-24 results | 7.0            |

**Source:** Emergency Department Visit Data Repository, Emergency Department Information Systems, AHS

<sup>1</sup>A lower time indicates faster access to care.

\*The methodology was revised to exclude data for patients that 'Left Without Being Seen' and results were updated from 4.6 to 4.5 hours in 2021-22 and 6.3 to 6.2 hours in 2022-23.

The performance measure on ED wait times tracks the time to see a physician after being triaged on arrival 90 per cent of the time in the 16 largest hospitals. The lower the number the better, as it demonstrates patients are receiving timely assessment and treatment in EDs. In 2024-25, the wait time to see a physician in the ED was 7.0 hours compared to 6.7 hours in 2023-24. The trend shows

that patients were waiting 0.3 hours longer in the ED to see a physician in 2024-25 compared with last year. The longer ED wait times was due to an increase in ED visits and the number of patients with the highest level of acuity that require aggressive interventions such as patients with cardiac arrest or major trauma.

An approximate 2.6 per cent increase in total ED visits and a 4.0 per cent increase in high-acuity presentations were observed in 2024-25. In addition, hospital overcrowding and delays in transferring patients to continuing care settings contributed to extended wait times.

To address these pressures, Alberta Health worked on initiatives such as:

- Expanding community care alternatives to divert non-urgent cases
- Improving access to continuing care beds
- Increasing hospital capacity
- Implementing streamlined patient discharge and treatment protocols in acute care.

EHS System Improvements

In 2024-25, the government spent \$704 million to expand EHS and respond to the 98 recommendations from the Alberta Emergency Services Provincial Advisory Committee (AEPAC) and the EHS Dispatch Review.

A new Alberta EMS Standing Committee was established and began meeting on April 24, 2024. This committee includes frontline paramedics, municipal and Indigenous representatives, and other community partners. Subcommittees address air ambulance, dispatch, ground ambulance, medical first response, and workforce. The core committee meets monthly, while subcommittees meet every eight weeks. These advisory groups are mandated until spring 2027.

Performance Measure 1.b:  
EMS 90th percentile response times (minutes) for the most urgent (life threatening) calls

|                                              | EMS Response Time for the Most Urgent Calls (minutes) <sup>1</sup> |         |         |                       |                |
|----------------------------------------------|--------------------------------------------------------------------|---------|---------|-----------------------|----------------|
|                                              | 2021-22                                                            | 2022-23 | 2023-24 | 2024-25 Target        | 2024-25 Actual |
| Metro/urban communities                      | 14.6                                                               | 17.5    | 13.8    | Below 2023-24 results | 14.2           |
| Communities with over 3,000 residents        | 18.6                                                               | 18.9    | 16.3    | Below 2023-24 results | 16.4           |
| Rural communities with under 3,000 residents | 33.8                                                               | 33.9    | 33.3    | Below 2023-24 results | 29.2           |
| Remote communities                           | 55.4                                                               | 61.8    | 64.9    | Below 2023-24 results | 52.0           |

Source: EMS Computer Aided Dispatch (CAD) data captured with EMS Communications Centres, Alberta Health

**Notes:** <sup>1</sup> Measures 90th percentile, meaning these are the response times for 90 per cent of activity. Sites that experienced less than 20 events were excluded from the calculations.

EMS response times for 90 per cent of activity dropped between 2023-24 to 2024-25 for remote communities and for rural community residents. However, EMS response times for 90 per cent of activity were similar for communities with more than 3,000 residents and have slightly increased for metro/urban communities over this same period. It is important to note that the very low call volumes in the two rural and remote categories result in high sensitivity of response times to changes in performance.

Alberta's government is committed to ensuring EMS is available to Albertans when and where they need it. Alberta Health and AHS continued working to divert low-acuity calls to 811, reduce EMS offload times, and free up ambulances faster.

The EMS Return to Service initiative, launched in 2023, helped reduce handover time from paramedics to ED staff. Average handover time dropped from 1.7 hours in December 2024 to 1.6 hours in February 2025. In January 2025, paramedics gained view-only access to Connect Care, enabling real-time access to patient histories and improving on-site decision-making.

## Surgical Volumes and Cancer Care Outcomes

In 2024-25, significant progress was achieved in expanding surgical capacity as part of the ASI implementation. CSFs enabled a greater volume of publicly funded surgeries at no cost to patients and continued to be an integral part of the ASI. There is a total of 48 CSF agreements, of which 20 pertain to oral and maxillofacial surgery (OMF) and are contracted directly with the surgeons rather than through the CSF. Only one new agreement was established in the 2023-24 financial year for Ear, Nose and Throat (ENT) (CLM208529), which is included in the total count. No new agreements have been initiated for the 2024-25 financial year. CSFs remain an extension of hospital capacity and are used in many other Canadian health systems. Under the *Health Facilities Act*, CSFs providing publicly funded insured services must be accredited by the College of Physicians and Surgeons of Alberta (CPSA) and have a signed service contract with AHS.

Through these dedicated efforts, as of March 31, 2025, approximately 318,601 surgeries had been completed, surpassing the 2024-25 fiscal year target of 310,000. This represents an increase of 14,006 surgeries, or 4.6 per cent, compared with the 304,595 completed in 2023-24.

As of March 31, 2025, 74,928 cases were waiting for surgery. Of these cases, 57.8 per cent were waiting within clinically recommended timelines. Efforts remain underway to reduce wait times further and ensure equitable access across the province.

For the fiscal year 2024-25, 52,810 cases have been completed in rural acute care facilities. This represents an increase in rural hospitals of 5.4 per cent in volumes completed in 2024-25 compared with fiscal year 2023-24, when 50,090 surgeries were completed.

Further, approximately 23,015 cancer surgeries were completed in 2024-25, a 9.4 per cent increase compared with the same period in 2023-24. Nearly 55 per cent of cancer surgeries were completed within clinically recommended wait times, helping to ensure patients receive timely access to potentially life-saving procedures.

The Arthur J.E. Child Comprehensive Cancer Centre became fully operational on October 28, 2024. The centre provides additional capacity for timely access to cancer diagnostics and treatment, supporting optimal health outcomes for cancer patients. This new cancer centre in Calgary engages patients at the centre of a multidisciplinary health system and offers a world-class environment for

cancer care. There are 160 inpatient beds and 90 chemotherapy treatment spaces available to meet patient needs. All clinical areas are in use with flexibility for future growth as demand increases. A new clinical trial space is expected to open later in 2025, with additional program expansions guided by patient and clinical priorities.

In 2024-25, Alberta Health provided Wellspring Alberta with over \$300,000 in funding to expand non-clinical, psycho-social cancer care support services for remote and rural areas, including Indigenous communities. With this grant, Wellspring Alberta is expanding both online and in-person programming to improve access to supportive care, strengthen mental health outcomes, and promote equity in the cancer care experience. Participants have reported improved well-being as a result of the expanded services.

System Initiatives and Workforce Investments

The increase in completed surgical procedures this year reflects meaningful progress in expanding Alberta’s surgical capacity and reducing wait times. This outcome demonstrates the Ministry of Health’s continued commitment to the ASI. Building on this progress, the ministry implemented system-wide improvements—including enhanced surgical referral pathways, prioritization based on clinical urgency and continued use of CSFs-to deliver less-complex procedures efficiently.

Percentage of surgical procedures that met national wait time benchmarks

| Surgery (national benchmark)                          | Prior Years’ Results |         |         |         | 2024-25 Actual |
|-------------------------------------------------------|----------------------|---------|---------|---------|----------------|
|                                                       | 2020-21              | 2021-22 | 2022-23 | 2023-24 |                |
| Hip replacement <sup>1</sup><br>(benchmark 182 days)  | 51.6%                | 51.2%   | 43.0%   | 62.4%   | 72.8%          |
| Knee replacement <sup>2</sup><br>(benchmark 182 days) | 43.3%                | 39.7%   | 32.5%   | 53.3%   | 62.5%          |
| Cataract surgery<br>(benchmark 112 days)              | 45.3%                | 64.7%   | 64.7%   | 59.7%   | 62.8%          |

**Source:** Local Operating Room (OR) Information Systems, AHS and OR Data Repository, AHS  
<sup>1</sup>Hip replacement data include totals, partials, resurfacings, primaries, and revisions for scheduled/elective cases.  
<sup>2</sup>Knee replacement data include totals, partials, primaries and revisions for scheduled/elective cases.

The 2024-25 results for hip, knee and cataract surgical procedures included in this indicator show an increase in the number of Albertans receiving these surgical procedures within national benchmark wait times when compared with 2023-24 results.

Hip replacements increased from 62.4 per cent to 72.8 per cent, and knee replacements improved from 53.3 per cent to 62.5 per cent. In addition to improved compliance with the national benchmark for hips and knees, the overall number of surgeries for all three procedures has increased at the same time.

In 2023–24, 59.7 per cent of cataract surgeries met the national benchmark, a five-percentage point decline from the previous year. This drop is attributed to prioritizing patients who had waited the longest, which lowered the percentage within target despite over 500 more surgeries being completed on time. In 2024-25, performance improved to 62.8 per cent, reflecting recovery from backlog reduction efforts.

Alberta's government continues to work with AHS to ensure more Albertans receive their surgeries in a timely manner. Efforts are ongoing to implement an \$8.2-million grant provided by Alberta Health to further initiatives. The funds support AHS in implementing surgical smoothing techniques—such as separating scheduled and urgent surgeries into different operating rooms—to improve patient flow through Alberta's busiest hospitals. AHS partnered with the Institute for Healthcare Optimization (IHO) to implement these methods. As of 2024-25, six hospitals had implemented the IHO methodology in Phase 1, with 10 more scheduled for Phase 2.

Under the ASI, Alberta performed more surgeries in rural communities and performed more publicly funded surgeries at CSFs. In fiscal year 2024-25, an additional 2,180 surgeries were completed in rural facilities compared with fiscal year 2023-24, while an additional 2,500 cases were completed at CSFs. Overall, approaches like these, and others, facilitated the completion of more surgeries in 2024-25 than any prior fiscal year, with a total of approximately 318,601 cases completed and more than 13,000 additional surgeries compared to the year prior.

In 2024-25, Alberta Health also amended a prior grant to the CPSA to provide additional funding to support expanding accreditation activities, in line with growth in CSF procedures. This builds on a previous Alberta Health grant and helps CPSA continue meeting its accreditation obligations.

Alberta Health entered into several agreements with AHS to implement ASI-related projects:

- Invested over \$3 million in the Rapid Access Clinics program to provide faster, more efficient access to care for musculoskeletal conditions.
- AHS launched a \$2.7 million prehabilitation pilot, helping patients prepare for surgery with education and supports. As of March 2025, 2,258 referrals had been received and 3,998 visits completed, with 84 per cent of participants reporting proved surgical recovery.
- Allocated over \$800,000 to the Rural Surgical and Obstetrical Network of Alberta to enhance surgical and maternity care in rural communities.
- Provided over \$1.1 million in funding to support CPSA regulatory oversight and accreditation related to CSFs and the monitoring of, and response to, safety incidents.

The Government of Alberta continued to address surgical workforce challenges to support system expansion. As of March 31, 2025, there were 466 anesthesiologists registered in Alberta, up from 455 in 2024, representing a 2.4 per cent increase.

In July 2024, the Royal Alexandra Hospital experienced a temporary disruption in orthopedic services due to staffing shortages. To address this, Alberta Health and AHS collaborated on a strategic action plan to restore orthopedic surgery capacity. This included resuming procedures at reduced volumes, prioritizing patients on wait lists, and continuing overnight-stay surgeries. AHS also maintained recruitment efforts to restore full physician coverage and mitigate similar disruptions elsewhere.

Province-wide, AHS continued working to increase operating room hours by addressing workforce gaps. These efforts included targeted hiring and leadership development. As of January 31, 2025, Cancer Care Alberta had recruited 41 full-time oncologists, 18 full-time hospitalists, and 115 clinical operational staff. These investments reflect Alberta's ongoing efforts to strengthen surgical and cancer care teams and reduce wait times across the province.

## Capital Investments, Diagnostic Access, and ICU/NICU Expansion

The Government of Alberta continued investing in medical device reprocessing (MDR) as part of broader efforts to reduce surgical backlogs and increase operating capacity. A total of \$159 million in funding was allocated across Phase 1 and Phase 2 of the MDR program over three years. These funds are supporting renovations in nine hospitals: Foothills Medical Centre (Calgary), Westlock Health Centre, Northern Lights Regional Hospital Centre (Fort McMurray), Sturgeon Community Hospital (St. Albert), Royal Alexandra Hospital (Edmonton), Grey Nuns Community Hospital (Edmonton), Misericordia Community Hospital (Edmonton), Alberta Children's Hospital (Calgary), and Drumheller Health Centre.

This capital program supports critical upgrades and expansions to sterilization and reprocessing areas, enhancing compliance with safety standards and improving operational efficiency. All nine projects advanced through planning and design in 2024-25. As part of the ASI, the province also provided \$313 million over three years for the ASI Capital Program to increase surgical capacity in 18 communities through 39 projects. These initiatives will deliver 21 new and 49 upgraded operating rooms across both urban and rural locations and include both Alberta Infrastructure-led and AHS-led projects.

The ministry continued to make strategic infrastructure investments to support the delivery of publicly funded health services. Significant advancements in 2024-25 included:

- **Arthur J.E. Child Comprehensive Cancer Centre:** A \$1.4-billion investment enabled the opening of this state-of-the-art centre on October 28, 2024, offering 160 inpatient beds, 90 chemotherapy spaces, 100-plus exam rooms, 12 radiation vaults (plus 3 shelled for future growth), clinical trials space, and full outpatient services.
- **Red Deer Regional Hospital Centre Redevelopment:** Supported by a \$1.8-billion investment, this project includes a new inpatient tower, expanded ED, and surgical suite renovations. Construction of the inpatient tower began in 2024.
- **Rockyview General Hospital:** An \$84-million expansion project was completed in November 2024, improving intensive care, coronary care, and endoscopy capacity. The redevelopment increased the ICU and CCU to 25 single patient rooms (13 ICU and 12 CCU) from the previous 17 and moved the unit into a space that better supports patient and family centred care.
- **Budget 2024** included \$313 million over three years for the Alberta Surgical Initiative Capital Program to increase surgical capacity in 18 communities through 39 projects. The following projects were completed and became operational:
  - The second operating room at Rocky Mountain House Health Centre became operational in April 2024.
  - An operating room upgrade at the McCaig Tower in Foothills Medical Centre in Calgary was completed in Dec 2024.
  - Upgrade to two operating rooms, Medical Device Reprocessing Department (MDRD) enhancements and inpatient room renovations at Chinook Regional Hospital in Lethbridge was completed in Jan 2025.

In *Budget 2024*, the government allocated \$512 million over three years, including \$172 million in 2024-25 through the Capital Maintenance and Renewal Program, to maintain existing infrastructure. The Rural Health Facilities Revitalization Program continued to deliver smaller-scale renovations in rural communities, helping maintain access to safe, modern care across the province. Notable projects included renovations to the emergency department at Taber Health Centre

(complete), emergency department triage at Drayton Valley Hospital and Care Centre (substantial completion), laboratory renovations at Fairview Health Complex (complete), and the purchase and installation of a CT scanner at Pincher Creek Health Centre (complete).

### Percentage of scheduled surgeries performed in Chartered Surgical Facilities

|                                                            | 2021-22 | 2022-23 | 2023-24 | 2024-25<br>Actual |
|------------------------------------------------------------|---------|---------|---------|-------------------|
| <b>Percentage of scheduled surgeries performed in CSFs</b> | 19%     | 16.2%   | 20.5%*  | 20.2%             |

**Source:** Main Operating Room (OR) Information Systems, OR Data Repository, AHS

\*AHS Main OR information system sources as extracted to OR data repository as of March 31, 2024. Sites that do not have OR information systems were not included.

This metric tracks the percentage of scheduled surgeries performed in CSFs. More scheduled surgeries performed in CSFs helps reduce wait times by increasing surgery volumes and improving access so that Albertans receive their procedures where and when they are needed. CSFs continue to be an integral part of the ASI implementation to enable increased volumes of publicly funded surgeries at no cost to patients. In 2024-25, there were 38 existing CSFs, and accredited CSFs provided around 64,500 surgeries (20.2 per cent of publicly scheduled surgeries). This is an increase of around 2,050 surgeries from 2023-24 (3.3 per cent increase).

### Annual number of surgeries provided

|                                            | 2020-21 | 2021-22 | 2022-23 | 2023-24 | 2024-25 |
|--------------------------------------------|---------|---------|---------|---------|---------|
| <b>Annual number of surgeries provided</b> | 277,000 | 278,600 | 294,300 | 304,595 | 318,601 |

**Source:** Local Operating Room (OR) Information Systems, AHS, and OR Data Repository, AHS

This performance indicator reflects the estimated number of scheduled and emergency surgeries that are completed and is used to monitor growth in surgical capacity resulting from implementation of initiatives such as the ASI and HCAP.

In 2024-25, 318,601 surgeries were performed in Alberta, up by almost 15,000 from 304,595 surgeries in 2023-24. This represents an increase of 4.6 per cent and can in part be credited to the ASI — the government's plan to increase the number of surgeries and ensure Albertans receive their procedures within clinically recommended wait times. Additionally, while the number of surgeries performed in CSFs continued to rise, this year's growth reflects a more modest increase compared to the larger increases seen in previous years. The volume of surgeries performed at CSFs is expected to continue to increase over the coming years as more CSFs become operational.

### Percentage of patients seen within established targets for CT scan and MRI scan

|                                                                                                                     | 2021-22 | 2022-23 | 2023-24 | 2024-25 |
|---------------------------------------------------------------------------------------------------------------------|---------|---------|---------|---------|
| <b>Percentage of MRI scans within target*: Percentage of patients seen within established targets for MRI scans</b> | 68%     | 49%     | 42%     | 41%     |
| <b>Percentage of CT scans within target**: Percentage of patients seen within established targets for CT scan</b>   | 87%     | 80%     | 80%     | 80%     |

**Source:** Path to Care DI Dashboard - OWL Production (1.0); Path to Care DI Access Owl - AHS Tableau Server

\*MRI targets: Emergency department patients - within 24 hours; hospital patients - 24-72 hours (condition dependent); priority 1 scheduled - within 7 days; priority 2 scheduled - within 30 days; priority 3 scheduled - within 90 days

\*\*CT targets: Emergency department patients - within 24 hours; hospital patients - within 24 hours; priority 1 scheduled - within 7 days; priority 2 scheduled - within 30 days; priority 3 scheduled - within 60 days

The percentage of patients receiving scans within provincial wait time targets remained consistent for CT scans and declined marginally for MRIs. This trend reflects factors that continue to impact service delivery, including rising demand and case complexity driven by Alberta's aging population, equipment and site infrastructure constraints from further scaling, and workforce recruitment and retention challenges.

To improve access to diagnostic imaging, the Government of Alberta allocated \$15 million in 2024-25 to community diagnostic imaging providers. With this investment, AHS expects to perform 41,000 additional CT scans and 12,000 additional MRI scans. As a result:

- Median CT wait time dropped from 28 to 21 days
- Median MRI wait time dropped from 57 to 53 days
- AHS delivered approximately 540,000 CT and 230,000 MRI scans in 2024-25

Wait times for community lab services were improved in the following ways:

- In-person waits dropped by 36 per cent (35.2 to 22.6 minutes)
- Next available appointment wait times decreased by 54 per cent (from 9.3 to 4.4 days)
- Patient visits increased by 11 per cent (from 3.94M to 4.38M)

Following the transition of services from DynaLIFE to Alberta Precision Laboratories (APL) in 2023-24, wait times have consistently met or exceeded targets. APL continues to increase staffing and appointment capacity, and 92 per cent of respondents to a recent patient survey reported satisfaction with their lab experience.

In 2024, Alberta signed a three-year, approximately \$54-million bilateral agreement with the federal government under the National Strategy for Drugs for Rare Diseases. This will provide earlier access to emerging rare disease drugs, as well as improve diagnostics and screening. Of the total, up to \$5.4 million annually is allocated to improving rare disease screening and testing.

As part of a \$283-million four-year grant through the 2022-26 Business Plan, the government continued to expand ICU baseline funding to support the development and operation of an additional 50 permanent ICU beds across the province. As of November 2024, AHS added one more ICU bed at Foothills Medical Centre, bringing the provincial total to 224 ICU beds.

To address neonatal capacity pressures, an additional 21 neonatal ICU beds were approved. As of March 31, 2025, six of these beds were operational, increasing Alberta's total NICU capacity to 315 beds. The new beds were distributed as follows:

- 3 in Calgary Zone at Peter Lougheed Centre
- 2 at Grey Nuns Community Hospital and 1 at Misericordia Community Hospital in Edmonton Zone

### **EHS Workforce Development and Response Capacity**

Alberta Health worked with AHS to implement workforce strategies aimed at reducing vacancies among non-casual EHS staff. Between Q1 and Q3 of 2024-25:

- The number of primary and advanced care paramedics increased from 3,176 to 3,220; and
- AHS hired 419 new staff, including 295 paramedics—164 into full-time roles.

To strengthen rural response capacity:

- The number of emergency medical responders (EMRs) in the public EMS system grew from 80 in November 2022 to 219 by December 2024.
- Significant growth occurred in the North Zone and South Zone, with increases across all zones.
- EMRs continue to serve lower-acuity transfer and response roles, particularly in rural and remote areas.

### **EHS Fleet Expansion**

To ensure sufficient vehicle supply, Alberta invested \$15 million in 2024-25 toward a \$35-million three-year commitment under the EMS Vehicle Capital Grant Program. This funded the purchase of new EHS vehicles and provided additional equipment to meet the needs of Albertans in both urban and rural areas.

As of February 18, 2025, EHS transport volumes were approximately 15 per cent higher than February 2023, highlighting the importance of fleet growth to meet rising demand.

## **1.2 Ensure equitable and timely access to health services across the province, including in remote, rural and Indigenous communities, by implementing workforce strategies and policies to attract, recruit and retain health professionals, and modernizing critical capabilities in the delivery of health care services**

In 2024-25, Alberta Health advanced efforts to improve maternal health outcomes and reduce strain on acute care services through the development and implementation of Alberta's Midwifery Strategy. Released in February 2025, the strategy provides a long-term framework to guide investments of up to \$10 million in funding over three years to expand midwifery services across the province, particularly in rural and remote communities with limited access. As of January 2025,

181 midwives were practising across Alberta, a 26 per cent increase from 2020. In 2024-25, the first \$1.4-million grant under this strategy was established to support the development of Indigenous second birth attendants, further supporting culturally safe maternity care.

Alberta Health provided over \$7.06 million in funding to the Rural Health Professions Action Plan (RhPAP) in 2024-25. Although the Rural Education Supplement and Integrated Doctor Experience Program ended March 31, 2024, efforts through RhPAP successfully committed 20 physicians for a total of 59 years of service in rural Alberta. In 2024-25, RhPAP spent \$445,000 to fulfil ongoing contractual commitments.

### **Strengthening Health Workforce Capacity and Retention**

Alberta Health and the Alberta Medical Association (AMA) completed a joint request for proposals to review the Rural Remote Northern Program, which provides incentives to attract physicians to underserved communities. Recommendations from the review are expected by March 31, 2025, and will inform future improvements. The Government of Alberta also continued to implement its Health Workforce Strategy, released in December 2024, to strengthen Alberta's capacity to recruit and retain health professionals. The number of physicians practising in Alberta increased by 474 (4.2 per cent) from December 2023 to December 2024, demonstrating meaningful year-over-year growth. This progress builds on the 2023-24 investments in workforce infrastructure and reflects Alberta's continued commitment to sustainable physician supply and improved patient access.

### **Modernizing Primary Care Models**

Alberta Health collaborated with the AMA to develop a new Primary Care Physician Compensation Model (PCPCM) to support family medicine and rural generalist physicians. Implemented on April 1, 2025, this new model is designed to ensure Alberta's family doctors are competitively compensated, while delivering patient-centred care, and aims to enhance access to primary care across the province, particularly in underserved areas.

The PCPCM is structured to:

- Encourage physicians to grow their patient panels and support full-time practice.
- Reward high-volume, high-quality care through incentives for after-hours access, technology use, and team-based service delivery.

As of March 31, 2025, 1059 physicians were approved to apply for PCPCM, of which 818 physicians submitted PCPCM applications to Alberta Health. This initiative represents a shift from fee for service to a more integrated, outcome-oriented approach, in line with national best practices and aligned with Alberta Health Workforce Strategy.

### **Nurse Practitioner Primary Care Program (NPPCP)**

The Nurse Practitioner Primary Care Program (NPPCP) launched on April 1, 2024, as part of broader efforts to expand access through team-based care. The program provides compensation that supports nurse practitioners (NPs) so they can practise comprehensive patient care autonomously, whether in their own practices or in existing primary care settings. As of March 24, 2025:

- 53 NPs were practising under the NPPCP, of which 20 are practising in rural communities.
- Over 31,905 patients had been seen through the program.

- Nearly 21,282 patients were paneled to NPPCP providers as their most responsible practitioner.

This early uptake highlights the program's potential to reduce access barriers and relieve pressure on the physician system.

### **Supporting Indigenous Health Access and Equity**

The Indigenous Patient Navigator Grant Program that distributed \$7.6 million over three years, continued to address access barriers and navigation challenges faced by First Nations, Métis, and Inuit patients and communities. The program provides financial support to recruit and retain patient navigators who assist individuals in accessing primary health care and community services.

In 2024-25:

- Alberta Health received 35 applications under the program.
- 18 Indigenous Patient Navigators were funded across the province.

Building on this success, the Government of Canada contributed an additional \$6.3 million over three years to launch a second round of the Navigator Program through the Bilateral Indigenous Patient Navigator Grant Program. Combined, total funding under both streams reached \$13.9 million over three years.

Through the bilateral program, eligible applicants may receive:

- Up to \$150,000 annually (or \$450,000 over three years)
- Support covering salary, benefits, and direct program costs

These navigators continue to play a vital role in advancing culturally safe care and improving Indigenous patients' experience with Alberta's health system.

### **Rural Health Priorities**

In October 2024, Alberta Health reaffirmed its commitment to rural and remote care by publishing the Rural Health Action Plan (RHAP), which is aligned with ongoing system refocusing initiatives, including the Modernizing Alberta's Primary Health Care System (MAPS) initiative and Alberta Health Workforce Strategy.

The RHAP outlines five rural priorities:

- Workforce
- Access
- Models of care
- Community care
- Prevention and wellness

These priorities reflect continued investment in rural health system readiness and equity of access for smaller communities.

## Investing in Future Rural Physicians

As of October 2024, the Government of Alberta launched the two-year \$16 million Rural and Remote Family Medicine Resident Physician Bursary Program. The program provides financial support to family medicine residents at the University of Alberta and University of Calgary who commit to practising in rural or remote Alberta for a minimum of three years following residency.

In its first year:

- Applications opened on January 6, 2025
- Approximately \$2.8 million was spent to support 21 residents
- Bursary recipients are expected to begin service placements in 2025–26

This program builds on Alberta’s ongoing investment in rural physician attraction and aligns with the broader Health Workforce Strategy by strengthening the rural physician pipeline early in medical training.

## Supporting Community Health Centres

The underserved grant program provided \$30 million in funding to support four community health centres (CHCs): The Alex (\$11.6 million) and CUPS Calgary Society (\$6.6 million) located in Calgary, and Jasper Place Wellness Centre (\$4.8 million) and Radius Community Health and Healing (\$7 million) located in Edmonton. Funds from the grant agreements executed in February 2025 will enhance the ability of CHCs to deliver team-based, community focused care by:

- Enhancing access to primary care for underserved and marginalized populations
- Expanding mental health and outreach services
- Supporting walk-in and team-based models of care
- Providing critical services such as housing navigation, recovery support, and food access programs

## Modernizing Pharmacy Infrastructure

To support medication safety and modernize infrastructure, the Government of Alberta committed \$57 million over three years to renovate and upgrade 33 pharmacies across the province. This pharmacy capital program ensures compliance with new sterile compounding standards set by the National Association of Pharmacy Regulatory Authorities, adopted by the Alberta College of Pharmacy. This initiative helps ensure consistent access to safe, high-quality pharmaceutical services in hospitals and care centres, especially in rural and remote areas.

### 1.3 Ensure processes for resolving patient concerns are effective, streamlined, and consistent across the province

The ministry worked to ensure that the patient complaints process remained fair, responsive, and accessible. AHS implemented targeted enhancements to its complaint management system, with a focus on strengthening mechanisms for reviewing and addressing feedback from patients and families in a timely and effective manner.

As part of the Alberta government's continued efforts to strengthen patient-centred care, transparency, and accountability, the ministry implemented recommendations from the Office of the Alberta Health Advocate and Mental Health Patient Advocate. Building on progress made in 2023-24, this work included:

- Advancing standardized follow-up protocols for patient complaints
- Establishing mandatory information-sharing requirements to support a consistent approach to resolution across the system

### **Strengthening the Voice of Patients and Caregivers**

In alignment with the MAPS Two-Year Implementation Plan, Alberta Health is supporting the launch of Care Opinion Canada, an online platform designed to collect public feedback on health care experiences. The initiative stems from the MAPS advisory panels' recommendation to strengthen the voice of patients and caregivers within the health system. In March 2024, \$704,000 in MAPS initiative funding was provided to support the pilot launch of Care Opinion by amending an existing grant with Imagine Citizens Network (ICN). ICN has achieved several key milestones, including signing a licensing agreement to establish Care Opinion Canada, hiring core staff, and securing early adopters for a successful pilot of the program.

### **Improving Culturally Appropriate Complaint Resolution**

Building on previous work, the Alberta government continued to improve the process for addressing Indigenous patient complaints and expanding access to culturally appropriate, high-quality health services for First Nations, Métis, and Inuit peoples.

In 2024-25, the government hired Alberta's first Indigenous Patient Safety Investigator and Advocate, Anthony Cardinal. The role was established to ensure Indigenous patients can advocate for themselves while also receiving support from a trusted voice when navigating the health system. The investigator's mandate includes:

- Listening to and supporting Indigenous patients and families in addressing their concerns
- Ensuring concerns are addressed in culturally safe, appropriate ways
- Addressing experiences of racism in the health system
- Supporting implementation of the Alberta Health Charter

This appointment aligns with recommendations made by the Indigenous Primary Health Care Implementation Panel and reinforces Alberta's commitment to improving trust, access, and culturally safe care for Indigenous communities.

#### **1.4 Increase oversight to improve safety while reducing red tape within the health system by streamlining processes and reducing duplication**

As part of its ongoing commitment to regulatory efficiency, the Government of Alberta implemented red tape reduction initiatives to remove unnecessary administrative barriers and modernize outdated practices. This included restructuring and modernizing health legislation by removing unnecessary requirements to access services, and by ensuring that policies reflect modern models of care. These efforts reduced administrative barriers, strengthened front-line capacity, and improved patient outcomes.

Through collaboration with the ministries of Service Alberta and Red Tape Reduction, Health, and Technology and Innovation, the Government of Alberta is advancing the health card modernization initiative, which aims to address fraudulent use of the Alberta Health Care Insurance Plan (AHCIP). For example, there are currently more than five million registered paper health cards despite Alberta's population being approximately 4.7 million. Options to modernize the health care card, while improving both its' security and durability, are under consideration.

The Body of Rules Agreement Netcare Dialog (BRAND) application has been developed to streamline the operation and management of Alberta Netcare. Once launched, BRAND will result in a simplified electronic agreement process, significantly reducing administrative burden for physicians.

Through these targeted initiatives, the ministry remains committed to:

- Enhancing timely and equitable access to care
- Expanding patient choice
- Improving overall quality of services

As these reforms are integrated into a refocused and unified provincial health system, continuous performance monitoring will ensure transparency, accountability, and better outcomes for Albertans.

## **Outcome Two: A modernized and refocused health care system that delivers the highest-quality patient care to all Albertans, including Indigenous Peoples, regardless of where they reside in the province, and is fiscally sustainable**

Alberta is refocusing the health care system to prioritize patient care and empower frontline health care workers, including those serving rural, remote and Indigenous communities, to deliver the highest quality health care.

*The Ministry of Health is reporting on advancements in palliative and end-of-life care, along with strengthened licensing and monitoring, as key components of a modernized and fiscally sustainable health care system that delivers high-quality care to all Albertans.*

### **Key Objectives**

#### **2.1 Improve patient outcomes and access through the creation of dedicated organizations for acute care, continuing care, and primary care, within a single functioning health care system**

The Government of Alberta continues to focus on controlling the rising costs of health care while providing Albertans with the health care services they expect and deserve. In 2024-25, the ministry worked with health system partners to balance between fiscal responsibility and necessary investments. The goal is a strong, responsive, and refocused health system that supports economic growth and world-class health care, with the capacity to respond to emerging public health challenges.

On October 31, 2024, the Government of Alberta introduced legislative amendments through the *Health Statutes Amendment Act* to continue building a health care system that responds to the changing needs of Albertans. The *Health Statutes Amendment Act* ensures Albertans have a health system that prioritizes their need to:

- Find a primary care provider
- Get urgent care without long wait times
- Access high-quality continuing care
- Receive mental health and addiction services across the province

These legislative amendments enabled a unified health care system with four provincial health agencies (PHAs). As part of this refocusing, responsibility for health system planning transitioned from AHS to Alberta Health. As part of the transition, Alberta Health led a province-wide health system assessment that looked at current and future health needs, services, workforce, and infrastructure. The assessment helped shape funding decisions and promoted long-term planning.

The Ministries of Health, Mental Health and Addiction, and Seniors, Community and Social Services created four PHAs:

- Acute Care Alberta (ACA) (operational as of April 1, 2025)
- Assisted Living Alberta (ALA) (legal entity as of April 1, 2025)
- Primary Care Alberta (PCA) (operational as of February 1, 2025)
- Recovery Alberta (RA) (legal entity as of July 1, 2024).

Ministerial orders were signed early to support smooth transitions. Recovery Alberta is led by the Ministry of Mental Health and Addiction and not covered in this report. Additionally, the responsibility for the stand-up of the continuing care agency was transferred to the Ministry of Seniors, Community, and Social Services in fall 2024.

AHS will continue to have a strong role in the refocused system by concentrating on delivering acute care services. New regulations have been introduced to formalize the designation and oversight of health services delivery organizations and to establish the AHS Provincial Health Corporation.

Each PHA has a specific focus. ACA manages hospital care, urgent care centres, CSFs, EHS and cancer care. ALA oversees continuing care, including care homes and home supports. PCA coordinates primary health services and works to connect Albertans with care teams. RA provides recovery-oriented mental health and addiction services and correctional health services in Alberta.

A health shared services entity (SSE) is being established to provide corporate and support services for all PHAs and health services delivery organizations. It will be independent and is expected to be operational by September 2025.

As the system establishes additional organizations, tools are being put in place to make sure it works collaboratively. The Integration Council was established that looks at system-wide strategic goals, finds efficiencies, and removes barriers to system coordination. The council began meeting regularly in March 2024. Membership includes ministers, deputy ministers, PHA and AHS leaders, and senior executives from HQCA and the Centre of Recovery Excellence.

To enhance local decision-making, Alberta Health will establish 14 regional advisory councils (RACs) and one Indigenous Advisory Council. RACs and the Indigenous Advisory Council are guiding Alberta's health care transformation by ensuring community perspectives are heard. RACs will provide advice and recommendations on workforce, capital, and clinical service planning, identify local health issues, and support locally driven solutions. The Indigenous Advisory Council will bring forward Indigenous priorities, inform culturally appropriate care, and explore integrating traditional practices. In 2024-25:

- Nearly 500 candidates applied and approximately 100 additional individuals were nominated by various stakeholders for minister's consideration to appoint
- 210 interviews were held
- Operational frameworks, policies and other materials were created
- Training and onboarding materials for council members were developed
- A custom system was built to support council work.

The following other changes have supported the Health Care System Refocusing Initiative:

- The management and oversight of all EHS — including ambulances and paramedic care — will be transferred from AHS to ACA later in 2025. These changes will help refocus EHS to better meet the needs of Albertans and ensure improved access to the best health care possible.
- Cancer care services will also move out of AHS under the oversight of ACA in 2025.

- Services for organ and tissue donation and transplantation will also move out of AHS and under the oversight of ACA.
- ALA will be operational by fall 2025, aligning medical and non-medical care, increasing continuing care spaces, reducing wait times, and offering wraparound supports for Albertans who require various levels of care. ALA will support seniors, individuals with disabilities, and vulnerable populations needing both temporary and long-term care.
- Effective January 6, 2025, the Ministry of Seniors, Community and Social Services assumed responsibility for the governance of Alberta's continuing care sector; however, some functions remain in Health, including the License and Compliance Monitoring Branch, Continuing Care Information Systems and Medical Assistance in Dying.

In 2024-25, the Procurement and System Optimization Secretariat, Alberta Health division, continued to work closely with AHS Contracting, Procurement and Supply Management and program areas to develop and implement contract assignment approaches that will enable the PHAs to continue the provision of appropriate health care and support to Albertans without disruption.

A total of 82 contracts were assigned from AHS to PCA effective February 1st, 2025, as per Ministerial Order 802/2025. These contracts were part of the initial wave of contracts that were directly related to the transition of programs and services which included Health Link, the Midwifery Program and the Provincial Primary Health Care Program.

In February 2025, the ministry reported \$22.5 million in operational site and zone rebates and forecasted \$288 million in pharmacy and cancer care rebates and \$38 million in surgical care rebates. Additionally, \$5.8 million in funding supported education, training, fellowships, and research – key components of clinical programs enabled through these realized benefits. These efficiencies support the long-term sustainability of Alberta's health system and reflect the ministry's ongoing efforts to modernize care delivery and optimize resources.

## **2.2 Engage the health workforce, patients, families, caregivers, and system stakeholders to understand front-line and local perspectives to effectively implement the Health Care System Refocusing Initiative**

In 2024-25, Alberta Health implemented a layered, province-wide engagement strategy to ensure the voices of Albertans informed the Health Care System Refocusing Initiative. This approach included in-person public sessions, targeted discussions with Indigenous and francophone communities, health workforce engagement, and online engagement. The goal was to gather diverse perspectives and lived experiences to guide the modernization of Alberta's health care system and support sustainable, effective, and locally responsive care.

### **Public Engagement**

Between January 24 and April 12, 2024, the Government of Alberta hosted 65 in-person public engagement sessions across the province. A second round of sessions began on January 21, 2025; by March 31, 2025, 54 of the 63 planned sessions had been completed, with the remaining 10 held in April and May 2025. These two-hour sessions began with a short presentation on the system refocusing effort, followed by facilitated small-group discussions. Conversations were guided by six questions exploring strengths in the local health system, existing gaps, collaboration opportunities, mechanisms for accountability, local decision-making, and supports needed for health workers, patients, caregivers, and communities.

In 2024, a total of 2,850 Albertans attended the sessions, including 192 who participated in the final 10 sessions held in April 2024. Key themes that emerged included the importance of timely access to care; clear and transparent communication; strong support for the health workforce; the unique needs of rural, remote, and Indigenous communities; the value of local decision-making; two-way accountability; and improved coordination across the system.

To maintain ongoing communication, Alberta Health distributed four e-newsletters between January and March 2025 to over 7,500 subscribers, totalling 27,935 emails. Topics included announcements and engagement opportunities related to ALA and emergency services.

### **Indigenous and Francophone Engagement**

From April to June 2024, the Government of Alberta conducted 18 Indigenous information gathering sessions — four in person and 14 virtual. The in-person sessions were hosted in locations aligned with the Medicine Wheel (north, south, east, and west regions). These sessions were facilitated with the support of the Indigenous Health Division and guided by seven culturally informed questions addressing cultural practices, health partnerships, equitable access, recruitment, accountability, and system improvements.

Feedback from First Nations, Métis, and Inuit participants highlighted significant barriers to accessing care, limited Indigenous engagement and representation, a lack of culturally safe services, insufficient health promotion and education, gaps in mental health and addiction supports, limited continuity in transitions of care, and concerns related to treaty rights and information-sharing agreements. An Indigenous-specific online feedback form was also available between April 30 and July 5, 2024.

To ensure French-speaking Albertans had equitable opportunities to participate, Alberta Health, in collaboration with the Association canadienne-française de l'Alberta, Réseau santé albertain, and the Francophone Secrétariat, hosted two French-language engagement sessions: an in-person session in Edmonton on June 17, 2024, and a virtual session on June 20, 2024. These were conducted entirely in French, with all materials provided in French. The sessions reinforced key public engagement themes and surfaced unique priorities, including improved access to health care in French, community governance, and language promotion. A French-language online feedback form was available from June 6 to July 5, 2024.

### **Online Engagement**

In addition to in-person and targeted sessions, Alberta Health provided online feedback tools in English, French, and Indigenous-specific formats. Two public feedback windows were held: November 10, 2023, to January 15, 2024, and January 23 to April 21, 2024. Over 18,000 responses were received from Albertans, with a higher representation of respondents identifying as female and aged 35 to 54 years compared with the general population. Respondents prioritized issues related to workforce development, improvements to service delivery, health system structure and governance, partnerships, integration of technology, and resource allocation. While the French and Indigenous online forms had smaller sample sizes, the themes mirrored those raised during the in-person sessions.

### **Health Workforce Engagement**

Alberta Health directly engaged health care providers through public sessions and targeted communications. Many providers participated in the in-person sessions from January to April 2024 and again between January and April 2025. Workforce feedback was continuously integrated into system planning.

The Alberta Health Change Management Program played a key role in supporting communications with staff affected by the transition to new PHAs. This included regular updates, reference documents, such as frequently asked questions and fact sheets, onboarding resources, and in-person and virtual engagement opportunities. The program also facilitated ongoing collaboration between AHS, ACA, and PCA, ensuring alignment and minimizing disruption to service delivery.

Additional engagement activities included seven regional sessions with health foundations between October 29 and December 11, 2024. These were held both virtually and in person, with 59 of Alberta's 70 health foundations participating, which includes an increasing participating rate to 84 per cent. Outreach was also conducted with medical associations, First Nations, Métis and Inuit health leaders, university researchers, and members of the PCA and ACA medical leadership. Physician-specific town halls were organized to enable direct dialogue with leadership teams.

### Outcomes and Next Steps

The feedback received through these engagement efforts was shared with the Minister of Health, the Minister of Mental Health and Addiction, and system leaders across the province. Alberta Health remains committed to demonstrating how this input is being used to guide decisions, develop policies, and shape the implementation of the refocused health care system. A comprehensive *"What We Heard"* report is available through the *Lead the Way* platform, and future updates will continue to reflect the voices of Albertans who contribute to this important work.

## 2.3 Modernize and transform the continuing care system to improve access and shift to more care in the community

*The Ministry of Health is reporting on progress in palliative and end-of-life care, as well as licensing and monitoring, to support the transformation of Alberta's continuing care system and improve access to community-based services.*

Responsibility for the stand-up of the continuing care agency was transferred to the Ministry of Seniors, Community and Social Services in fall 2024. In 2024-25, the Ministry of Health made significant strides toward strengthening and modernizing Alberta's continuing care, primary health care, and digital health systems. Alberta's government maintained its commitment to shift care to community-based settings and expand access to high-quality continuing care services.

### Palliative and End-of-Life Care

In Alberta, palliative and end-of-life care is offered in a range of settings including, but not limited to, acute care hospitals, outpatient clinics, primary care provider settings, continuing care homes (including type C- hospice), EMS, and home and community care settings.

To address the growing needs among Alberta's aging population, the Government of Alberta has prioritized investments that enhance access to palliative and end-of-life care as part of its continuing care transformation.

These investments are aimed at ensuring Albertans receive compassionate, appropriate, and dignified care at all stages of life, including end-of-life.

Licensing and Monitoring in Continuing Care

To strengthen accountability, transparency, and oversight in Alberta’s continuing care system, the Government of Alberta introduced a new legislative framework:

- On April 1, 2024, the *Continuing Care Act* and associated regulations—Continuing Care Regulation and Continuing Care (Ministerial) Regulation—came into effect. This legislative update marked a major milestone in transforming Alberta’s continuing care system.
- The Act enables Alberta Health to enhance licensing, monitoring, and compliance processes across all levels of continuing care, including home and community care, supportive living, and palliative services. These enhanced oversight mechanisms are intended to ensure quality, safety, and equitable access to services.

Additionally, ALA, expected to become operational by fall 2025, will align medical and non-medical services, expand access to continuing care, and support system accountability through integrated service delivery and performance monitoring.

The performance metric 2.b, included in the 2024-27 Business Plan, pertains to Continuing Care which has been transitioned to the Ministry of Seniors, Community and Social Services. Hence, this data will not be discussed in this report.

Unplanned medical readmissions to hospital within 30 days of discharge

|                                                                                           | 2020-21 | 2021-22 | 2022-23 | 2023-24 | 2024-25 |
|-------------------------------------------------------------------------------------------|---------|---------|---------|---------|---------|
| Percentage of patients with an unplanned medical re-admission within 30 days of discharge | 13.2%   | 12.9%   | 12.6%   | 12.7%   | 12.7%   |

Source: Alberta’s Morbidity and Ambulatory Care Abstract Reporting System (MACAR)

Note: The result is for Acute Care data submissions

The above performance measure estimates the percentage of medical patients with an unplanned readmission to hospital within 30 days of being discharged. A lower percentage means fewer patients have been readmitted to hospital within one month of discharge. A high rate of readmissions increases costs and may mean the health system is not performing as well as it could be. Although readmission may involve many factors, lower readmission rates may show that Albertans are supported by discharge planning and continuity of care. Results exclude hospital admissions for surgery, pregnancy, childbirth, mental health diseases and disorders, palliative care, and cancer therapy. The desired result is to decrease the percentage of unplanned readmissions to hospital within 30 days of discharge.

In 2024-25, the percentage of medical patients with an unplanned hospital readmission within 30 days of discharge from hospital was 12.7 per cent, identical to as that of previous years.

2.4 Strengthen and modernize Alberta’s primary health care system in partnership with Albertans and Indigenous communities by implementing innovations to ensure access to safe, timely and appropriate primary health care services in the community

A strong primary health care system is foundational for better health care for Albertans. In 2024-25, the Government of Alberta took significant steps to stabilize and strengthen primary health care so that every Albertan can access timely and high-quality care when and where they need it.

To advance this goal, Alberta continued implementing recommendations from the MAPS initiative. These actions support the province's commitment to building a more patient-centred, integrated, and equitable health care system.

In response to the MAPS Strategic Advisory Panel and the Indigenous Advisory Panel final reports, several initiatives were launched in 2024-25:

- \$57 million over three years was provided to family doctors and NPs to help manage rising patient care costs.
- A three-year grant of \$704,000 in 2024-25 supported Imagine Citizens' pilot of Care Opinion, an online platform for public feedback on health care experiences.
- A \$400,000 grant supported the continued operation and expansion of the Alberta Find a Doctor website, including family physicians and NPs not affiliated with Primary Care Networks.
- \$5 million over two years was invested in the Primary Care Fund to reduce administrative burden, improve access to primary care, and enhance team-based care.

These investments are addressing urgent pressures and helping to stabilize Alberta's primary health care system.

The Government of Alberta remains committed to improving access to culturally safe and appropriate care for First Nations, Métis and Inuit peoples. In 2024-25, \$27.6 million was allocated to advance Indigenous health equity through two major grants:

- \$7.6 million over three years funded the Navigator Program, supporting 18 organizations (up to \$450,000 each) to assist Indigenous patients in navigating the health system, reducing barriers, and improving primary care access. The funding aligns with Alberta's new Indigenous Health Division and ongoing efforts to support Indigenous-led health care solutions. Following the success of the program, the Government of Canada contributed an additional \$6.3 million over three years in funding through the Bilateral Indigenous Patient Navigator Grant Program.
- The \$20 million Indigenous Primary Health Care Innovation Fund is expanding Indigenous virtual care services through the Alberta Indigenous Virtual Care Clinic (AIVCC); increasing Indigenous-led cancer screening programs; supporting health promotion and disease prevention; along with funding infrastructure and capital projects.

To enhance provider capacity, Alberta Health made key investments in team-based care models:

- \$30 million over two years was allocated to expand team-based care in four community health centres: The Alex, CUPS Calgary Society, Jasper Place Wellness Centre, and Radius Community Health & Healing.

Recognizing the critical role of family physicians, the Government of Alberta announced a modernized physician compensation model on December 19, 2024. This model is designed to attract, retain, and support family physicians and rural generalists. In addition, \$13.8 million was allocated over two-years to PCA to identify new, and expand and scale existing innovative solutions that provide comprehensive, team-based, primary care services. The NPPCP also saw significant

uptake. This program supports NPs to operate independently. Since April 2024, 145 applications have been submitted, with 65 approved. As of March 31, 2025, 53 NPs are practising autonomously under the NPPCP, with 63,769 medically necessary services provided to 31,905 Albertans. NPPCP NPs are primarily practising urban communities, with 20 of the 53 NPs in rural communities such as Beaverlodge, Cold Lake, Two Hills, and Vermilion.

To further support rural emergency care, Alberta launched a pilot project on November 25, 2024, introducing virtual emergency physicians in five rural emergency departments (EDs): Edson, Hinton, Lac La Biche, Elk Point, and Beaverlodge. This program enables remote physicians to assess patients by video, supported by on-site nurses and paramedics. It helps prevent temporary ED closures, allowing more patients to receive local care. Patients needing in-person emergency care will still be transferred by EHS to a nearby hospital.

**Performance Indicator 2.a:**  
**Number of practicing family physicians and registered nurse practitioners in Alberta**

|                                                       | Prior Years' Results |       |       |       | 2024 Actual <sup>1</sup> |
|-------------------------------------------------------|----------------------|-------|-------|-------|--------------------------|
|                                                       | 2020                 | 2021  | 2022  | 2023  |                          |
| Number of practising family physicians (as of Dec 31) | 5,398                | 5,388 | 5,521 | 5,656 | 5,951                    |
| Number of registered NPs                              | 715                  | 819   | 844   | 887   | 1,017                    |

Source: CPSA – <https://cpsa.ca/about-cpsa/statistics/>

Note: <sup>1</sup> Historical data has been adjusted and restated for prior years to align with the fiscal year reporting periods.

The performance indicator 2.a reflects the number of registered family physicians and NPs in Alberta. Having an adequate supply of physicians and nurse practitioners would ensure that Alberta's health care system is always responsive to Albertans' health needs. The number of registered family physicians and nurse practitioners reflects the system's capacity to provide timely, accessible, and community-based care across Alberta.

The number of registered family physicians in Alberta continues to grow annually. More family physicians and NPs are now registered to practice in Alberta compared with previous years. As of December 31, 2024, there were 5,951 family physicians and 1,017 NPs registered in Alberta, which was a net gain of 295 physicians (5.2 per cent increase) and 130 NPs (14.7 per cent increase), respectively, compared with the same period last year. Expanding the family physician and registered nurse workforce will improve the efficiency and quality of primary health care delivery across the province.

**2.5 Strengthen the health care system for Albertans and health care providers through the use and integration of information technology**

The Government of Alberta is actively improving the digital health environment to provide Albertans with access to their health information and to give health care providers more complete digital patient information at the point of care. These efforts enhance care quality and support informed decision-making. Considerable investments are being made to modernize Alberta's digital health infrastructure.

As of November 2024, Connect Care was fully implemented across AHS and affiliated facilities. It currently supports 125,000 staff, physicians, and clinicians across AHS, affiliated facilities, and provincial health agencies that currently operate. Significant progress was also made in advancing the following digital tools for both patients and providers:

- As of March 31, 2025, MyHealth Records (MHR), a portal that allows Albertans to access their health information, surpassed 1.9 million registrants.
- Over 2,100 community clinics and 1,700 pharmacies were connected to Alberta Netcare, Alberta's Electronic Health Record system, improving provider access to patient information.
- Alberta Netcare enhancements included the Patient Quick View dashboard, Patient Summary Report, and docking capabilities, improving usability for more than 45,000 clinicians.
- The Community Information Integration (CII) and Central Patient Attachment Registry (CPAR) continued to expand. Over 2.7 million patients were paneled in CPAR and over 28.6 million Clinical Encounter Digests were shared through CII.
- The Electronic Referral Solution completed several key software upgrades in Phase 2. The system is progressing through a closed-loop provincial eReferral model, which remains the objective in the next reporting period.
- The ministry completed stakeholder consultations and developed a roadmap for an Electronic Medical Records (EMR) Certification Program. The initial \$500,000 phase was completed on December 15, 2024. This program will strengthen system interoperability and patient data integration across Alberta.

The ministry completed a proof of concept of AI-driven automated rules extraction and analysis. A detailed roadmap was developed for modernizing the eligibility and premiums (EAP) legacy system. Future modernization work includes implementing the EAP roadmap and further replacement of legacy mainframe systems used in the Canadian public health benefits management and claims processing domain.

Data integration through the provincial Health Information Exchange was strengthened. New clinical data sources were added to Alberta Netcare, including diagnostic imaging and reports, cardiology, pulmonary function tests, laboratory results, community encounter records, and clinic consultation reports. These additions support more comprehensive and coordinated patient care.

In 2024–25, the Government of Alberta initiated a phased implementation of Lab eOrdering. This will introduce a closed-loop electronic ordering solution to enhance the efficiency and accuracy of laboratory test requisitions. The first phase developed a minimum viable product, creating a foundation for a fully integrated electronic laboratory ordering system.

An independent review of Alberta's IT systems was completed in summer 2024. A new Digital Health Operating Model that aligns with a refocused health system is currently in development based on findings from the independent review.

Alberta Health is also in the process of standing up a Shared Services Entity (SSE) that consolidates Alberta's digital health IT and supports the new Digital Health Operating Model. The SSE consolidates digital health IT resourcing to provide services across the new Provincial Health Agencies and Health Service Delivery Organizations of the Refocused Health System. The SSE is slated to become operational in September 2025.

Alberta Health, collaboratively with PCA, is working on an initiative to enhance Alberta Netcare with access to Connect Care for mixed context community providers. A targeted rollout to a select

group of community providers (determined in partnership with the AMA) is underway and scheduled to complete by fall 2025.

**Performance Indicator 2.c:**  
**Number of Albertan registered for MyHealth Records**

|                                                            | 2020-21 | 2021-22   | 2022-23   | 2023-24   | 2024-25<br>Actual |
|------------------------------------------------------------|---------|-----------|-----------|-----------|-------------------|
| <b>Number of Albertans registered for MyHealth Records</b> | 539,800 | 1,259,500 | 1,486,300 | 1,728,500 | 1,993,500         |

**Source:** Alberta Health

As of March 31, 2025, approximately 1.99 million Albertans were registered for MHRs, an increase from around 1.73 million users at the end of March 2024. This growth reflects a steady rise in registrations over recent years. This upward trend indicates a growing adoption of digital health tools among Albertans.

**2.6 Implement strategies that ensure the sustainability of publicly funded pharmaceutical benefits**

The Government of Alberta remains committed to addressing gaps in drug coverage while ensuring that drug benefits programs are sustainable and meet the needs of Albertans. Prescription drugs help manage illnesses, improve quality of life, and, in some cases, preclude the need for more extensive treatments. The province continued to focus on ensuring value for money spent on these benefits through improving existing drug benefit programs and adding innovative, effective therapies.

In December 2024, Alberta signed a bilateral agreement with the federal government to receive approximately \$54 million annually over three years. This funding supports access to new and emerging drugs for rare diseases and expands screening and diagnostics, improving health outcomes for Albertans. This funding will provide Albertans with earlier access to a common set of rare disease drugs, and additional funding for rare disease drugs that are currently offered in Alberta.

The first new drugs funded under the agreement include Poteligeo for Sézary syndrome, Oxlumo for hyperoxaluria type 1, and Epkinly for large B-cell lymphoma treatments. It is expected that approximately 30 Albertans will be eligible annually for these new treatments. Additional drugs added include Koselugo for neurofibromatosis type 1, Yescarta for follicular lymphoma, and Welireg for Von Hippel-Lindau disease.

The Government of Alberta also reviewed the federal government’s An Act Respecting Pharmacare (*National Pharmacare Act*), which received royal assent on October 10, 2024. The Minister of Health notified the federal government that Alberta will monitor developments and outcomes of the federal government’s discussions with other provinces and territories on the National Pharmacare Act. Alberta requested their fair share of federal funding - without conditions - to strengthen its existing pharmacare programs and expand coverage.

In 2024, Alberta Health worked with the Alberta Pharmacists' Association (RxA) and identified that pharmacy expenditures were projected to exceed the 2024-25 budgeted threshold. To address this, cost-saving measures were introduced on November 1, 2024:

- The initial assessment fee for a Comprehensive Annual Care Plan (CACP) was reduced to \$70 from \$100.
- The maximum allowed annual follow-up visits for the CACP and Standard Medication Management Assessment care plans were reduced from 12 to 4.

These changes align with practices in other jurisdictions and actual service use in Alberta. While pharmacy fees for some clinical pharmacy services, specifically care plans offered by pharmacists to Albertans, were adjusted, Albertans continue to have access to assessments and necessary follow-ups. Core pharmacy services – including dispensing, injections, prescribing, and prescription renewals – were not affected.

The policy changes on November 1, 2024, have resulted in approximately \$19 million in cost-offsets for 2024-25. Although fee changes provided some financial relief, ongoing expenditures are still exceeding budget projections due to additional pressures such as increased demand for pharmacy services and population growth. Alberta Health will continue consultations with the RxA into 2025, aiming to identify further sustainability measures and develop a new pharmacy framework for 2026 onwards.

The Government of Alberta continued its efforts to reduce prescription drug costs and expanded access to life-saving drugs. On November 5, 2024, Alberta expanded access to Trikafta to include individuals with rare mutations of cystic fibrosis who could benefit from treatment. An estimated 70 additional patients will now be able to access this therapy.

In January 2024, COVID-19 vaccine fees were reduced from \$25 per dose to \$17 per dose. This was further reduced to \$13 per dose on April 1, 2024. This aligns with the fee for administration of other publicly funded vaccines. The fee for COVID assessment was removed in October 2023.

Access to additional vaccines was expanded. The publicly funded list now includes tetanus, diphtheria and acellular pertussis (TDaP) and pneumococcal and respiratory syncytial virus (RSV) vaccines. To support outreach programs, the administration fee for pharmacists delivering vaccines to vulnerable populations in outreach facilities was increased from \$13 to \$17 as part of the Alberta Immunization Outreach Program.

Alberta also advanced its biosimilars initiative with the addition of two categories: Ustekinumab and Denosumab. These additions are expected to save \$10 million annually. Alberta also benefited from national negotiations through the pan-Canadian Pharmaceutical Alliance (pCPA). In 2023-24, public drug programs saved an estimated \$4.63 billion, with cumulative savings over \$24 billion since inception in 2010. Alberta's portion of these savings is estimated to be \$463 million in 2023-2024 and \$2.4 billion since 2010 based on our portion of public drug spending.

**Outcome Three: The health and well-being of all Albertans is protected, supported and improved, and health inequities among underserved populations, including Indigenous Peoples, are reduced**

The ministry continues to work with its partners to address health inequities for underserved populations, empower patients, promote health and wellness and give Albertans choice when receiving care from health care professionals.

**Key Objectives****3.1 Safeguard Albertans from communicable diseases, such as food-borne illnesses, vaccine preventable diseases (e.g., measles, influenza), respiratory illnesses, and sexually transmitted and blood borne infections that can cause severe health outcomes through prevention and health promotion measures and policies.**

Alberta Health remains committed to safeguarding the health and well-being of Albertans through proactive measures to prevent the spread of several communicable diseases, such as influenza, measles, and sexually transmitted and blood borne infections (STBBIs). Immunization programs remain one of the core public health functions that continues to be a key strategy in preventing disease and reducing strain on the acute care system.

In 2024-25, Alberta allocated approximately \$148 million to support the continued delivery of provincially funded vaccines, including routine infant, preschool, and school programs. DTaP-IPV-Hib dose four immunization coverage rates by children aged two decreased from 71 per cent in 2023 to 69 per cent in 2024. Measles, mumps, rubella immunization coverage rates by age two decreased from 82 per cent in 2023 to 80 per cent in 2024. These immunization rates are both lower than the national target of 95 per cent for these vaccines (table below).

Childhood immunization rates in Alberta have continued to decline over the last five years, which led to an increase in vaccine-preventable disease cases and outbreaks (e.g., ongoing pertussis circulation and newly identified measles cases across the province). To improve these rates, AHS implemented targeted initiatives to address vaccine hesitancy. For example, AHS is recalling children for overdue immunizations, monitoring wait times, and developing a provincial appointment standard to address delays. The AHS Immunization Priority Working Group advanced its five-year workplan, including the development of:

- Provincial immunization orientation modules, including a vaccine hesitancy module to assist immunizers in conversations with vaccine hesitant individuals
- Immunization resources in multiple languages

### Children by age two immunized for diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b) (DTaP-IPV-Hib) and for measles, mumps, rubella (MMR)

|                                                                                                                                                                      | 2020 | 2021 | 2022 | 2023 | 2024 |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------|------|------|------|------|------|
| <b>Children by age two immunized with four doses of diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b) (DTaP-IPV-Hib) vaccine</b> | 76%  | 73%  | 71%  | 71%  | 69%  |
| <b>Children by age two immunized with one dose of MMR vaccine</b>                                                                                                    | 87%  | 83%  | 82%  | 82%  | 80%  |

**Source:** AHCIP Quarterly Population Registries and Immunization/Adverse Reactions to Immunization (Imm/ARI)

This performance measure reports the percentage of children in Alberta who, by two years of age, have received the required immunization for the following vaccine preventable diseases: diphtheria, tetanus, pertussis (whooping cough), Haemophilus influenza type b, measles, mumps and rubella. A high rate of immunization reduces the incidence of these diseases and serves to control outbreaks.

Measles cases have surged globally since 2024, partly due to reduced vaccinations during the COVID-19 pandemic. In response to outbreaks, including one in Canada, Alberta Health is updating guidelines and developing a response plan with partners, including AHS, Indigenous Services Canada, and Public Health Agency of Canada (PHAC), including PHAC's measles case and contact management guidance document. Two cases of travel-acquired measles were reported in Alberta in 2024. As of March 31, 2025, 23 confirmed measles cases were reported, with most of the confirmed cases being in unimmunized individuals. Measles activity is across multiple AHS zones, with two cases linked to a multi-jurisdictional outbreak. The source cases in each zone were primarily travel-acquired.

A pertussis outbreak was declared in January 2023 in the South Zone and outbreaks were subsequently reported across all zones in the province. A Provincial Pertussis Coordinating Committee (PPCC) was convened in August 2023 to May 2024 to respond to the increase in reported pertussis cases to limit further spread. The PPCC included representatives from Alberta Health, AHS, Indigenous Services Canada, and the Alberta Provincial Laboratory. The last associated zone pertussis outbreak was closed in July 2024.

The provincial incidence rate for pertussis increased from 0.3 (2022) to 19.1 (2023) per 100,000 population, then declined to 12.3 (2024) per 100,000 population. Between January 1, 2023, and March 31, 2025, a total of 1,608 cases were reported across all zones, and most cases (52.1 per cent) involved under-immunized or unimmunized children under age 10. During this time, one pertussis-related death was reported in 2024.

On November 27, 2024, Alberta Health released its One Health Antimicrobial Resistance (AMR) Framework for Action, backed by \$5 million in funding and a public web page to promote stakeholder engagement. Alberta Health is now implementing the AMR Framework using a phased approach in collaboration with Agriculture and Irrigation and Environment and Protected Areas. Funding will support antimicrobial stewardship in primary, acute, and veterinary care.

The Edmonton Zone *shigella* outbreak was declared over in March 2025. Alberta Health continued to participate on the Shigella Outbreak Task Force, alongside AHS, shelters, inner-city agencies, the

City of Edmonton, local family physicians, and APL, to coordinate resources and discuss options to limit the spread of *shigella*. Since the start of the outbreak, there were 448 cases identified, with 309 hospitalizations.

The PHAC, in collaboration with provincial and territorial partners, has incorporated culture-independent diagnostic tests (CIDTs) into case definitions for some enteric diseases as part of routine surveillance and reporting. CIDTs rapidly detect organisms such as shigellosis and Shiga toxin-producing *E. coli* for timely case and contact management. Alberta Health and APL helped develop these updated national case definitions that were published by the Government of Canada in January 2024. The work is being continued with the federal government to review and implement the updated case definitions, while also updating the provincial enteric case definitions and disease management guidelines.

Alberta has experienced a rapid increase in rates of STBBIs over the last decade like several jurisdictions in Canada and around the world. In 2022, rates reached an all-time high for the current syphilis outbreak in Alberta, where 73.5 infectious syphilis cases (per 100,000 population) and 174.4 congenital syphilis cases (per 100,000 live births) were reported. Since then, rates have been decreasing but remain high overall.

**Performance Indicator 3.a:**  
**Rate of infectious syphilis per 100,000 population and the rate of congenital syphilis per 100,000 live births in Alberta**

|                                                      | 2020  | 2021  | 2022  | 2023  | 2024 |
|------------------------------------------------------|-------|-------|-------|-------|------|
| <b>Infectious syphilis (per 100,000 population)</b>  | 57.2  | 73.3  | 73.5  | 58.0  | 43.9 |
| <b>Congenital syphilis (per 100,000 live births)</b> | 116.8 | 153.7 | 174.4 | 121.6 | 98.4 |

**Source:** Communicable Disease & Outbreak Management (CDOM) - Communicable Disease Reporting System (CDRS), AHCIP Adjusted Mid-Year Population Registry Files, Alberta Health Postal Code Translator File (PCTF), and Vital Statistics Live Births

In 2024, there were 43.9 infectious syphilis cases (per 100,00 population) and 98.4 congenital syphilis cases (per 100,000 live births) reported. The goal in Alberta is to have zero congenital syphilis cases. Despite these decreases, Alberta’s current rates of human immunodeficiency virus (HIV), chlamydia, and gonorrhea have increased by 66.2 per cent, 0.8 per cent, and 4.7 per cent respectively compared with 2022, highlighting the continued need for public health vigilance and intervention.

The decrease in infectious syphilis rates reflects a multi-pronged approach to effectively address the outbreak including:

- Syphilis point-of-care testing
- Dedicated resources for partner notification, contact tracing, and outreach teams
- Targeted education, testing and treatment programs

Despite an improvement from the previous year, the rates of syphilis and congenital syphilis are still too high and continue to represent a significant public health concern for Alberta.

From April 1, 2024, to September 30, 2024, organizations providing STBBI prevention and wrap-around supports administered over 13,000 STBBI tests and provided treatment to over 468 individuals for a sexually transmitted infection. These organizations provided intensive support to over 459 people living with an STBBI, including assistance with service navigation and delivered over 100 education sessions to health care professionals and service providers across Alberta.

AHS continues to work on initiatives to increase syphilis awareness, which include a provincial webinar for primary care providers: *Current State of Syphilis: The Critical Role of Primary Care*. Alberta Health continues to participate in the Syphilis Response Committees at the provincial and federal levels to address the ongoing syphilis outbreak. A total of \$3.2 million is provided annually for syphilis specific projects, including \$2 million provided to eight organizations for Syphilis Outbreak Action Response (SOAR). In 2024-2025 the SOAR projects:

- Held over 60 syphilis awareness events, reaching over 3,500 individuals
- Administered over 3,000 STBBI tests
- Provided syphilis treatments to over 120 people
- Developed education materials, including accredited training and Indigenous-specific resources

The Alberta Outreach Program started the week of September 30, 2024, to immunize those at highest risk of severe outcomes from influenza and COVID-19. The program focuses on providing immunization services to those at highest risk of severe outcomes, hospitalization, and death due to vaccine preventable diseases, such as, influenza and COVID-19, as well as invasive pneumococcal disease, and RSV.

The 2024-25 Influenza and COVID-19 Immunization Programs for the general public began on October 15, 2024.

As of March 31, 2025:

- 21 per cent of all Albertans had received the influenza vaccine
- Highest uptake: 61 per cent in those 75 and older
- 13.7 per cent had received a COVID-19 vaccine
- Highest uptake: 54.4 per cent in those 75 and older

Albertans accessed these vaccines at more than 1,700 sites across the province, supported by the Alberta Vaccine Booking System.

### **3.2 Modernize and strengthen public health legislation to better respond to future public health emergencies.**

The Government of Alberta remains committed to supporting all Albertans by continuing to respond promptly, flexibly, and collaboratively to future public health emergencies across all areas of government. In 2024-25, Alberta Health has focused on strengthening the Alberta workforce and relationships with the AMA to achieve this goal.

Alberta Health provided \$24 million to the AMA to administer a Panel Management Support Program (PMSP) to help offset costs of maintaining and growing a panel of patients. The PMSP is a three-year initiative designed to increase access for Albertans to insured medical services.

Alberta Health provided \$92 million to the AMA to administer a Transitional Funding Program to provide physicians with financial support for the cost of eligible expenses related to panel

management and practice improvement activities. This grant funding was designed to increase access for Albertans to insured medical services.

Alberta Health collaborated with the AMA to develop a new PCPCM to compensate Alberta's family medicine and rural generalist physicians. The PCPCM was implemented on April 1, 2025, and reaffirms Alberta's family doctors as among the best compensated and most patient-focused in the country. The model is structured to:

- Encourage physicians to grow the number of patients they care for
- Promote encouragement of full-time practice
- Provide incentives to physicians maintaining high panel numbers, after-hours care, use of improved technology to enhance patient care, and team-based care

These efforts aim to strengthen Alberta's primary care system and help reduce pressure on other parts of the health care system.

### **3.3 Improve access for underserved populations including First Nations, Métis, and Inuit Peoples to high-quality and culturally safe health care services that support improved health outcomes.**

The Government of Alberta continues to prioritize the health care needs of First Nations, Métis, and Inuit peoples residing in Alberta. Alberta Health works with health and community partners including First Nations and Métis leaders and the Government of Canada to ensure that health services are accessible and culturally appropriate. This commitment entails working with Indigenous leaders, the Government of Canada, and other partners in a way that respects the unique and complex needs of these communities. Part of this commitment includes several initiatives underway to improve health outcomes.

The Indigenous Primary Health Care Implementation Panel developed implementation strategies, including an implementation road map for the 22 recommendations outlined in the March 31, 2023, final report to the former Minister of Health on improving the primary health care system for First Nations, Métis, and Inuit patients in Alberta. On April 19, 2024, *The Way Forward* document was publicly released, including the panel's implementation roadmap, which outlines the sequencing and timelines for implementing the recommendations. To date, Indigenous MAPS continues to make progress on the recommendations. *The Way Forward* is a living document to be updated over time as implementation moves forward and the provincial health system evolves.

Although the Indigenous Primary Health Care Implementation Panel's mandate formally concluded on December 31, 2024, *The Way Forward* document continues to ensure a more inclusive, culturally safe health care system for First Nations, Métis, and Inuit communities in Alberta. Ongoing updates to the document will help improve health outcomes with the design of new health care policies and programs informed by direct input from these communities, leading to more culturally appropriate, accessible, and effective care.

Administered by Alberta Health under the Indigenous MAPS stream, the Indigenous Primary Health Care Innovation Fund (Innovation Fund) aims to enhance access to culturally safe and accessible primary health care for First Nations, Métis, and Inuit peoples. The Innovation Fund improves health outcomes by fostering better engagement, increased adherence to treatment, reduced health disparities, and ensuring that care respects cultural needs and knowledge.

The Innovation Fund offers two funding streams: Tier One provides up to \$2 million for community capacity-building and testing promising practices, while Tier Two offers up to \$3 million to scale or expand Indigenous-led programs, initiatives, and infrastructure. In the first round, two grant

recipients were selected to receive funding: the First Nations Technical Services Advisory Group Inc., expanding the AIVCC, and the Métis Nation of Alberta, enhancing their Medical Travel Program to improve health access for Métis peoples. The third and final round of the Innovation Fund closed on November 15, 2024.

The Innovation Fund was awarded to the First Nations Technical Services Advisory Group Inc. This grant will support the existing core clinic and mental health teams hire staff and develop resources to better meet growing demands and expand cancer screening. The fund helps to improve access to primary and mental health care for First Nations, Métis, and Inuit individuals, especially in remote or underserved areas. Expanding cancer screening programs and increasing mental health support will help with early detection, prevention, and better management of health conditions, ultimately leading to improved overall health for these communities.

The Indigenous Patient Navigator Program provides financial support to recruit and retain navigators who assist First Nations, Métis, and Inuit patients, communities, and organizations in accessing primary health care services. The \$7.6 million over three years provided by the program improves care coordination, reduces barriers, and enhances health outcomes by ensuring personalized support, timely access to health care services, and better management of diseases or health conditions. Following the success of this program, the Government of Canada contributed an additional \$4 million to launch a second call of navigator program funding through the Bilateral Indigenous Patient Navigator Grant Program.

Alberta Health approved the Indigenous Anti-Racism in Health Care Strategy to address perceived racism in Alberta's health care system. To help shape the strategy, the Indigenous Health Division hosted 28 virtual sessions and 11 in-person sessions, in addition to an online feedback form in fall 2024. The sessions aimed to hear the lived experiences and perspectives of First Nations, Métis, and Inuit peoples, and health system partners. The anti-racism information gathering sessions fostered collaboration between these communities, health care providers, and key organizations to create a more culturally sensitive and inclusive health care system. The feedback gathered will help shape the Anti-Racism in Health Care Strategy, ensuring it addresses the specific needs and concerns of communities, ultimately improving health care delivery and trust. The anti-racism information gathering sessions will help contribute to improved health outcomes by ensuring that the action plan is tailored to the unique needs of these communities, promoting a health care system that is more culturally sensitive and responsive. The ministry continues to uphold its commitment to Jordan's Principle by ensuring First Nations children receive timely access to publicly funded health, education, and social services without denials, delays, or disruptions due to jurisdictional issues tied to their First Nations status.

In early January 2025, the Minister of Health met with Health Canada, ISC, and representatives from various Indigenous communities in Edmonton, including the Métis Nation of Alberta, Metis Settlements General Council, Treaty 6 and Treaty 8 First Nations, the Blackfoot Confederacy, and the Stoney Nakoda Tsuut'ina Tribal Council. These meetings are crucial for addressing key priorities and concerns of communities. The discussions aimed to improve access to essential health services and ensure that policies better meet the unique needs of First Nation, Métis, and Inuit communities.

### **3.4 Prevent injuries and manage chronic diseases and conditions through policy development, health and wellness promotion, improved access to screening, and initiatives that facilitate wellbeing in healthy environments.**

The Government of Alberta is leading policies and initiatives to improve the well-being of Albertans. In 2024-25, this included areas such as maternal, infant and early childhood development; injury prevention; and the advancement of population wellness and health equity.

Alberta Health and AHS continue to prioritize access to reproductive health and maternity services for Indigenous, vulnerable, and rural populations. *Budget 2024* provided over \$2 million in annual funding to help vulnerable mothers and their babies access prenatal and postnatal care and social supports. Alberta Health has also committed \$20 million in funding to the Alberta Women's Health Foundation and the Calgary Health Foundation to support research on women's health matters. The funding for both foundations will support the implementation and evaluation of clinical programs including a rapid access clinic as well as support research of women's health conditions.

In 2024-25, the government focused on ensuring safe and reliable drinking water for Albertans by working with national groups and Environment and Protected Areas to implement the Canadian Drinking Water guidelines. The Alberta government also supported and funded a provincial educational program and provided water testing for well water users, helping them manage their water resources safely and effectively.

Under the Alberta Safe Beach protocol, a prioritized list of recreational water sites were analyzed for water quality in support of healthy environments and prevention of waterborne illness. This contributes to the Ministry's objective to promote health and wellness and prevent chronic disease through initiatives that facilitate wellbeing in safe environments. An estimated 488 cyanobacterial bloom toxicity (microcystin) tests, 2,403 Enterococcus (fecal indicator) tests, and 1,070 speciation of fecal source (i.e. human vs ruminant) tests were completed. Informed by these results, 23 fecal contamination and 38 cyanobacteria advisories were issued to safeguard public health and prevent waterborne illnesses. A Waterborne Pathogen Monitoring Grant was used to investigate new recreational water testing methods and has concluded as of March 31, 2025.

The Lifesaving Society's Alberta Safe Beach Drowning Reduction Project targeted waterfront operators and the public, focusing on standardized water safety signage, public education, and promoting safe aquatic environments. The project focused on 15 provincially identified high-risk locations to enhance aquatic safety for Albertans. The project has concluded and came in underbudget with an approximate total cost of \$25,300 (against approximately \$28,200).

Alberta Health facilitated the management of chronic diseases and promotion of health and wellness through the administration of grants that contribute to building healthier communities across the province. These grants supported community-based programs focused on healthy eating, active living, physical literacy, recreational programming, mental wellness, and social connection while also empowering health care providers to prescribe physical activity. Alberta Health provided \$960,000 in funding to promote physical activity, healthy eating, and active living for the management of chronic disease and promotion of health and wellness. It also provided the Injury Prevention Centre \$800,000 per year for three years through programs and campaigns that reduce the burden of injuries on individuals and the health care system, enhancing the system's capacity to respond to other needs.

Government continues to support cancer screening programs to return screening rates to pre-pandemic levels. Screening programs continue to be supported, such as the Alberta Lung Cancer screening pilot project, the Colorectal Cancer Screening Program, and the Breast Cancer Screening program, which support the early detection and treatment of cancer. The Alberta Newborn Screening Program continues to be supported. The program screens infants for conditions that can be treated early, when treatment can benefit the most, with the primary goal of reducing morbidity and mortality among infants born in Alberta.

In response to the September 4, 2023, E. coli outbreak in 11 licensed childcare facilities in Calgary, the Government of Alberta appointed the Food Safety and Licensed Facility-Based Child Care Review Panel. The panel identified opportunities for strengthening the legislation and inspection processes that govern food safety in kitchens for licensed childcare facilities. In June 2024, the

review panel submitted its final report of 12 main recommendations and 27 sub-recommendations. Implementation of the review panel's recommendations began in July 2024. The impacts of this work include extensive review and anticipated changes to ensure food safety is practiced and enforced in licensed childcare facilities in our province. This will ensure that children and vulnerable Albertans will experience improved safety when they are served food. Although the review panel started its review in November 2023, the expenditures on this initiative were applied in the 2024-2025 budget. Close to \$144,000 paid for activities related to the review panel's operations, including about \$59,000 in honoraria, \$3,200 in travel expenses, \$50,000 in consulting fees to conduct engagement with families, and about \$21,600 for an independent investigation on the response and outcomes of the outbreak.

The Government of Alberta is committed to improving access to medical technology that helps Albertans better manage their health. On December 16, 2024, Alberta Health expanded access to continuous glucose monitors to Albertans over the age of 18 enrolled in an Alberta government sponsored supplementary benefit program, who are living with diabetes and require certain insulin therapies. This fulfils a key recommendation from the Diabetes Working Group, which provided advice to government on improving Alberta's diabetes care pathway. Approximately 23,500 Albertans aged 18 years and older will now be eligible for continuous glucose monitor coverage. Funding for the first two years of this initiative is provided through the Canada-Alberta agreement to improve health care.

## Performance Measure and Indicator Methodology

**Performance Measure 1.a:**

**Emergency department wait times: 90<sup>th</sup> percentile time to initial physician assessment in the 16 largest sites (hours)**

**Methodology**

This measure represents the length of time (in hours) between patient triage and the time of the initial physician assessment.

The 90th percentile response interval is calculated for all EMS events triaged by the EMS Communications Centre as ‘delta’ or ‘echo’, the most emergent or potentially life-threatening calls.

**Source**

AHS – Emergency Department Information Systems and Emergency Department Visit Data Repository AHS.

**Performance Measure 1.b:**

**EMS 90th percentile response times (minutes) for the most urgent (life threatening) calls**

**Methodology**

Response time is calculated based on events triaged in the EMS Communication Centre, using an internationally accredited EMS call taking system, to be emergency or potentially life threatening at the time of triage by the EMS Communications Centre. This includes events categorized as ‘delta’ and ‘echo.’

- Inclusion: Event level response interval for all requests for EMS response triaged as a ‘delta’ or ‘echo’ by geographical location of call (Metro/Urban, Communities > 3,000, Rural or Remote).
- Exclusions:
  - Events triaged by the EMS Communications Centre as an alpha, bravo or charlie response.
  - Events or requests identified as inter-facility transfers.
  - Events with missing timestamps (such as arrive on scene) related to metric calculation.

The 90th percentile response interval is calculated for all EMS events triaged by the EMS Communications Centre as ‘delta’ or ‘echo’, the most emergent or potentially life-threatening calls.

**Source**

EMS Computer Aided Dispatch (CAD) data captured with EMS Communications Centres, Alberta Health

**Performance Indicator 2.a:****Number of practising family physicians and registered nurse practitioners in Alberta****Methodology**

Family physician counts are as of December 31 of each year as reported in the Q4 Quarterly Updates by the College of Physicians and Surgeons of Alberta (CPSA). CPSA's year end is December 31, and it reports the number of physicians every quarter – as of March 31, June 30, September 30, and December 31.

Note that physician registration counts are cyclical throughout the year. Counts decrease in the first quarter of the calendar year between January – March, as physicians who do not renew their registration as of December of the previous year are removed. Numbers increase throughout the year, particularly in the third quarter between July – September when the majority of new physicians register after completing residency programs. To reflect the count as at fiscal year end, the registered family physician numbers have been updated using CPSA data as of March 31, 2025.

**Source**

College of Physicians and Surgeons of Alberta –

**Performance Indicator 2.c:****Number of Albertans registered for MyHealth Records****Methodology**

Number of Albertans who have registered for a MyHealth Records Accounts at the end of each day at 2359EST.

**Source**

Alberta Health

**Performance Indicator 3.a:****Rate of infectious syphilis per 100,000 population and the rate of congenital syphilis per 100,000 live births in Alberta****Methodology**

The infectious syphilis rates are calculated by dividing the number of cases (i.e. numerator) by the number of persons (i.e. denominator). All rates are based on Alberta mid-year population for the appropriate year and reported per 100,000 population.

The numerator is the number of congenital syphilis cases, derived from the Communicable Disease & Outbreak Management (CDOM) database for each calendar year. Confirmed and probable cases are grouped together. Early congenital syphilis cases and syphilitic stillbirths are provided whilst

late congenital syphilis cases are excluded. STI information, with links to case definitions, are available online: <https://www.alberta.ca/notifiable-diseaseguidelines>. Non-Alberta residents are not included in the numerator. Whenever possible, disease diagnosis dates were used to determine the year of infection. If the diagnosis date was not available, then the collection date was used. The denominator is the number of live births in Alberta. The congenital syphilis rate is reported as the number of congenital syphilis cases diagnosed in Alberta per 100,000 live births.

Source

Communicable Disease & Outbreak Management (CDOM) - Communicable Disease Reporting System (CDRS), AHCIP Adjusted Mid-Year Population Registry Files, Alberta Health Postal Code Translator File (PCTF), and Vital Statistics Live Births

Additional Performance Metrics not included in Health Business Plan 2024-27

Performance Indicator (Outcome 1, Key Objective 1.1):  
Percentage of surgical procedures that met national wait time benchmarks

Methodology

Ready-to-treat (RTT) wait time (in days) is calculated for each relevant record meeting specified inclusion/exclusion criteria.

- Inclusion Criteria: All elective hip, knee replacement and cataract surgeries based on surgery procedure catalogue descriptions. All urgency levels.
- Exclusion Criteria: persons who received emergency surgical care. Cases with an invalid RTT date. When cataract surgery is required for both of a patient’s eyes, only the wait time for surgery on the first eye is included in the wait time calculations.

RTT Date = The date that the surgeon determines the patient is ready for the surgical intervention. The RTT Date excludes patient delays or voluntary waits.

Treatment Date = The date the treatment (surgical procedure) took place.

RTT Wait time = Treatment (surgical procedure) Date minus RTT Date

National RTT wait time benchmarks:

- Hip replacement national benchmark is 182 days
- Knee replacement national benchmark is 182 days
- Cataract surgery national benchmark (for first eye) is 112 days.

Percentage =

Number of cases completed with a RTT wait time less than or equal to the national RTT wait time benchmark

Number of completed cases with valid RTT dates

X 100

**Source**

Local Operating Room (OR) Information Systems, AHS, and OR Data Repository, AHS

**Performance Indicator (Outcome 1, Key Objective 1.1):**  
**Percentage of scheduled surgeries performed in CSFs**

**Methodology**

Main operating room activity is used as representative of “surgeries” in the province. Based on identified main operating cases, those which are non-emergency are identified and the percentage where the site of delivery was a CSF is reported.

- Numerator: Non-Emergency MAIN OR cases completed in a CSF.
- Denominator: Non-Emergency MAIN OR cases.

$$\text{Percentage} = \frac{\text{Number of surgeries performed in CSFs under contract with AHS}}{\text{Total number of surgeries in CSFs and hospitals from April 1 to Mar 31}} \times 100$$

**Source**

Main Operating Room (OR) Information Systems, OR Data Repository, AHS

**Performance Indicator (Outcome 1, Key Objective 1.1):**  
**Annual number of surgeries provided**

**Methodology**

$$\text{Annual number of surgeries provided} = \frac{\text{Annual Count}}{\text{Volume}}$$

**Source**

Local Operating Room (OR) Information Systems, AHS, and OR Data Repository, AHS.

**Performance Indicator (Outcome 1, Key Objective 1.1):**  
**Percentage of patients seen within established targets for CT scan and MRI scan**

| Methodology                                                                                                                                                                                               |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Data reflects performance as of the last day of the fiscal year (March 31, 2025).                                                                                                                         |
| Target Met Percentage includes all patient types: outpatients, emergency, and inpatients.                                                                                                                 |
| Target: Refers to the maximum allowable wait time (in days or hours) for an imaging exam, as defined by provincial benchmarks. Targets vary by imaging modality, clinical priority, and patient category. |
| Source                                                                                                                                                                                                    |
| Path to Care DI Dashboard - OWL Production (1.0): Path to Care DI Access Owl - AHS Tableau Server                                                                                                         |

**Performance Indicator (Outcome 2, Key Objective 2.3):**  
**Unplanned medical readmissions to hospital within 30 days of discharge**

| Methodology                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Numerator: Total number of medical patients with unplanned readmission to hospital within 30 days of discharge.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| <ul style="list-style-type: none"><li>Inclusion Criteria: Residents of Alberta covered by the Alberta Health Care Insurance Plan. Admission day of subsequent readmission is less than or equal to 30 days of initial discharge date from an acute care hospital.</li><li>Exclusion Criteria: Transfers (admitted within 6 hours of discharge from another hospital, or 6-12 hours after transfer from or to an acute care facility); newborn, still birth, cadaver admissions; non-acute care admissions; pediatric (less than 20 years of age), surgery, obstetrics, palliative care, mental health admissions; hospital admissions for cancer therapy; non-urgent, planned admissions.</li></ul> |
| Denominator: Total number of medical patients discharged.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
| <ul style="list-style-type: none"><li>Inclusion Criteria: Residents of Alberta covered by the Alberta Health Care Insurance Plan; discharge from an acute care hospital, within a reporting period.</li><li>Exclusion Criteria: Admissions where patients died in hospital; admissions from March 2 to March 31; transfers (admitted within 6 hours of discharge from another hospital, or 6-12 hours after transfer from or to an acute care facility); newborn, still birth, cadaver admissions; non-acute care admissions; pediatric (less than 20 years of age), surgery, obstetrics, palliative care, mental health admission.</li></ul>                                                       |

|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |                                                                                                                                                                                                  |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Hospital Readmission Percentage (unadjusted) =                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | $\frac{\text{Number of hospital inpatient discharges where a patient was readmitted to hospital within 30 days of index discharge}}{\text{Number of hospital inpatients discharged}} \times 100$ |
| <p>Risk Adjustment: Accounts for differences in patient characteristics that may vary over years. Based on the list of risk factors (e.g., age, sex, case mixing grouping, Charlson comorbidity score) published by the Canadian Institute for Health Information (CIHI), and the Alberta-built, provincial specific logistic regression model using data from the past five years to estimate the Alberta average and expected readmissions based on patients' risk profile. The risk-adjusted rate is calculated using the following formula:</p> |                                                                                                                                                                                                  |
| Risk-adjusted rate =                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | $\frac{\text{Observed cases}}{\text{Expected cases}} \times \text{Alberta rate}$                                                                                                                 |
| <p>The methods used by Alberta and CIHI differ only in that Alberta applies the Alberta Rate instead of the Canada Rate. Using the Alberta rate allows for timely reporting and calculation of results for sub-populations. The limitation of this method is that the estimates produced in Alberta are not comparable with other provinces produced by CIHI. Interprovincial comparisons are still possible by using the CIHI estimates only (i.e., using the CIHI produced estimate for Alberta, and not the Alberta produced estimate).</p>      |                                                                                                                                                                                                  |
| <b>Source</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                  |
| <p>Alberta's Morbidity and Ambulatory Care Abstract Reporting System (MACAR)</p> <p>Note: Data in MACAR is then provisioned to the ministry's Business Intelligence warehouse. It is also submitted to CIHI's Discharge Abstract Database (DAD) and National Ambulatory Care Reporting System (NACRS) datasets.</p>                                                                                                                                                                                                                                 |                                                                                                                                                                                                  |

### Performance Indicator (Outcome 3, Key Objective 3.1):

**Children by age two immunized for diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b) (DTaP-IPV-Hib) and for measles, mumps, rubella**

### Methodology

The AHCIP Quarterly Population Registries (AHCIP) provides the basis for the cohorts. Individuals who are residents of Alberta (including First Nations people) are followed until the earliest of (i) immunized (ii) leaving Alberta, (iii) death, or (iv) end-of-study period. Immunization records from Imm/ARI are linked to this cohort using Unique Lifetime Identifier. Residents of Lloydminster are removed from the analysis, since those residents are captured as part of Saskatchewan Health.

The coverage rates are based on a birth cohort and reported in percentage by ages of two. To be considered immunized, a child must have received their age-appropriate dose(s) by their birthday (e.g., if a child received a dose after their 2nd birthday, they are not counted as having received a dose by age two). In terms of birth cohort, the 2024 coverage rates relate to the 2022 birth cohort (i.e. those who were born in 2022) that turned age two in 2024.

Calculation: Childhood immunization coverage is calculated using a survival analysis (time-to-immunization) method based on the specified population-based birth cohort. The analysis measures the probability that the child will receive required vaccines by age two.

Source

AHCIP Quarterly Population Registries and Immunization/Adverse Reactions to Immunization (Imm/ARI).

# Financial Information

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Ministry of Health

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Financial Information

Year Ended March 31, 2025

## Reporting Entity and Method Consolidation

The consolidated ministry financial information is prepared in accordance with government's stated accounting policies, which are based on Canadian Public Sector Accounting Standards.

The reporting entity is the Ministry of Health for which the Minister of Health is accountable. The accounts of the Ministry of Health, which includes the Department of Health, Alberta Health Services (AHS), Primary Care Alberta (PCA), and Health Quality Council of Alberta (HQCA), are consolidated using the line-by-line method.

The audited financial statements of AHS and HQCA are included in the annual report. Accounts of entities consolidated by AHS are listed in note 2a (i) and (ii) of AHS consolidated financial statements. PCA financial statements were not audited and published due to their short operational period, being operational effective February 1, 2025. Acute Care Alberta (ACA), established as a legal entity effective February 1, 2025, will be operational April 1, 2025.

Under the line-by-line method, accounting policies of the consolidated entities are adjusted to conform to those of the government and the results of each line item in their financial statements (revenue, expense, assets, and liabilities) are included in government's results. Revenue and expense, capital, investing and financing transactions, and related asset and liability balances between the consolidated entities have been eliminated. Restated budget and comparable reflect appropriation administration responsibilities for Continuing Care programs transferred to Ministry of Seniors, Community and Social Services. Additionally, mental health and addiction services were transferred from AHS to Recovery Alberta, an entity controlled and consolidated by Ministry of Mental Health and Addiction.

A list of the individual entities making up the ministry are shown on the "Management's Responsibility for Reporting" statement included in this annual report.

## Ministry Financial Highlights

### Consolidated Statement of Revenues and Expenses (unaudited)

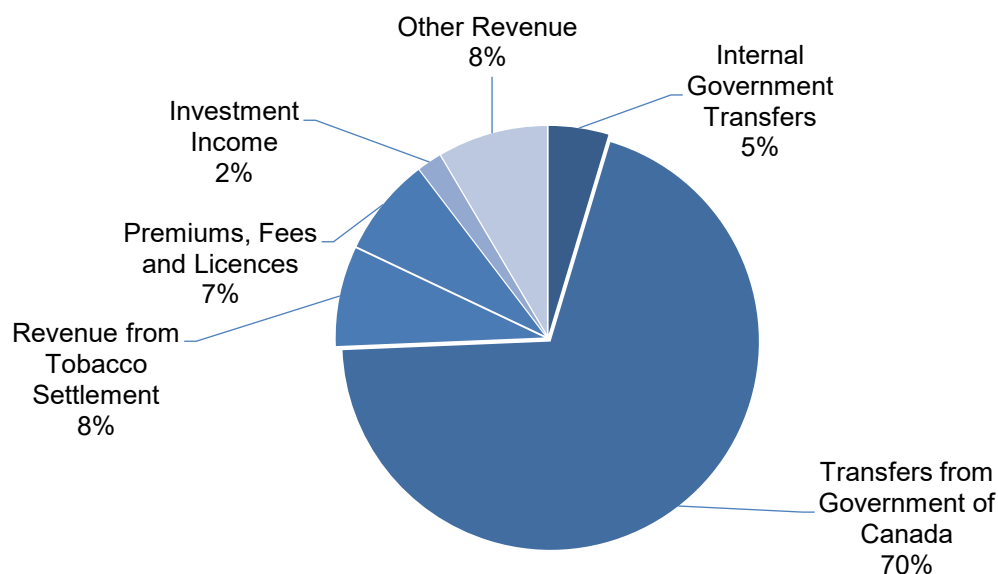
Year ended March 31, 2025

(in thousands)

|                                                    | 2025                   |                        | 2024                   |                    | Change from         |             |
|----------------------------------------------------|------------------------|------------------------|------------------------|--------------------|---------------------|-------------|
|                                                    | Budget<br>(Restated)   | Actual                 | Actual<br>(Restated)   |                    | Budget              | 2024 Actual |
| <b>Revenues</b>                                    |                        |                        |                        |                    |                     |             |
| Internal Government Transfers                      | \$ 379,441             | \$ 428,556             | \$ 418,925             | \$ 49,115          | \$ 9,631            |             |
| Transfers from Government of Canada                | 6,469,821              | 6,443,737              | 6,088,175              | (26,084)           | 355,562             |             |
| Revenue from Tobacco Settlement                    | -                      | 712,530                | -                      | 712,530            | 712,530             |             |
| Premiums, Fees and Licences                        | 661,024                | 701,558                | 661,946                | 40,534             | 39,612              |             |
| Investment Income                                  | 78,620                 | 173,361                | 107,747                | 94,741             | 65,614              |             |
| Other Revenue                                      | 779,412                | 784,672                | 627,987                | 5,260              | 156,685             |             |
| <b>Ministry Total</b>                              | <b>8,368,318</b>       | <b>9,244,414</b>       | <b>7,904,780</b>       | <b>876,096</b>     | <b>1,339,634</b>    |             |
| Inter-Ministry Consolidation Adjustments           | (671,755)              | (627,567)              | (465,712)              | 44,188             | (161,855)           |             |
| <b>Adjusted Ministry Total</b>                     | <b>7,696,563</b>       | <b>8,616,847</b>       | <b>7,439,068</b>       | <b>920,284</b>     | <b>1,177,779</b>    |             |
| <b>Expenses - Directly Incurred</b>                |                        |                        |                        |                    |                     |             |
| Ministry Support Services                          | 66,738                 | 104,861                | 57,939                 | 38,123             | 46,922              |             |
| Physician Compensation and Development             | 6,679,611              | 7,150,758              | 6,415,470              | 471,147            | 735,288             |             |
| Acute Care                                         | 4,456,007              | 4,719,389              | 4,419,457              | 263,382            | 299,932             |             |
| Diagnostic, Therapeutic, and Other Patient Service | 2,507,930              | 2,569,171              | 2,443,061              | 61,241             | 126,110             |             |
| Drugs and Supplemental Health Benefits             | 2,800,863              | 2,984,660              | 2,792,668              | 183,797            | 191,992             |             |
| Community Care                                     | 1,446,377              | 1,408,562              | 1,311,947              | (37,815)           | 96,615              |             |
| Continuing Care                                    | 1,425,244              | 1,480,965              | 1,371,332              | 55,721             | 109,633             |             |
| Home Care                                          | 930,200                | 958,955                | 843,510                | 28,755             | 115,445             |             |
| Population and Public Health                       | 1,102,015              | 907,892                | 1,005,866              | (194,123)          | (97,974)            |             |
| Emergency Medical Services                         | 730,200                | 704,133                | 660,449                | (26,067)           | 43,684              |             |
| Support Services                                   | 2,681,890              | 2,732,235              | 2,644,222              | 50,345             | 88,013              |             |
| Information Technology                             | 983,437                | 932,964                | 861,976                | (50,473)           | 70,988              |             |
| Administration                                     | 552,241                | 561,462                | 533,283                | 9,221              | 28,179              |             |
| Research and Education                             | 117,478                | 134,831                | 123,025                | 17,353             | 11,806              |             |
| Infrastructure Support                             | -                      | -                      | 281                    | -                  | (281)               |             |
| Other Health Services                              | 572,077                | 636,826                | 568,987                | 64,749             | 67,839              |             |
| Debt Servicing                                     | 18,000                 | 15,335                 | 15,788                 | (2,665)            | (453)               |             |
| <b>Ministry Total</b>                              | <b>27,070,308</b>      | <b>28,002,999</b>      | <b>26,069,261</b>      | <b>932,691</b>     | <b>1,933,738</b>    |             |
| Inter-Ministry Consolidation Adjustments           | (513,507)              | (449,543)              | (289,909)              | 63,964             | (159,634)           |             |
| <b>Adjusted Ministry Total</b>                     | <b>26,556,801</b>      | <b>27,553,456</b>      | <b>25,779,352</b>      | <b>996,655</b>     | <b>1,774,104</b>    |             |
| <b>Adjusted Annual Deficit</b>                     | <b>\$ (18,860,238)</b> | <b>\$ (18,936,609)</b> | <b>\$ (18,340,284)</b> | <b>\$ (76,371)</b> | <b>\$ (596,325)</b> |             |

## Revenue and Expense Highlights

### Consolidated Revenues (prior to inter-ministry consolidation adjustments)



**Total revenue for the year was \$9.2 billion, which was \$876 million higher than budget mainly due to:**

- Internal Government Transfers due to new grant initiatives including additional funding to support seniors aging with dignity in their community;
- Revenue from Tobacco Settlement due to the recent Court settlement decision against the tobacco companies – see note below;
- Premiums, Fees and Licenses due to increased clinical activity resulting in higher number of patients billable to non-residents of Canada, the Workers Compensation Board, and other provinces; and
- Investment Income due to increased net realized gains from active portfolio management, higher interest and dividend income as a result of strong market performance, and higher bond amortization income from the purchase of bonds and treasury bills at a discount.

**Actual revenue increased by \$1.3 billion over prior year mainly due to:**

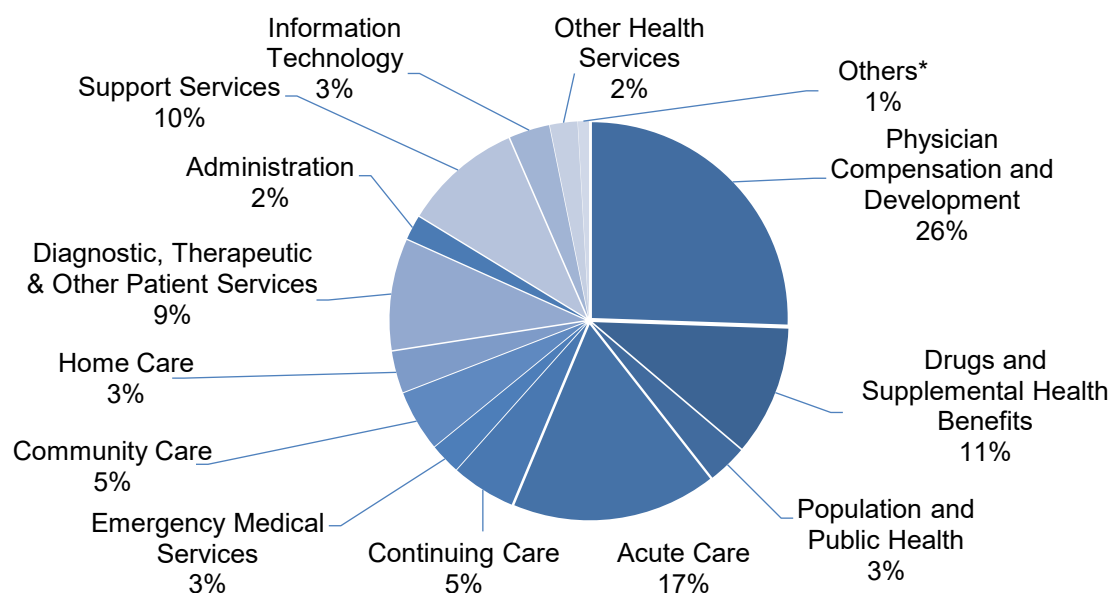
- Transfers from Government of Canada mainly due to Canada Health Transfer as a result of higher national entitlement and Working Together to Improve Health Care for Canadians, and new funding for Drugs for Rare Diseases;
- Premiums, Fees and Licenses due to increased interprovincial billing rates and increased number of patients billable to non-residents of Canada;
- Investment Income due to increased net realized gain from active portfolio management and increased interest and dividend due to favourable market conditions; and
- Other Revenue mainly due to recoveries under the new transition services agreement between Alberta Health Services and Recovery Alberta.

**Note:** On March 6, 2025, the Ontario Superior Court of Justice approved the settlement agreement arising from the litigation against three major tobacco companies to recover smoking related healthcare costs. The total settlement was \$32.5 billion to be paid partly by up-front lump sum and partly through annual payments, as a percentage of annual net after tax income. The percentage starts at 85 percent of the net after tax income for the first 5 years, reduced by 5 percent for each increment of 5 years. The

annual payments are reduced to 70 percent starting year 16 until the amount is fully paid. Provinces and Territories share of the allocation was \$24.7 billion with upfront payment being \$6.2 billion. Alberta's share is roughly 12.63 percent, representing \$3.1 billion of the total settlement with \$783 million being upfront payment and the balance to be paid annually over the settlement period.

The Revenue from Tobacco settlement reported represents upfront payment of \$783 million net legal fees of \$70 million. The remaining balance has not been recorded given the significant uncertainty related to future annual payments which are based on future net after tax income. The balance will be recorded annually when there is certainty of the amount to be received.

## Consolidated Expenses (prior to inter-ministry consolidation adjustments)



\* includes Ministry Support Services, Research and Education, Debt Servicing, and Infrastructure Support.

**Total expense for the year was \$28 billion, which was \$933 million higher than budget mainly due to:**

- Physician Compensation and Development primarily due to increase in physician service volumes, growth in utilization of physician services, and Clinical Alternative Relationship Plan expansions;
- Acute Care due to increased clinical activity and cost pressures from higher number of inpatients, emergency department visits, and operating room cases; use of agency nursing and overtime, and increase related to collective bargaining process;
- Diagnostic, Therapeutic and Other Patient Services due to increased clinical activity, higher patient volumes, and increase related to the collective bargaining process;
- Drugs and Supplemental Health due to increased benefit utilization driven by aging population and higher cost of medication, increased Pharmacy Services utilization as alternate to primary care providers, partially offset by higher than anticipated Product Listing Agreement recoveries;
- Continuing Care due to increased funding to long-term care providers to address increased activity and support additional hours of care, funding for seniors aging with dignity in their community, and increase related to collective bargaining process;
- Support Services due to increased costs related to collective bargaining process and cost of inventory transferred to Recovery Alberta; and
- Other Health Services due to population growth resulting in growth in optometry and oral surgery services and increased payments to Canadian Blood Services for higher than anticipated utilization of blood services and products.

This was partially offset by lower than budgeted expenses in:

- Population and Public Health due to deferral of Primary Care initiatives until next fiscal year, delayed implementation of Health Innovation and Alberta Surgical Initiative, and reduced pharmacist injection services spending; and
- Information Technology due to resource constraints resulting in deferral of system enhancement projects.

**Actual expenses increased by \$1.9 billion from prior year mainly due to:**

- Physician Compensation and Development due to increase in physician service volumes, growth in utilization of physician services, and Clinical Alternative Relationship Plan expansions;
- Acute Care due to increased clinical activity and emergency department visits, higher number of inpatients, and increase related to the collective bargaining process;
- Diagnostic, Therapeutic and Other Patient Services due to increased clinical activity, higher patient volumes resulting in increased laboratory, CT and MRI testing, and increase related to the collective bargaining process;
- Drugs and Supplemental Health Benefits due to increased benefits utilization driven by an aging population, increased costs of drugs in various benefit programs, partially offset by increased Product Listing Agreement recoveries;
- Continuing Care due to increased funding to long-term care providers for contract inflation, additional funding to support Continuing Care Transformation Initiatives, additional long term care beds introduced related to the Continuing Care Capacity Plan Initiative, and increase related to the collective bargaining process;
- Community Care due to opening of new designated supportive-living and community beds, contract inflation related to community care providers, and increase related to the collective bargaining process;
- Home Care due to increased activity to provide additional hours and choices to home care patients, additional funding provided to home care operators to support seniors aging with dignity in their community, and increase related to collective bargaining process;
- Support Services due to costs associated with the transition of community lab services to AHS partway through prior fiscal year and increase related to the collective bargaining process;
- Information Technology due to higher amortization related to Connect Care system additions during the year, resumption of projects deferred from prior year, and increase related to collective bargaining process; and
- Other Health Services due to volume growth in optometry and oral surgery services as a result of population growth and increased payments to Canadian Blood Services for blood services and products.

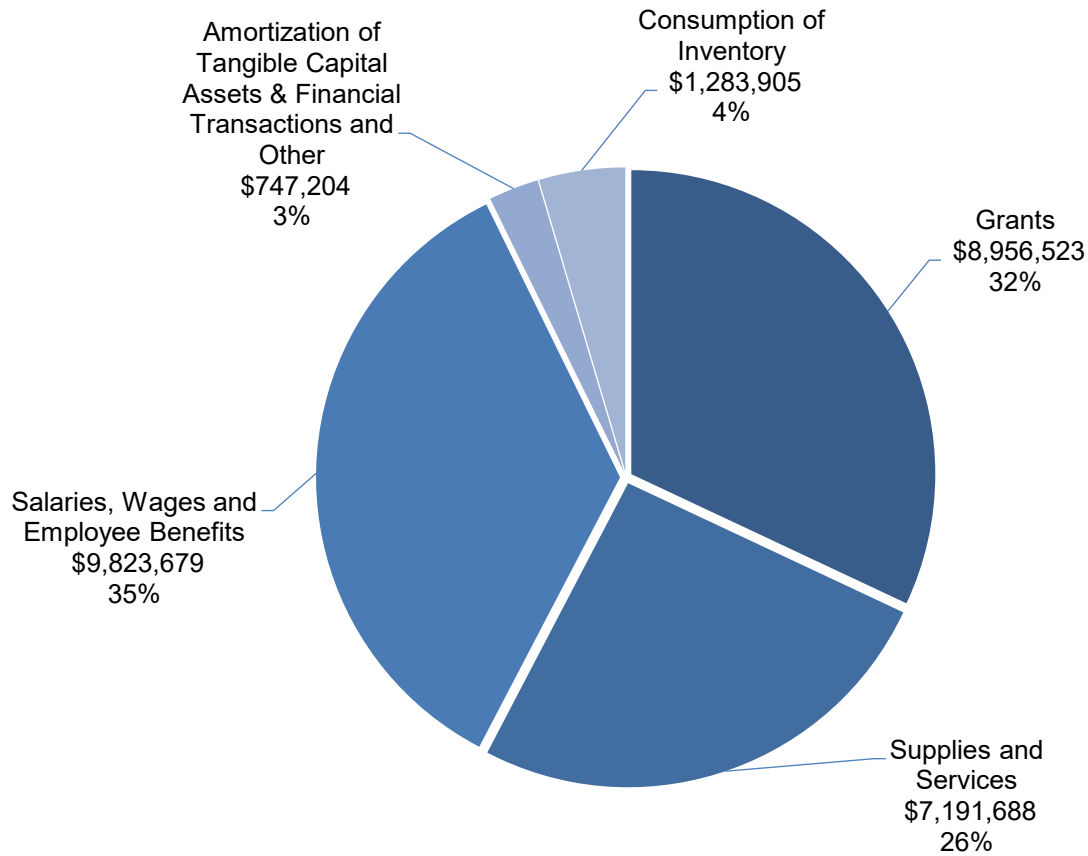
This was partially offset by decrease in:

- Population and Public Health due to COVID-19 rapid tests write-offs in prior year partially offset by costs related to implementation of Modernizing Alberta's Primary Health Care System (MAPS) initiatives in current year.

## Expenses – Directly Incurred Detailed by Object (unaudited)

(in thousands)

### 2025 Actual (unaudited)

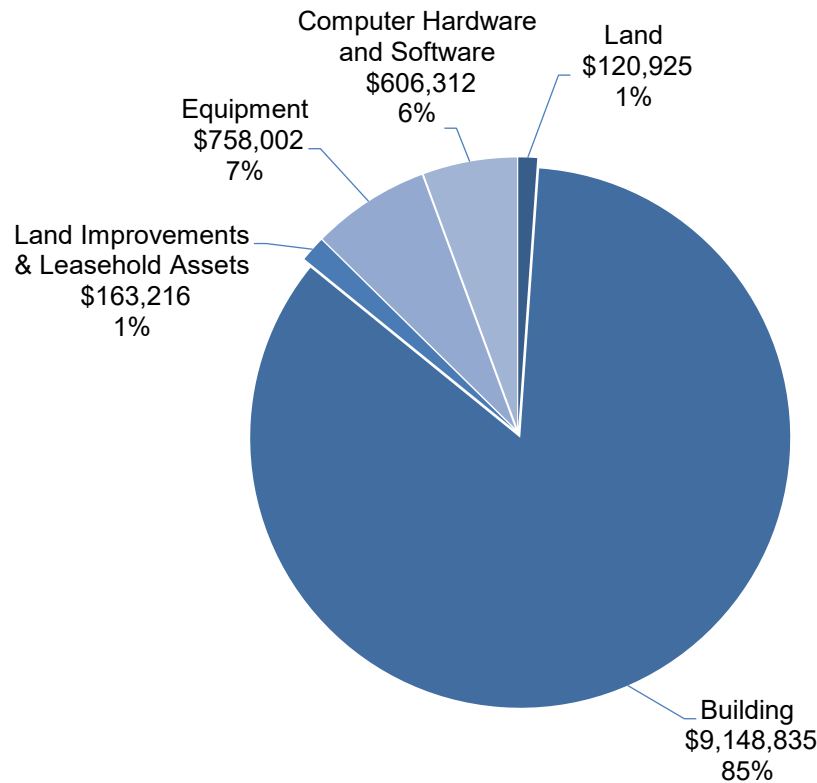


- Ministry's expenses were primarily for grants, salaries, wages and employee benefits, and supplies and services, which combined, accounted for 93 percent of the total expense.
- Grants were \$9.0 billion or 32 percent of total expense, primarily for Physician Compensation and Development, and Drugs and Supplemental Health Benefit programs. Other grant expenses included restricted funding to support organizations and communities through various programs or initiatives, and funding for Out-of-Province Health Care Services and Allied Health Services.
- Salaries, wages and employee benefits were \$9.8 billion or 35 percent of total expense, supporting delivery of health services across the province.
- Supplies and services were \$7.2 billion or 26 percent of total expense, mainly attributed to contracts with voluntary and private health service providers, and various purchased supplies and services.
- Amortization of tangible capital assets (\$589 million), consumption of inventory (\$1.3 billion), and financial transactions and other expenses (\$158 million) comprised the balance of ministry's total expense.

## Supplemental Financial Information

### Tangible Capital Assets (unaudited)

**Year ended March 31, 2025**  
(in thousands)



- Total net book value of tangible capital assets was \$10.8 billion.
- Buildings are the largest component and account for \$9.1 billion or 85 percent of the ministry's total tangible capital assets.
- The remainder of the ministry's tangible capital assets comprises of computer hardware and software, equipment, land, land improvements, and leasehold assets.
- Tangible capital assets of the Ministry are recognized at historical cost and are amortized, excluding land, on a straight-line basis over the estimated useful lives of the assets.
- Buildings and Equipment include \$295 million (net of amortization) for obligations associated with retirement of tangible capital assets.

**Note:** The Government of Alberta enacted the Real Property Governance Act in 2024, as part of its efforts to centralize the management of public property to enhance accountability and transparency. Under this legislation, all freehold real property owned by AHS will be transferred to the Ministry of Infrastructure effective April 1, 2025.

**Portfolio Investments (unaudited)**

**Year ended March 31, 2025**  
*(in thousands)*

|                                    | 2025                |                     | 2024                |                     |
|------------------------------------|---------------------|---------------------|---------------------|---------------------|
|                                    | Cost                | Fair Value          | Cost                | Fair Value          |
| Interest bearing securities:       |                     |                     |                     |                     |
| Deposits and short-term securities | \$ 685,016          | \$ 689,144          | \$ 746,326          | \$ 746,331          |
| Bonds and mortgages                | 1,413,967           | 1,444,907           | 1,326,783           | 1,304,133           |
|                                    | <u>2,098,983</u>    | <u>2,134,051</u>    | <u>2,073,109</u>    | <u>2,050,464</u>    |
| Equities:                          |                     |                     |                     |                     |
| Canadian equities                  | 172,446             | 162,401             | 181,927             | 199,709             |
| Global equities                    | 302,168             | 344,898             | 224,040             | 299,807             |
|                                    | <u>474,614</u>      | <u>507,299</u>      | <u>405,967</u>      | <u>499,516</u>      |
| Real estate                        | <u>31,069</u>       | <u>38,777</u>       | <u>30,926</u>       | <u>37,712</u>       |
| Total Portfolio Investments        | <u>\$ 2,604,666</u> | <u>\$ 2,680,127</u> | <u>\$ 2,510,002</u> | <u>\$ 2,587,692</u> |

The following is a breakdown of portfolio investments:

|                             | 2025                |                     | 2024                |                     |
|-----------------------------|---------------------|---------------------|---------------------|---------------------|
|                             | Cost                | Fair Value          | Cost                | Fair Value          |
| Operating                   | \$ 2,494,746        | \$ 2,570,207        | \$ 2,404,729        | \$ 2,482,419        |
| Endowments                  | 109,920             | 109,920             | 105,273             | 105,273             |
| Total Portfolio Investments | <u>\$ 2,604,666</u> | <u>\$ 2,680,127</u> | <u>\$ 2,510,002</u> | <u>\$ 2,587,692</u> |

# Financial Statements of Other Reporting Entities

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## Alberta Health Services

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# Management’s Responsibility for Financial Reporting

The accompanying consolidated financial statements for the year ended March 31, 2025 are the responsibility of management and have been reviewed and approved by senior management. The consolidated financial statements were prepared in accordance with Canadian Public Sector Accounting Standards and include certain disclosures required by the financial directives issued by Alberta Health, and of necessity include some amounts based on estimates and judgment.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains systems of financial management and internal control which give consideration to costs, benefits and risks that are designed to:

- provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded so as to maintain accountability of public funds;
- safeguard the assets and properties of the “Province of Alberta” that are the responsibility of Alberta Health Services.

The Official Administrator meets with management and the Auditor General of Alberta to review financial matters, and approves the consolidated financial statements upon finalization of the audit. The Auditor General of Alberta has free access to the Official Administrator.

The Auditor General of Alberta provides an independent audit of the consolidated financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures which allow him to report on the fairness of the consolidated financial statements prepared by management.

[Originally signed by]  
\_\_\_\_\_  
Andre Tremblay  
Interim President and Chief Executive Officer  
Alberta Health Services

[Originally signed by]  
\_\_\_\_\_  
Emily Ma  
Interim Vice President and Chief  
Financial Officer  
Alberta Health Services

June 2, 2025

# Independent Auditor's Report



**Auditor  
General**  
OF ALBERTA

To the Official Administrator of Alberta Health Services, the Minister of Primary and Preventative Health Services, and the Minister of Hospital and Surgical Health Services

## Report on the Consolidated Financial Statements

### Opinion

I have audited the consolidated financial statements of Alberta Health Services (the Group), which comprise the consolidated statement of financial position as at March 31, 2025, and the consolidated statements of operations, remeasurement gains and losses, change in net debt, and cash flows for the year then ended, and notes to the consolidated financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying consolidated financial statements present fairly, in all material respects, the consolidated financial position of the Group as at March 31, 2025, and the results of its operations, its remeasurement gains and losses, its changes in net debt, and its cash flows for the year then ended in accordance with Canadian public sector accounting standards.

### Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Consolidated Financial Statements* section of my report. I am independent of the Group in accordance with the ethical requirements that are relevant to my audit of the consolidated financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

### Emphasis of matter

I draw attention to Note 1, Note 3 and Note 30 of the consolidated financial statements, which describes the authority, purpose and operations, restructuring, and subsequent events of the Group. My opinion is not modified in respect of this matter.

### Other information

Management is responsible for the other information. The other information comprises the information included in the *Annual Report*, but does not include the consolidated financial statements and my auditor's report thereon. The *Annual Report* is expected to be made available to me after the date of this auditor's report.

My opinion on the consolidated financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the consolidated financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is

materially inconsistent with the consolidated financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

### **Responsibilities of management and those charged with governance for the consolidated financial statements**

Management is responsible for the preparation and fair presentation of the consolidated financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the consolidated financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the consolidated financial statements, management is responsible for assessing the Group's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the Group's financial reporting process.

### **Auditor's responsibilities for the audit of the consolidated financial statements**

My objectives are to obtain reasonable assurance about whether the consolidated financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists.

Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these consolidated financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgment and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the consolidated financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Group's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Group's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my

auditor's report to the related disclosures in the consolidated financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit evidence obtained up to the date of my auditor's report. However, future events or conditions may cause the Group to cease to continue as a going concern.

- Evaluate the overall presentation, structure and content of the consolidated financial statements, including the disclosures, and whether the consolidated financial statements represent the underlying transactions and events in a manner that achieves fair presentation.
- Plan and perform the group audit to obtain sufficient appropriate audit evidence regarding the financial information of the entities or business units within the Group as a basis for forming an opinion on the group financial statements. I am responsible for the direction, supervision and review of the audit work performed for purposes of the group audit. I remain solely responsible for my audit opinion.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Originally signed by W. Doug Wylie FCPA, FCMA, ICD.D]  
Auditor General

June 2, 2025  
Edmonton, Alberta

## Consolidated Statement of Operations

| CONSOLIDATED STATEMENT OF OPERATIONS<br>YEAR ENDED MARCH 31 |                                    |                     |                     |
|-------------------------------------------------------------|------------------------------------|---------------------|---------------------|
|                                                             | 2025                               |                     | 2024                |
|                                                             | Budget<br>(Note 4)<br>(Schedule 4) | Actual              | Actual<br>(Note 29) |
| <b>Revenues:</b>                                            |                                    |                     |                     |
| Alberta Health transfers                                    |                                    |                     |                     |
| Base operating                                              | \$ 14,602,924                      | \$ 14,562,674       | \$ 15,138,434       |
| One-time base operating                                     | -                                  | 362,916             | 286,083             |
| Other operating                                             | 1,653,528                          | 1,640,502           | 1,681,469           |
| Recognition of expended deferred capital revenue            | 102,100                            | 110,130             | 106,297             |
| Other government transfers (Note 5)                         | 1,652,176                          | 910,784             | 558,891             |
| Fees and charges                                            | 621,000                            | 667,747             | 625,668             |
| Ancillary operations                                        | 92,700                             | 95,426              | 87,178              |
| Donations and non-government contributions (Note 6)         | 170,900                            | 203,410             | 213,382             |
| Investment and other income (Note 7)                        | 248,700                            | 550,210             | 330,376             |
| <b>TOTAL REVENUES</b>                                       | <b>19,144,028</b>                  | <b>19,103,799</b>   | <b>19,027,778</b>   |
| <b>Expenses:</b>                                            |                                    |                     |                     |
| Continuing care                                             | 1,425,800                          | 1,484,701           | 1,375,360           |
| Community care                                              | 2,132,200                          | 1,667,596           | 1,983,628           |
| Home care                                                   | 930,200                            | 959,280             | 843,709             |
| Acute care                                                  | 6,081,528                          | 6,208,453           | 6,084,875           |
| Emergency medical services                                  | 735,400                            | 709,763             | 665,954             |
| Diagnostic and therapeutic services                         | 2,956,600                          | 3,038,465           | 2,932,784           |
| Population and public health                                | 420,900                            | 431,697             | 439,173             |
| Research and education                                      | 378,400                            | 375,341             | 352,707             |
| Information technology                                      | 804,300                            | 813,626             | 793,775             |
| Support services (Note 8)                                   | 2,713,800                          | 2,764,742           | 2,798,372           |
| Administration (Note 9)                                     | 564,900                            | 576,314             | 573,810             |
| Net effect of restructuring transaction (Note 3(a))         | -                                  | 2,761               | -                   |
| <b>TOTAL EXPENSES</b>                                       | <b>19,144,028</b>                  | <b>19,032,739</b>   | <b>18,844,147</b>   |
| <b>ANNUAL OPERATING SURPLUS</b>                             | <b>-</b>                           | <b>71,060</b>       | <b>183,631</b>      |
| Accumulated surplus, beginning of year                      | 1,304,458                          | 1,304,458           | 1,120,827           |
| <b>Accumulated surplus, end of year (Note 23)</b>           | <b>\$ 1,304,458</b>                | <b>\$ 1,375,518</b> | <b>\$ 1,304,458</b> |

The accompanying notes and schedules are part of these consolidated financial statements.

## Consolidated Statement of Financial Position

| CONSOLIDATED STATEMENT OF FINANCIAL POSITION<br>AS AT MARCH 31 |                     |                     |
|----------------------------------------------------------------|---------------------|---------------------|
|                                                                | 2025                | 2024                |
|                                                                | Actual              | Actual              |
| <b>Financial Assets:</b>                                       |                     |                     |
| Cash and cash equivalents                                      | \$ 526,238          | \$ 243,462          |
| Portfolio investments (Note 11)                                | 2,677,619           | 2,587,692           |
| Accounts receivable (Note 12)                                  | 979,173             | 755,525             |
|                                                                | 4,183,030           | 3,586,679           |
| <b>Liabilities:</b>                                            |                     |                     |
| Accounts payable and accrued liabilities (Note 14)             | 2,633,239           | 1,855,955           |
| Employee future benefits (Note 15)                             | 742,757             | 818,539             |
| Unexpended deferred operating revenue (Note 16)                | 532,642             | 697,922             |
| Unexpended deferred capital revenue (Note 17)                  | 253,901             | 214,072             |
| Debt (Note 19)                                                 | 375,587             | 415,813             |
| Asset retirement obligations (Note 20)                         | 635,463             | 539,421             |
|                                                                | 5,173,589           | 4,541,722           |
| <b>NET DEBT</b>                                                | <b>(990,559)</b>    | <b>(955,043)</b>    |
| <b>Non-Financial Assets:</b>                                   |                     |                     |
| Tangible capital assets (Note 21)                              | 10,761,267          | 10,467,655          |
| Inventories of supplies (Note 22)                              | 189,437             | 197,169             |
| Prepaid expenses, deposits, and other non-financial assets     | 397,629             | 355,181             |
|                                                                | 11,348,333          | 11,020,005          |
| <b>NET ASSETS BEFORE EXPENDED DEFERRED CAPITAL REVENUE</b>     | <b>10,357,774</b>   | <b>10,064,962</b>   |
| Expended deferred capital revenue (Note 18)                    | 8,912,554           | 8,696,431           |
| <b>NET ASSETS</b>                                              | <b>1,445,220</b>    | <b>1,368,531</b>    |
| Net Assets is comprised of:                                    |                     |                     |
| Accumulated surplus (Note 23)                                  | 1,375,518           | 1,304,458           |
| Accumulated remeasurement gains                                | 69,702              | 64,073              |
|                                                                | \$ <b>1,445,220</b> | \$ <b>1,368,531</b> |

Contractual Rights (Note 13)

Contractual Obligations, Contingent Liabilities and Other Matters (Note 24)

*The accompanying notes and schedules are part of these consolidated financial statements.*

Approved by:

[Originally signed by]

**Andre Tremblay**  
**Official Administrator**  
**Alberta Health Services**

## Consolidated Statement of Change in Net Debt

| CONSOLIDATED STATEMENT OF CHANGE IN NET DEBT<br>YEAR ENDED MARCH 31                                               |                     |                     |                     |
|-------------------------------------------------------------------------------------------------------------------|---------------------|---------------------|---------------------|
|                                                                                                                   | 2025                |                     | 2024                |
|                                                                                                                   | Budget<br>(Note 4)  | Actual              | Actual              |
| Annual operating surplus                                                                                          | \$ -                | \$ 71,060           | \$ 183,631          |
| Effect of changes in tangible capital assets:                                                                     |                     |                     |                     |
| Acquisition of tangible capital assets:                                                                           |                     |                     |                     |
| Purchased                                                                                                         | (407,100)           | (487,388)           | (452,501)           |
| Purchased as part of DynaLIFEDX asset purchase                                                                    | -                   | -                   | (71,760)            |
| Leased                                                                                                            | (22,000)            | (15,185)            | (37,753)            |
| Constructed by Alberta Infrastructure on behalf of AHS                                                            | (302,000)           | (258,231)           | (198,774)           |
| Contributed by others                                                                                             | -                   | -                   | (24)                |
| Revision to asset retirement cost estimates                                                                       | -                   | (101,256)           | 41,773              |
| Amortization and loss on disposals/write-downs of tangible capital assets                                         | 561,900             | 563,648             | 555,033             |
| Transferred as a result of restructuring (Note 3(a))                                                              | -                   | 4,800               | -                   |
| Effect of other changes:                                                                                          |                     |                     |                     |
| Net increase in expensed deferred capital revenue                                                                 | 105,500             | 218,468             | 170,966             |
| Net decrease in expensed deferred operating revenue                                                               | -                   | -                   | (116,636)           |
| Net decrease in inventories of supplies                                                                           | -                   | 7,732               | 118,442             |
| Net (increase) decrease in prepaid expenses, deposits and other non-financial assets                              | 49,000              | (59,629)            | (118,968)           |
| Net increase in other non-financial assets due to DynaLIFEDX asset purchase                                       | -                   | -                   | (12,845)            |
| Net decrease in other non-financial assets and expensed deferred capital revenue due to restructuring (Note 3(a)) | -                   | 14,836              | -                   |
| Net remeasurement gains for the year                                                                              | 20,000              | 5,629               | 43,803              |
| <b>Decrease (increase) in net debt for the year</b>                                                               | <b>5,300</b>        | <b>(35,516)</b>     | <b>104,387</b>      |
| Net debt, beginning of year                                                                                       | (955,043)           | (955,043)           | (1,059,430)         |
| <b>Net debt, end of year</b>                                                                                      | <b>\$ (949,743)</b> | <b>\$ (990,559)</b> | <b>\$ (955,043)</b> |

The accompanying notes and schedules are part of these consolidated financial statements

## Consolidated Statement of Remeasurement Gains and Losses

| CONSOLIDATED STATEMENT OF REMEASUREMENT GAINS AND LOSSES<br>YEAR ENDED MARCH 31 |                  |                  |
|---------------------------------------------------------------------------------|------------------|------------------|
|                                                                                 | 2025             | 2024             |
|                                                                                 | Actual           | Actual           |
| Unrestricted unrealized (losses) gains attributable to:                         |                  |                  |
| Derivatives                                                                     | \$ (4,643)       | \$ 586           |
| Portfolio investments                                                           |                  |                  |
| Quoted in an active market                                                      | (360)            | 6,600            |
| Designated at fair value                                                        | 58,408           | 35,815           |
| Amounts reclassified to the Consolidated Statement of Operations:               |                  |                  |
| Derivatives                                                                     | (1,369)          | (998)            |
| Portfolio investments                                                           |                  |                  |
| Quoted in an active market                                                      | (6,086)          | -                |
| Designated at fair value                                                        | (40,321)         | 1,800            |
| <b>Net remeasurement gains for the year</b>                                     | <b>5,629</b>     | <b>43,803</b>    |
| Accumulated remeasurement gains, beginning of year                              | 64,073           | 20,270           |
| <b>Accumulated remeasurement gains, end of year (Note 11)</b>                   | <b>\$ 69,702</b> | <b>\$ 64,073</b> |

*The accompanying notes and schedules are part of these consolidated financial statements.*

## Consolidated Statement of Cash Flows

| CONSOLIDATED STATEMENT OF CASH FLOWS<br>YEAR ENDED MARCH 31               |                   |                   |
|---------------------------------------------------------------------------|-------------------|-------------------|
|                                                                           | 2025              | 2024              |
|                                                                           | Actual            | Actual            |
| <b>Operating transactions:</b>                                            |                   |                   |
| Annual operating surplus                                                  | \$ 71,060         | \$ 183,631        |
| Non-cash items:                                                           |                   |                   |
| Amortization and loss on disposals/write-downs of tangible capital assets | 563,648           | 555,033           |
| Revenue recognized for acquisition of land                                | (168)             | -                 |
| Recognition of expended deferred capital revenue                          | (365,803)         | (355,407)         |
| Recognition of expended deferred operating revenue                        | -                 | (116,636)         |
| Gain on disposal of portfolio investments                                 | (74,048)          | (13,736)          |
| Change in employee future benefits                                        | (2,007)           | 24,268            |
| Net effect of restructuring transaction                                   | 2,761             | -                 |
| (Increase) decrease in:                                                   |                   |                   |
| Cash transferred due to restructuring (Note 3(a))                         | (53,284)          | -                 |
| Accounts receivable related to operating transactions                     | (235,140)         | (14,564)          |
| Inventories of supplies                                                   | 7,732             | 118,442           |
| Prepaid expenses, deposits, and other non-financial assets                | (59,629)          | (118,968)         |
| Increase (decrease) in:                                                   |                   |                   |
| Accounts payable and accrued liabilities                                  | 669,650           | 23,669            |
| Unexpended deferred operating revenue                                     | (29,954)          | 115,570           |
| Asset retirement obligations                                              | (5,214)           | (1,978)           |
| Cash provided by operating transactions                                   | 489,604           | 399,324           |
| <b>Capital transactions:</b>                                              |                   |                   |
| Purchased tangible capital assets                                         | (487,388)         | (452,501)         |
| DynaLIFEDX purchase consideration net of cash acquired                    | -                 | (29,388)          |
| Cash applied to capital transactions                                      | (487,388)         | (481,889)         |
| <b>Investing transactions:</b>                                            |                   |                   |
| Purchase of portfolio investments                                         | (6,658,023)       | (5,284,617)       |
| Proceeds on disposals of portfolio investments                            | 6,634,834         | 4,948,882         |
| Cash applied to investing transactions                                    | (23,189)          | (335,735)         |
| <b>Financing transactions:</b>                                            |                   |                   |
| Restricted capital contributions received                                 | 372,924           | 367,372           |
| Unexpended deferred capital revenue returned                              | (6,887)           | (3,626)           |
| Proceeds from debt                                                        | -                 | 20,000            |
| Principal payments on debt                                                | (40,226)          | (38,275)          |
| Payments on obligations under capital leases                              | (22,422)          | (17,745)          |
| Net receipt (repayment) of life lease deposits                            | 360               | (613)             |
| Cash provided by financing transactions                                   | 303,749           | 327,113           |
| <b>Increase (decrease) in cash and cash equivalents</b>                   | <b>282,776</b>    | <b>(91,187)</b>   |
| Cash and cash equivalents, beginning of year                              | 243,462           | 334,649           |
| <b>Cash and cash equivalents, end of year</b>                             | <b>\$ 526,238</b> | <b>\$ 243,462</b> |

The accompanying notes and schedules are part of these consolidated financial statements.

# Notes to the Consolidated Financial Statements for the Year Ended March 31, 2025

## Note 1 Authority, Purpose and Operations

Alberta Health Services (AHS) was established under the *Regional Health Authorities Act* (Alberta), effective April 1, 2009, as a result of the merger of 12 formerly separate health entities in Alberta. AHS is currently undergoing a significant restructuring which aligns with the *Provincial Health Agencies Act* (Alberta) (PHA Act). According to the PHA Act, the restructuring establishes four sector-based provincial health agencies: Acute Care Alberta (ACA), Primary Care Alberta (PCA), Assisted Living Alberta (ALA), and Recovery Alberta. As a result, through a ministerial order, all powers, duties, responsibilities or functions will be transferred from AHS to the newly established provincial health agencies, provincial health corporations, or other successors. Pursuant to *Ministerial Order 800/2024*, on completion of the transfers, AHS will be wound up as a regional health authority. In accordance with the *Alberta Health Services Provincial Health Corporation Regulation* (Alberta), the entity Alberta Health Services Provincial Health Corporation (PHC) was established on June 1, 2025, for the purposes of delivering health services in the acute care health services sector. This new PHC is accountable to Acute Care Alberta.

To date, provincial health agencies for the mental health and addiction, primary care, and acute care sectors have been established. Recovery Alberta, the provincial health agency responsible for the mental health and addiction sector, was legally established on July 1, 2024, became operational on September 1, 2024, and is accountable to the Minister of Mental Health and Addiction. PCA, the provincial health agency responsible for the primary care sector, was established on November 18, 2024, became operational on February 1, 2025, and is accountable to the Minister of Health. ACA, the provincial health agency responsible for the acute care sector was established on February 1, 2025, and became operational on April 1, 2025. Matters pertaining to restructuring transactions occurring in the current year are discussed further in Note 3. Matters pertaining to restructuring transactions occurring subsequent to year end are discussed further in Note 30.

Pursuant to Section 5 of the *Regional Health Authorities Act* (Alberta) and prior to the effective date of the *Provincial Health Agencies Act* on June 21, 2024, AHS was responsible in Alberta to:

- promote and protect the health of the population and work toward the prevention of disease and injury;
- assess on an ongoing basis the health needs of the population;
- determine priorities in the provision of health services and allocate resources accordingly;
- ensure that reasonable access to quality health services is provided and;
- promote the provision of health services in a manner that is responsive to the needs of individuals and communities and supports the integration of services and facilities.

Pursuant to Section 5 of the *Provincial Health Agencies Act* (Alberta), effective June 21, 2024, AHS is responsible in Alberta to:

- promote and protect the health of the population and work toward the prevention of disease and injury;
- assess on an ongoing basis the health needs of the population;
- support that reasonable access to quality health services is provided and;
- promote the delivery of health services in a manner that is responsive to the needs of individuals and communities and supports the integration of services and facilities.

AHS is accountable to the Minister of Health (the Minister). The responsibilities under section 5 of the PHA Act do not apply to AHS with respect to health services that have been transferred by ministerial order to one or more provincial health agencies or other successor. Additionally, AHS has entered into a Transition Services Framework Agreement with each operational PHA to provide clinical and corporate support services.

The AHS consolidated financial statements include the assets, liabilities, revenues and expenses associated with its responsibilities.

## Note 2 Significant Accounting Policies and Reporting Practices

### (a) Basis of Presentation

These consolidated financial statements have been prepared in accordance with Canadian Public Sector Accounting Standards. In addition, the consolidated financial statements include certain disclosures required by the financial directives issued by Alberta Health (AH).

These consolidated financial statements reflect the assets, liabilities, revenues, and expenses of the reporting entity, which is comprised of the organizations controlled by AHS as noted below:

### (i) Controlled Entities

AHS controls the following three wholly owned subsidiaries:

- Alberta Precision Laboratories Ltd. - provides medical diagnostic services throughout Alberta.
- Capital Care Group Inc. - manages continuing care programs and facilities in the Edmonton area.
- Carewest - manages continuing care programs and facilities in the Calgary area.

AHS has majority representation on, or the right to appoint, the governance boards, indicating control of the following entities:

- Foundations:

The largest foundations controlled by AHS are the Alberta Cancer Foundation and the Calgary Health Foundation. AHS also controls 33 other foundations that facilitate fundraising for various initiatives including enhancements to healthcare delivery (including equipment), programs, renovations, and research and education.

- Provincial Health Authorities of Alberta Liability and Property Insurance Plan (LPIP):

The LPIP's main purpose is to share the risks of general and professional liability to lessen the impact on any one healthcare subscriber. Effective April 1, 2020, the LPIP ceased providing new liability coverage and continues in operation for the limited purpose of winding up its affairs.

The LPIP has a fiscal year end of December 31, 2024. Significant transactions occurring between this date and March 31, 2025 have been recorded in these consolidated financial statements.

All inter-entity accounts and transactions between these organizations and AHS are eliminated upon consolidation.

### (ii) Government Partnerships

AHS proportionately consolidates its 50% ownership interest in the Northern Alberta Clinical Trials Centre (NACTRC) partnership with the University of Alberta, and its 33.33% ownership interest in the Institute for Reconstructive Sciences in Medicine (iRSM) partnership with the University of Alberta and Covenant Health (Note 26). Additionally, AHS proportionately consolidated its 50% ownership interests in 39 (2024 – 39) Primary Care Network (PCN) partnerships with physician groups up until February 1, 2025.

AHS had entered into local primary care initiative agreements to jointly manage and operate the delivery of primary care services to achieve the PCN plan objectives, and to contract and hold property interests required in the delivery of PCN services. Effective February 1, 2025, due to the restructuring, AHS' 50% ownership interests in the 39 PCNs has been transferred to PCA, and AHS is no longer named as a partner (Note 3(a)).

All inter-entity accounts and transactions between these organizations and AHS are eliminated upon consolidation.

**Note 2 Significant Accounting Policies and Reporting Practices (continued)****(iii) Trusts under Administration**

These consolidated financial statements do not include trusts administered on behalf of others (Note 27).

**(iv) Other**

AHS contracts with various voluntary and private health service providers to provide certain health services throughout Alberta. The largest of these service providers is Covenant Health, a denominational health care organization, providing a full spectrum of care including operating several hospitals and long-term care facilities. Covenant Health is an independent, separate legal entity with a separate Board of Directors and accordingly, these consolidated financial statements do not include their assets, liabilities or results of operations. However, the payments for contracts with health service providers such as Covenant Health are recorded as expenses in the Consolidated Statement of Operations.

In addition, AHS provides administrative services to certain foundations and contracted health care providers not included in these consolidated financial statements.

**(b) Revenue Recognition**

All revenues are recorded on an accrual basis, except when the accrual cannot be determined within a reasonable degree of certainty or when estimation is impracticable. Revenues from transactions with performance obligations are recognized when AHS provides the promised goods and/or services to a payor. Revenue from transactions with no performance obligations are recognized at their realizable value when AHS has the authority to claim or retain an inflow of economic resources and identifies a past transaction or event that gives rise to an asset. Unallocated costs comprising of materials and services contributed by related parties in support of AHS' operations, are not recognized in these consolidated financial statements.

**(i) Government Transfers**

Transfers from AH, other Province of Alberta ministries and agencies, and other government entities are referred to as government transfers.

Government transfers and, if applicable, the associated externally restricted investment income are recorded as deferred revenue when the stipulations together with AHS' actions and communications as to the use of the transfer, create a liability. These transfers are recognized as revenue as the stipulations are met and, when applicable, AHS complies with the communicated use of the transfer.

All other government transfers, without stipulations for the use of the transfer, are recorded as revenue when the transfer is authorized and AHS meets the eligibility criteria.

Deferred revenue consists of unexpended deferred operating revenue (Note 16), unexpended deferred capital revenue (Note 17), and expended deferred capital revenue (Note 18). The term deferred revenue in these consolidated financial statements refers to the components of deferred revenue as described.

**(ii) Donations and Non-Government Contributions**

Donations and non-government contributions are received from individuals, corporations, registered charities, and other not-for-profit organizations. Donations and non-government contributions may be unrestricted or externally restricted for operating or capital purposes.

Unrestricted donations, and non-government contributions are recorded as revenue in the year received or in the year the funds are committed to AHS if the amount can be reasonably estimated and collection is reasonably assured.

Externally restricted donations, non-government contributions, and, if applicable, the associated externally restricted investment income are recorded as deferred revenue if the terms for their use, or the terms along with AHS' actions and communications as to their use, create a liability. These resources are recognized as revenue as the terms are met and, when applicable, AHS complies with the communicated use.

**Note 2 Significant Accounting Policies and Reporting Practices (continued)**

In-kind contributions of services and materials from non-related parties are recorded at fair value when such value can reasonably be determined. While volunteers contribute a significant amount of time each year to assist AHS, the value of their services is not recognized as revenue and expenses in the consolidated financial statements because fair value cannot be reasonably determined.

Endowment contributions are recognized in the Consolidated Statement of Operations in the period in which they are received or receivable.

**(iii) Transfers and Donations related to Land**

Transfers and donations for the purchase of land are recorded as deferred revenue when received and as revenue when the land is purchased. In-kind donations of land from non-related entities are recorded as revenue at the fair value of the land. When AHS cannot determine the fair value, it records such donations at nominal value. In-kind donations of land from related entities are recorded as revenue at the net book value of the transferring entity.

**(iv) Fees and Charges, Ancillary Operations, and Other Income**

Fees and charges, ancillary operations, and other income are considered revenue arising from exchange transactions with performance obligations. These are recognized in the year that goods are delivered or services are provided by AHS. Amounts received for which goods or services have not been provided by year-end are recorded as deferred revenue.

**(v) Investment Income**

Investment income includes dividend income, interest income, and realized gains or losses on the sale of portfolio investments. Unrealized gains and losses on portfolio investments (excluding gains or losses from restricted transfers, endowments, or donations) are recognized in the Consolidated Statement of Remeasurement Gains and Losses until the related portfolio investments are sold. When realized, these gains or losses are recognized in the Consolidated Statement of Operations. Investment income and unrealized gains and losses from restricted transfers including endowments or donations are deferred until recognized according to the provisions within the individual funding agreements.

**(c) Expenses**

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed. Interest expense includes debt servicing costs.

Expenses include grants and transfers under shared cost agreements. Grants and transfers are recorded as expenses when the transfer is authorized and eligibility criteria have been met by the recipient.

**(d) Financial Instruments**

Financial instruments comprise financial assets and financial liabilities. Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations. Financial liabilities are contractual obligations to deliver cash or another financial asset to another entity or to exchange financial instruments with another entity under conditions that are potentially unfavourable to AHS.

**Note 2     Significant Accounting Policies and Reporting Practices (continued)**

All of AHS' financial assets and financial liabilities are initially recorded at their fair value. The following table identifies AHS' financial assets and financial liabilities and identifies how they are subsequently measured:

| <b>Financial Assets and Financial Liabilities</b>                                                                                                                           | <b>Subsequent Measurement and Recognition</b>                                                                                                                                                                                                                                               |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Portfolio investments                                                                                                                                                       | Measured at fair value with unrealized changes in fair values recognized in the Consolidated Statement of Remeasurement Gains and Losses or deferred revenue until realized, at which time the cumulative changes in fair value are recognized in the Consolidated Statement of Operations. |
| Cash and cash equivalents, accounts receivable, payroll payable and related accrued liabilities, trade accounts payable and accrued liabilities, other liabilities and debt | Measured at cost or amortized cost.                                                                                                                                                                                                                                                         |

Amortized cost is the amount at which a financial instrument asset or a financial instrument liability is measured at initial recognition minus principal repayments, plus or minus the cumulative amortization using the effective interest method of any difference between that initial amount and the maturity amount, and minus any reduction (directly or through the use of an allowance account) for impairment or uncollectibility.

AHS records equity investments quoted in an active market at fair value and may choose to record other financial assets under the fair value category if there is an investment strategy to evaluate the performance of a group of these financial assets on a fair value basis. AHS has elected to record all portfolio investments at fair value. The relative reliability of information used to measure the fair value is disclosed in Note 11.

Derivatives are recorded at fair value in the Consolidated Statement of Financial Position. Derivatives with a positive or negative fair value are recognized as increases or decreases to portfolio investments respectively. Unrealized gains and losses from changes in the fair value of derivatives are recognized in the Consolidated Statement of Remeasurement Gains and Losses until realized, at which time the gains or losses are recognized in the Consolidated Statement of Operations.

Contractual obligations are evaluated for the existence of embedded derivatives. AHS measures and recognizes embedded derivatives separately from the host contract when the economic characteristics and risk of the embedded derivative are not closely related to those of the host contract, when it meets the definition of a derivative and when the entire contract is not measured at fair value. An election can be made to either measure the entire contract at fair value or measure the value of the derivative component separately when characteristics of the derivative are not closely related to the economic characteristics and risks of the contract itself. Contracts to buy or sell non-financial items for AHS' normal course of business are not recognized as financial assets or liabilities. AHS does not have any embedded derivatives.

All financial assets are assessed for impairment on an annual basis. When a decline is determined to be other than temporary, the amount of the loss is reported as a realized loss on the Consolidated Statement of Operations. A write-down of a portfolio investment to reflect a loss in value is not reversed for a subsequent increase in value.

A financial liability or a part thereof is derecognized when it is extinguished.

Transaction costs associated with the acquisition and disposal of portfolio investments are expensed as incurred. Investment management fees are expensed as incurred. The purchase and disposition of portfolio investments are recognized on the trade date.

**(e) Cash and Cash Equivalents**

Cash is comprised of cash on hand and demand deposits. Cash equivalents include amounts in interest bearing accounts and are subject to an insignificant risk of change in value. Cash and cash equivalents are held for the purpose of meeting short-term commitments rather than for investment purposes.

**Note 2    Significant Accounting Policies and Reporting Practices (continued)**

**(f)    Accounts receivable**

Accounts receivable are recognized at the lower of cost or net recoverable value. A valuation allowance is recognized when recovery is uncertain.

**(g)    Inventories of Supplies**

Purchased inventories of supplies are valued at lower of cost (defined as moving average cost) and replacement cost. Contributed inventories of supplies are recorded at fair value when such value can reasonably be determined.

**(h)    Tangible Capital Assets**

Tangible capital assets are recorded at cost less accumulated amortization, which includes amounts that are directly related to the acquisition, design, construction, development, improvement, or betterment of the assets. Cost includes overhead directly attributable to construction and development as well as interest costs that are directly attributable to the acquisition or construction of the asset, and asset retirement cost. Costs incurred by Alberta Infrastructure (AI) to construct tangible capital assets on behalf of AHS are recorded by AHS as work in progress as AI incurs costs.

Contributed tangible capital assets from non-related entities are recognized at their fair value at the date of the contribution when fair value can be reasonably determined. When AHS cannot determine the fair value, it records such contributions at nominal value.

The costs less residual values of tangible capital assets, excluding land, are amortized over their estimated useful lives on a straight-line basis as follows:

|                             | <u>Useful Life</u> |
|-----------------------------|--------------------|
| Facilities and improvements | 10-70 years        |
| Equipment                   | 3-20 years         |
| Information systems         | 3-15 years         |
| Building service equipment  | 5-40 years         |
| Land improvements           | 5-40 years         |

Work in progress, which includes facility and improvement projects and development of information systems, is not amortized until after a project is substantially complete and the tangible capital assets are available for use.

Leases of tangible capital assets which transfer substantially all benefits and risks of ownership to AHS are accounted for as leased tangible capital assets and leasehold improvements and are amortized over the shorter of the term of the lease or their estimated useful lives. Obligations under capital leases are recorded at the present value of the minimum lease payments excluding executory costs (e.g. insurance, maintenance costs, etc.). The discount rate used to determine the present value of the lease payments is the lower of AHS' rate for incremental borrowing and the interest rate implicit in the lease.

Tangible capital assets are written down to their net recoverable amount when conditions indicate that they no longer contribute to AHS' ability to provide goods and services or when the value of future economic benefits associated with the tangible capital assets are less than their net book value. Write-downs are recorded as part of amortization and loss on disposals / write-downs of tangible capital assets.

Intangibles and other assets inherited by right and that have not been purchased are not recognized in these consolidated financial statements. Similarly, works of art, historical treasures, and collections are not recognized as tangible capital assets.

## Note 2 Significant Accounting Policies and Reporting Practices (continued)

### (i) Employee Future Benefits

#### (i) Defined Benefit Pension Plans

##### **Local Authorities Pension Plan (LAPP) and Management Employees Pension Plan (MEPP)**

AHS participates in the LAPP and MEPP, both of which are multi-employer registered defined benefit pension plans. AHS accounts for these plans on a defined contribution basis. Accordingly, the pension expense recorded for these plans in the consolidated financial statements is comprised of the employer contributions that AHS is required to pay for participating employees during the fiscal year. LAPP and MEPP set the employer contribution rates on an annual basis based on actuarially pre-determined amounts that are expected to provide the plans' future benefit obligations.

##### **Supplemental Executive Retirement Plan (SERP)**

The SERP covers certain employees and supplements the benefits under AHS' registered plans that are limited by the *Income Tax Act* (Canada). The SERP has been closed to new entrants since April 1, 2009. The SERP provides future pension benefits to participants based on years of service and earnings.

As required under the *Income Tax Act* (Canada), approximately half of the assets are held in a non-interest bearing Refundable Tax Account with the Canada Revenue Agency. The remaining assets of the SERP are invested in a combination of Canadian equities and Canadian fixed income securities.

#### (ii) Defined Contribution Pension Plans

##### **Group Registered Retirement Savings Plans (GRRSPs)**

AHS sponsors GRRSPs for certain employee groups. Under the GRRSPs, AHS matches a certain percentage of any contribution made by plan participants up to certain limits. AHS also sponsors a defined contribution pension plan for certain employee groups where the employee and employer each contribute specified percentages of pensionable earnings.

##### **Supplemental Pension Plan (SPP)**

Subsequent to April 1, 2009, staff that would have otherwise been eligible for SERP have been enrolled in a defined contribution SPP. The SPP supplements the benefits under AHS registered plans that are limited by the *Income Tax Act* (Canada). AHS contributes a percentage of an eligible employee's pensionable earnings, in excess of the limits of the *Income Tax Act* (Canada). The SPP provides participants with an account balance at retirement based on the contributions made to the plan and investment income earned on the contributions based on investment decisions made by the participant.

#### (iii) Other Benefit Plans

##### **Accumulating Non-Vesting Sick Leave**

Sick leave benefits accumulate with employees' service and are provided by AHS to certain employee groups, as defined by employment agreements, to cover illness related absences that are outside of short-term and long-term disability coverage. Benefit amounts are determined and accumulate with reference to employees' earnings at the time they are paid out. The cost of the accumulating non-vesting sick leave benefits is expensed as the benefits are earned.

AHS recognizes a liability and expense for accumulating non-vesting sick leave benefits using an actuarial cost method as the employees render services to earn the benefits. The liability and expense is determined using the projected benefit method pro-rated for service and management's best estimates of expected discount rate, inflation, rate of compensation increase, termination and retirement dates, sick leave accumulation and utilization, and mortality. Actuarial gains and losses are amortized on a straight-line basis over the expected average remaining service life of the related employee groups.

**Note 2 Significant Accounting Policies and Reporting Practices (continued)**

AHS does not record a liability for sick leave benefits that do not accumulate beyond the current reporting year as these are renewed annually.

**Other Benefits**

AHS provides its employees with basic life, accidental death and dismemberment, short-term disability, long-term disability, extended health, dental, and vision benefits through benefits carriers. AHS fully accrues its obligations for employee non-pension future benefits.

**(j) Asset Retirement Obligations**

Asset retirement obligations are legal obligations associated with the retirement of tangible capital assets. A liability for an asset retirement obligation is recognized when, as at the financial reporting date:

- (i) there is a legal obligation to incur retirement costs in relation to a tangible capital asset;
- (ii) the past transaction or event giving rise to the liability has occurred;
- (iii) it is expected that future economic benefits will be given up; and
- (iv) a reasonable estimate of the amount can be made.

Asset retirement obligations are initially measured as of the date the legal obligation was incurred, based on management's best estimate of the amount required to retire tangible capital assets.

When a liability for asset retirement obligation is recognized, asset retirement costs related to recognized tangible capital assets in productive use are capitalized by increasing the carrying amount of the related asset and are amortized on a straight-line basis over the estimated useful life of the underlying tangible capital asset (Note 2(h)). Asset retirement costs related to unrecognized tangible capital assets and those not in productive use are expensed. Revisions in estimates are recognized as a change to both the liability and related tangible capital asset in the Consolidated Statement of Financial Position.

**(k) Foreign Currency Translation**

Transaction amounts denominated in foreign currencies are translated into their Canadian dollar equivalents at exchange rates prevailing at the transaction dates. Carrying values of monetary assets and liabilities and non-monetary items denominated in foreign currencies included in the fair value category reflect the exchange rates at the Consolidated Statement of Financial Position date. Unrealized foreign exchange gains and losses are recognized in the Consolidated Statement of Remeasurement Gains and Losses.

In the year of settlement, foreign exchange gains and losses are reclassified to the Consolidated Statement of Operations, and the cumulative amount of remeasurement gains and losses are reversed in the Consolidated Statement of Remeasurement Gains and Losses.

**(l) Reserves**

Certain amounts, as approved by the Official Administrator, may be set aside in accumulated surplus for use by AHS for future purposes. Transfers to, or from, are recorded to the respective reserve account when approved. Reserves include Invested in Tangible Capital Assets and Internally Restricted Surplus for Insurance Equity Requirements, Foundations and Health Benefit Trust of Alberta.

**(m) Measurement Uncertainty**

The consolidated financial statements, by their nature, contain estimates and are subject to measurement uncertainty. Measurement uncertainty exists when there is a difference between the recognized or disclosed amount and another reasonably possible amount. These estimates and assumptions are reviewed at least annually. Actual results could differ from the estimates determined by management in these consolidated financial statements, and these differences could require adjustment in subsequent reporting years.

## Note 2 Significant Accounting Policies and Reporting Practices (continued)

Measurement uncertainty exists in the fair values reported for portfolio investments designated to the fair value hierarchy (see Note 11). The fair values of these investments are based on estimates. Estimated fair values may not reflect amounts that could be recognized upon immediate sale or amounts that ultimately may be recognized.

The amount recorded for amortization of tangible capital assets is based on the estimated useful life of the related assets while the recognition of expended deferred capital revenue depends on when the terms for the use of the funding are met and, when applicable, AHS complies with its communicated use of the funding. The amounts recorded for accumulating non-vesting sick leave are based on various assumptions including the estimated service life of employees, drawdown rate of sick leave banks and rate of salary escalation. The establishment of the provision for unpaid claims relies on judgment and estimates including historical precedent and trends, prevailing legal, economic, social, and regulatory trends; and expectation as to future developments.

There is measurement uncertainty related to asset retirement obligations as it involves estimates in determining settlement amount and timing of settlement. Changes to any of these estimates and assumptions may result in change to the obligation.

### (n) Future Accounting Changes

On April 1, 2026, AHS will adopt the following new conceptual framework and accounting standard approved by the Public Sector Accounting Board:

- **The Conceptual Framework for Financial Reporting in the Public Sector**  
The Conceptual Framework is the foundation for public sector financial reporting standard setting. It replaces the conceptual aspects of Section PS 1000 Financial Statement Concepts and Section PS 1100 Financial Statement Objectives. The conceptual framework highlights considerations fundamental for the consistent application of accounting issues in the absence of specific standards.
- **PS 1202 Financial Statement Presentation**  
Section PS 1202 sets out general and specific requirements for the presentation of information in general purpose financial statements. The financial statement presentation principles are based on the concepts within the Conceptual Framework.

Management is currently assessing the impact of the conceptual framework and the standard on the AHS consolidated financial statements.

## Note 3 Restructuring

In accordance with the Government of Alberta's ministerial orders related to the restructuring of AHS, all responsibilities, the oversight and coordination of the delivery of mental health and addiction services, along with certain related assets, liabilities and funding agreements, were transferred from AHS to provincial health agency Recovery Alberta effective September 1, 2024. Similarly, effective February 1, 2025, AHS transferred all responsibilities, the oversight and coordination of the delivery of primary care services, along with certain related assets, liabilities and funding agreements to the provincial health agency PCA. AHS, Recovery Alberta, and PCA are accountable to their respective Ministries within Government of Alberta. AHS did not provide or receive any compensation related to this transfer.

The two remaining provincial health agencies for the acute care and continuing care sectors will become operational after March 31, 2025 (Note 30).

**Note 3 Restructuring (continued)****(a) Assets and liabilities**

Assets and liabilities were transferred based on a methodology approved by the Minister of Health, which involved transferring an integrated set of assets and liabilities that support mental health and addictions operations for Recovery Alberta and primary care operations for PCA. AHS has signed agreements with Recovery Alberta and PCA for the transfer of these assets and liabilities. AHS is providing cash management services, including investment and portfolio oversight, therefore cash-settled assets and liabilities are shown in the 'Net due from AHS' line below.

The assets and liabilities were transferred at their carrying amounts as follows:

|                                                            | <b>Recovery Alberta</b><br>September 1, 2024 | <b>Primary Care Alberta</b><br>February 1, 2025 <sup>(i)</sup> | <b>Total</b>    |
|------------------------------------------------------------|----------------------------------------------|----------------------------------------------------------------|-----------------|
| <b>Financial assets:</b>                                   |                                              |                                                                |                 |
| Cash and cash equivalents                                  | \$ 27                                        | \$ 53,257                                                      | \$ 53,284       |
| Portfolio investments                                      | -                                            | 2,574                                                          | 2,574           |
| Accounts receivable                                        | 8,830                                        | 2,662                                                          | 11,492          |
| Net due from AHS                                           | 190,825                                      | 23,269                                                         | 214,094         |
|                                                            | 199,682                                      | 81,762                                                         | 281,444         |
| <b>Liabilities:</b>                                        |                                              |                                                                |                 |
| Accounts payable and accrued liabilities                   | 75,227                                       | 24,356                                                         | 99,583          |
| Employee future benefits                                   | 64,718                                       | 9,058                                                          | 73,776          |
| Unexpended deferred operating revenue                      | 66,786                                       | 58,174                                                         | 124,960         |
|                                                            | 206,731                                      | 91,588                                                         | 298,319         |
| <b>NET DEBT</b>                                            | <b>(7,049)</b>                               | <b>(9,826)</b>                                                 | <b>(16,875)</b> |
| <b>Non-financial assets:</b>                               |                                              |                                                                |                 |
| Tangible capital assets <sup>(ii)</sup>                    | 4,738                                        | 62                                                             | 4,800           |
| Prepaid expenses, deposits, and other non-financial assets | 5,278                                        | 11,903                                                         | 17,181          |
|                                                            | 10,016                                       | 11,965                                                         | 21,981          |
| <b>NET ASSETS BEFORE EXPENDED DEFERRED CAPITAL REVENUE</b> | <b>2,967</b>                                 | <b>2,139</b>                                                   | <b>5,106</b>    |
| Expended deferred capital revenue                          | 2,288                                        | 57                                                             | 2,345           |
| <b>Net effect of restructuring</b>                         | <b>\$ 679</b>                                | <b>\$ 2,082</b>                                                | <b>\$ 2,761</b> |

(i) PCA amounts include 50% ownership in the PCN partnership with physician groups which includes total financial assets of \$57,238, liabilities of \$68,189 and non-financial assets of \$10,951.

(ii) The tangible capital assets transferred to Recovery Alberta and PCA include vehicle leases and equipment with a net book value of \$4,738 (cost of \$18,203 and accumulated amortization of \$13,465) and \$62 (cost of \$1,281 and accumulated amortization of \$1,219) respectively. No real property was transferred to Recovery Alberta or PCA as the properties will be transferring to Alberta Infrastructure effective April 1, 2025 (Note 30).

The restructuring-related costs incurred were attributable to severance costs and were recognized as salaries and benefits expense in the statement of operations. The financial impact of these costs was insignificant.

**Note 3 Restructuring (continued)**

The related sector contractual obligations were also transferred to Recovery Alberta and PCA. Contracts transferred to Recovery Alberta amounting to \$262,703 were related to the continuum of mental health and addiction services and supports, from prevention and intervention to treatment and recovery. Contracts transferred to PCA amounted to \$23,635 mainly related to midwifery services.

Recovery Alberta and PCA have each signed a Transition Services Framework Agreement and a Cooperation Agreement with AHS. Together, these agreements will govern the relationship between AHS and the two provincial health agencies throughout the transition. Under the Transition Services Framework Agreement, AHS provides clinical and corporate support services including but not limited to nutrition, food, linen & environmental services, capital management, information technology, protective services, pharmacy services, human resources, financial operations, APL services, and health information management.

**(b) Revenue and expenses**

The revenues and expenses for mental health and addictions and primary care operations, included in the Statement of Operations before transferring responsibilities to Recovery Alberta on September 1, 2024, and PCA on February 1, 2025, are as follows:

|                                                  | <b>Mental health<br/>and addictions<br/>5 months ended<br/>August 31, 2024</b> | <b>Primary care<br/>10 months ended<br/>January 31, 2025</b> | <b>Mental health<br/>and addictions<br/>12 months<br/>ended<br/>March 31, 2024</b> | <b>Primary care<br/>12 months<br/>ended<br/>March 31, 2024</b> |
|--------------------------------------------------|--------------------------------------------------------------------------------|--------------------------------------------------------------|------------------------------------------------------------------------------------|----------------------------------------------------------------|
| <b>Revenues:</b>                                 |                                                                                |                                                              |                                                                                    |                                                                |
| Alberta Health transfers                         |                                                                                |                                                              |                                                                                    |                                                                |
| Base operating                                   | \$ -                                                                           | \$ 92,872                                                    | \$ 990,104                                                                         | \$ 106,437                                                     |
| Other operating                                  | 284                                                                            | 106,560                                                      | 1,055                                                                              | 124,735                                                        |
| Recognition of expended deferred capital revenue | 115                                                                            | 1                                                            | 281                                                                                | 2                                                              |
| Other government transfers                       | 470,863                                                                        | 7,244                                                        | 108,962                                                                            | 8,131                                                          |
| Fees and charges                                 | 1,397                                                                          | 3,546                                                        | 5,805                                                                              | 4,040                                                          |
| Donations and non-government contributions       | 2,010                                                                          | 3                                                            | 5,318                                                                              | -                                                              |
| Investment and other income                      | 7,594                                                                          | 4,460                                                        | 16,468                                                                             | 8,312                                                          |
| <b>TOTAL REVENUES</b>                            | <b>482,263</b>                                                                 | <b>214,686</b>                                               | <b>1,127,993</b>                                                                   | <b>251,657</b>                                                 |
| <b>Expenses by function:</b>                     |                                                                                |                                                              |                                                                                    |                                                                |
| Continuing care                                  | 1,818                                                                          | -                                                            | 4,351                                                                              | -                                                              |
| Community care                                   | 275,973                                                                        | 202,252                                                      | 634,581                                                                            | 236,868                                                        |
| Acute care                                       | 170,067                                                                        | 3,794                                                        | 389,392                                                                            | 4,469                                                          |
| Diagnostic and therapeutic services              | 31,849                                                                         | 1,304                                                        | 72,498                                                                             | 1,417                                                          |
| Population and public health                     | 2,194                                                                          | 269                                                          | 5,963                                                                              | 406                                                            |
| Research and education                           | 1,026                                                                          | -                                                            | 2,547                                                                              | -                                                              |
| Information technology                           | 283                                                                            | 65                                                           | 511                                                                                | -                                                              |
| Support services                                 | 5,687                                                                          | 1,567                                                        | 12,945                                                                             | 1,568                                                          |
| Administration                                   | 2,945                                                                          | 7,503                                                        | 5,205                                                                              | 6,929                                                          |
| <b>TOTAL EXPENSES</b>                            | <b>491,842</b>                                                                 | <b>216,754</b>                                               | <b>1,127,993</b>                                                                   | <b>251,657</b>                                                 |
| <b>NET OPERATING DEFICIT</b>                     | <b>(9,579)</b>                                                                 | <b>(2,068)</b>                                               | <b>-</b>                                                                           | <b>-</b>                                                       |

**Note 3      Restructuring (continued)**

|                                                                                 | <b>Mental health and<br/>addictions</b><br>5 months ended<br>August 31, 2024 | <b>Primary care</b><br>10 months ended<br>January 31, 2025 | <b>Mental health<br/>and addictions</b><br>12 months<br>ended<br>March 31, 2024 | <b>Primary care</b><br>12 months<br>ended<br>March 31, 2024 |
|---------------------------------------------------------------------------------|------------------------------------------------------------------------------|------------------------------------------------------------|---------------------------------------------------------------------------------|-------------------------------------------------------------|
| <b>Expenses by object:</b>                                                      |                                                                              |                                                            |                                                                                 |                                                             |
| Salaries and benefits                                                           | \$ 376,717                                                                   | \$ 167,931                                                 | \$ 856,453                                                                      | \$ 188,441                                                  |
| Contracts with health service<br>providers                                      | 97,858                                                                       | 25,892                                                     | 226,283                                                                         | 29,092                                                      |
| Drugs and gases                                                                 | 5,346                                                                        | 91                                                         | 11,919                                                                          | 167                                                         |
| Medical supplies                                                                | 938                                                                          | 172                                                        | 1,751                                                                           | 175                                                         |
| Other contracted services                                                       | 4,155                                                                        | 1,907                                                      | 14,196                                                                          | 1,808                                                       |
| Other                                                                           | 6,520                                                                        | 20,745                                                     | 15,842                                                                          | 31,963                                                      |
| Amortization and loss on<br>disposals/write-downs of tangible<br>capital assets | 308                                                                          | 16                                                         | 1,549                                                                           | 11                                                          |
|                                                                                 | <b>491,842</b>                                                               | <b>216,754</b>                                             | <b>1,127,993</b>                                                                | <b>251,657</b>                                              |

**Note 4      Budget**

The 2024-25 original annual budget was approved by the former AHS Board on March 28, 2024 for submission to the Minister who approved it on June 24, 2024. The restated annual budget, resulting from the restructuring of AHS (Note 1) was approved by the former AHS Board on January 27, 2025 and submitted to the Minister. A reconciliation from the original to the restated annual budget has been provided in the Consolidated Schedule of the Revised Budget (Schedule 4).

**Note 5      Other Government Transfers**

|                                                            | <b>Budget<br/>(Note 4)</b> | <b>2025</b>       | <b>2024<br/>(Note 29)</b> |
|------------------------------------------------------------|----------------------------|-------------------|---------------------------|
| Recognition of expended deferred capital revenue (Note 18) | \$ 234,200                 | \$ 222,678        | \$ 216,354                |
| Restricted operating (Note 16(a))                          | 255,338                    | 234,567           | 282,222                   |
| Unrestricted operating                                     | 1,162,638                  | 453,539           | 60,315                    |
|                                                            | <b>\$ 1,652,176</b>        | <b>\$ 910,784</b> | <b>\$ 558,891</b>         |

Other government transfers include \$897,739 (2024 – \$523,929) (Note 25) transferred from the Province of Alberta, \$13,045 (2024 – \$34,962) from government entities outside the Province of Alberta and exclude amounts from AH as these amounts are separately disclosed on the Consolidated Statement of Operations.

**Note 6      Donations and Non-Government Contributions**

|                                                            | <b>Budget<br/>(Note 4)</b> | <b>2025</b>       | <b>2024</b>       |
|------------------------------------------------------------|----------------------------|-------------------|-------------------|
| Recognition of expended deferred capital revenue (Note 18) | \$ 33,500                  | \$ 32,995         | \$ 34,252         |
| Restricted operating (Note 16(a))                          | 136,200                    | 147,036           | 139,182           |
| Unrestricted operating                                     | 1,200                      | 18,732            | 12,167            |
| Endowment contributions (Note 23)                          | -                          | 4,647             | 27,781            |
|                                                            | <b>\$ 170,900</b>          | <b>\$ 203,410</b> | <b>\$ 213,382</b> |

**Note 7 Investment and Other Income**

|                                                | Budget<br>(Note 4) | 2025              | 2024              |
|------------------------------------------------|--------------------|-------------------|-------------------|
| Investment income                              | \$ 78,600          | \$ 176,531        | \$ 118,227        |
| Other income:                                  |                    |                   |                   |
| AH                                             | 10,525             | 24,976            | 11,507            |
| Other Province of Alberta Ministries (Note 25) | 26,000             | 184,252           | 29,791            |
| Other <sup>(i)</sup>                           | 133,575            | 164,451           | 170,851           |
|                                                | <b>\$ 248,700</b>  | <b>\$ 550,210</b> | <b>\$ 330,376</b> |

<sup>(i)</sup> Other mainly relates to recoveries for services provided to third parties.

**Note 8 Support Services**

|                                                                               | Budget<br>(Note 4)  | 2025                | 2024                |
|-------------------------------------------------------------------------------|---------------------|---------------------|---------------------|
| Facilities operations                                                         | \$ 1,104,300        | \$ 980,945          | \$ 1,070,364        |
| Patient health records, food services, and transportation                     | 517,100             | 516,153             | 513,793             |
| Housekeeping, laundry, and linen                                              | 258,300             | 265,727             | 264,093             |
| Materials management                                                          | 253,000             | 214,042             | 233,350             |
| Support services expense of full-spectrum contracted health service providers | 179,500             | 200,641             | 182,462             |
| Ancillary operations                                                          | 63,100              | 60,676              | 61,412              |
| Fundraising expenses and grants awarded                                       | 53,800              | 62,085              | 53,310              |
| Other <sup>(i)</sup>                                                          | 284,700             | 464,473             | 419,588             |
|                                                                               | <b>\$ 2,713,800</b> | <b>\$ 2,764,742</b> | <b>\$ 2,798,372</b> |

<sup>(i)</sup> Other includes valuation adjustments of \$588 (2024 – \$78,997). The prior year valuation adjustment was due to reduced demand for inventories purchased to support public health emergencies in previous years (Note 22).

**Note 9 Administration**

|                                                                             | Budget<br>(Note 4) | 2025              | 2024              |
|-----------------------------------------------------------------------------|--------------------|-------------------|-------------------|
| General administration                                                      | \$ 259,900         | \$ 261,283        | \$ 276,775        |
| Human resources                                                             | 154,500            | 154,777           | 143,236           |
| Finance                                                                     | 85,900             | 80,472            | 80,422            |
| Communications                                                              | 26,300             | 26,307            | 28,038            |
| Administration expense of full-spectrum contracted health service providers | 38,300             | 53,475            | 45,339            |
|                                                                             | <b>\$ 564,900</b>  | <b>\$ 576,314</b> | <b>\$ 573,810</b> |

**Note 10 Financial Risk Management**

AHS is exposed to a variety of financial risks associated with its financial instruments. These financial risks include market risk, credit risk, and liquidity risk.

**(a) Market Risk**

Market risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate due to changes in market prices. Market risk is comprised of three types of risk: price risk, interest rate risk, and foreign currency risk.

In accordance with the AHS investment bylaw and policy, AHS manages market risk by maintaining a conservative and diversified portfolio, and engages Alberta Investment Management Corporation, a related party, to manage the portfolio. Compliance with the bylaw and policy is monitored and reported to the Official Administrator on a quarterly basis.

**Note 10 Financial Risk Management (continued)**

In order to earn financial returns at an acceptable level of market risk, the consolidated investment portfolio is governed by investment bylaws and policies with clearly established target asset mixes. The target assets range between 0% to 100% for cash and money market securities, 0% to 80% for fixed income securities and 0% to 70% for equity holdings.

Risk is reduced through asset class diversification, diversification within each asset class, and portfolio quality constraints governing the quality of portfolio holdings.

AHS assesses the sensitivity of its portfolio to market risk based on historical volatility of equity and fixed income markets. Volatility is determined using a ten-year average based on fixed income and equity market fluctuations and is applied to the total portfolio. Based on the volatility average of 3.72% (2024 – 3.70%) increase or decrease, with all other variables held constant, the portfolio could expect an increase or decrease in unrealized net gains and losses of \$99,607 (2024 – \$95,745).

**(i) Price Risk**

Price risk relates to the possibility that equity portfolio investments will change in fair value due to future fluctuations in market prices caused by factors specific to an individual equity investment or other factors affecting all equities traded in the market. AHS is exposed to price risk associated with the underlying equity portfolio investments held in pooled funds. If equity market indices (S&P/TSX, S&P1500 and MSCI ACWI and their sectors) declined by 10%, and all other variables are held constant, the potential loss in fair value to AHS would be approximately \$54,607 or 2.02% of total portfolio investments (2024 – \$53,723 or 2.06%).

**(ii) Interest Rate Risk**

Interest rate risk is the risk that the value of a financial instrument might be adversely affected by a change in market interest rates. AHS manages the interest rate risk exposure of its fixed income securities by management of average duration and laddered maturity dates.

AHS is exposed to interest rate risk through its investments in fixed income securities with both fixed and floating interest rates. AHS has fixed interest rate loans for all debt, thereby mitigating interest rate risk from rate fluctuations over the term of the outstanding debt. The fair value of fixed rate debt fluctuates with changes in market interest rates but the related future cash flows will not change.

In general, investment returns for fixed income securities are sensitive to changes in the level of interest rates, with longer term interest bearing securities being more sensitive to interest rate changes than shorter term bonds and money market instruments.

A 1% change in market yield relating to fixed income securities would have increased or decreased fair value by approximately \$72,416 (2024 – \$62,499).

Interest bearing securities have the following average maturity structure:

|                    | 2025 | 2024 |
|--------------------|------|------|
| Less than one year | 31%  | 33%  |
| 1 – 5 years        | 49%  | 49%  |
| 6 – 10 years       | 13%  | 10%  |
| Over 10 years      | 7%   | 8%   |

|                          | Average Effective Market Yield |       |
|--------------------------|--------------------------------|-------|
| Asset Class              | 2025                           | 2024  |
| Money market instruments | 2.81%                          | 5.06% |
| Fixed income securities  | 3.13%                          | 4.41% |

**Note 10      Financial Risk Management (continued)****(i) Foreign Currency Risk**

Foreign currency risk is the risk that the fair value of future cash flows of a financial instrument will fluctuate because of changes in foreign exchange rates. Cash and cash equivalents and portfolio investments denominated in foreign currencies are translated into Canadian dollars on a daily basis using the reporting date exchange rate. Both the realized gain/loss and remeasurement gain/loss comprise actual gains or losses on the underlying instrument as well as changes in foreign exchange rates at the time of the valuation. AHS is exposed to foreign exchange fluctuations on its cash denominated in foreign currencies. AHS is also exposed to changes in the valuation on its global equity funds attributable to fluctuations in foreign currency.

Foreign currency risk is managed by the investment policies which limit non-Canadian equities to a maximum of 10% to 45% of the total investment portfolio, depending on the policy. At March 31, 2025, investments in non-Canadian equities represented 12.9% (2024 – 11.6%) of total portfolio investments.

Foreign exchange fluctuations on cash balances are mitigated by derivatives and holding minimal foreign currency cash balances. AHS holds US dollar forward contracts to manage currency fluctuations relating to its US dollar accounts payable requirements. As at March 31, 2025, AHS held derivatives in the form of forward contracts for future settlement of \$22,000 (2024 – \$18,000). The fair value of these forward contracts as at March 31, 2025 was \$1,139 (2024 – \$434) and is included in portfolio investments (Note 11).

**(b) Credit Risk**

Credit risk is the risk of loss arising from the failure of a counterparty to fully honour its contractual obligations. The credit quality of financial assets is generally assessed by reference to external credit ratings. Credit risk can also lead to losses when issuers and debtors are downgraded by credit rating agencies. The investment policies restrict the types and proportions of eligible investments, thus mitigating AHS' exposure to credit risk.

Accounts receivable primarily consists of amounts receivable from AH, other Alberta government reporting entities, patients, other provinces and territories, and the federal government. AHS periodically reviews the collectability of its accounts receivable and establishes an allowance based on its best estimate of potentially uncollectible amounts.

The carrying amounts of financial assets represent the maximum credit exposure.

Under the investment bylaws and policies governing the consolidated investment portfolio, money market securities are limited to a rating of R1 or equivalent or higher, and no more than 10% may be invested in any one issuer unless guaranteed by the Government of Canada or a Canadian province. Investments in corporate bonds are limited to BBB or equivalent rated bonds or higher and no more than 40% of the total investment portfolio. Not more than 20% of the investment portfolio may be BBB or equivalent rated bonds. AHS holds unrated mortgage fund investments which are classified as part of AHS' fixed income securities.

The following table summarizes AHS' investment in debt securities by counterparty credit rating at March 31.

The unrated securities consist of low volatility pooled mortgages that are not rated on an active market.

| Credit Rating | 2025        | 2024        |
|---------------|-------------|-------------|
| AAA           | 56%         | 58%         |
| AA            | 15%         | 10%         |
| A             | 14%         | 17%         |
| BBB           | 11%         | 11%         |
| Unrated       | 4%          | 4%          |
|               | <b>100%</b> | <b>100%</b> |

**Note 10 Financial Risk Management (continued)****(a) Liquidity Risk**

Liquidity risk is the risk that AHS will encounter difficulty under both normal and stressed conditions in meeting obligations associated with financial liabilities that are settled by delivery of cash and cash equivalents or another financial asset. Liquidity requirements of AHS are met through funding provided by AH, income generated from portfolio investments, and by investing in liquid assets, such as money market securities, fixed income securities and equities traded in an active market that are easily sold and converted to cash. Short-term borrowing to meet financial obligations would be available through established credit facilities, which have not been drawn upon, as described in Note 19(b).

AHS issued debentures and the committed repayments with respect to these debentures are described in Note 19(c). The following are contractual maturities of the remaining financial liabilities as at March 31, 2025, based on expected undiscounted cash flows.

|                                                 | Due in less than 1 year | Due in 1-5 years | Due after 5 years |
|-------------------------------------------------|-------------------------|------------------|-------------------|
| Payroll payable and related accrued liabilities | \$ 1,326,452            | \$ -             | \$ -              |
| Trade accounts payable and accrued liabilities  | 979,321                 | -                | -                 |
| Other liabilities                               | 6,873                   | 13,323           | 4,413             |
|                                                 | <b>\$ 2,312,646</b>     | <b>\$ 13,323</b> | <b>\$ 4,413</b>   |

**Note 11 Portfolio Investments**

|                                       | 2025                |                     | 2024                |                     |
|---------------------------------------|---------------------|---------------------|---------------------|---------------------|
|                                       | Fair Value          | Cost                | Fair Value          | Cost                |
| Cash held for investing purposes      | \$ 89,207           | \$ 89,207           | \$ 131,690          | \$ 131,690          |
| Interest bearing securities:          |                     |                     |                     |                     |
| Money market securities               | 597,429             | 595,809             | 614,641             | 614,636             |
| Fixed income securities               | 1,444,907           | 1,438,535           | 1,304,133           | 1,326,783           |
|                                       | <b>2,042,336</b>    | <b>2,034,344</b>    | <b>1,918,774</b>    | <b>1,941,419</b>    |
| Equities:                             |                     |                     |                     |                     |
| Canadian equity investments and funds | 162,401             | 147,878             | 199,709             | 181,927             |
| Global equity investments and funds   | 344,898             | 302,168             | 299,807             | 224,040             |
|                                       | <b>507,299</b>      | <b>450,046</b>      | <b>499,516</b>      | <b>405,967</b>      |
| Real estate pooled funds              | <b>38,777</b>       | <b>31,069</b>       | <b>37,712</b>       | <b>30,926</b>       |
|                                       | <b>\$ 2,677,619</b> | <b>\$ 2,604,666</b> | <b>\$ 2,587,692</b> | <b>\$ 2,510,002</b> |

|                                                                                 | 2025                | 2024                |
|---------------------------------------------------------------------------------|---------------------|---------------------|
| Items at fair value                                                             |                     |                     |
| Portfolio investments designated to the fair value category                     | \$ 2,683,197        | \$ 2,509,350        |
| Portfolio investments in equity instruments that are quoted in an active market | -                   | 77,908              |
| Derivative (liability) asset, net                                               | (5,578)             | 434                 |
|                                                                                 | <b>\$ 2,677,619</b> | <b>\$ 2,587,692</b> |

As at March 31, 2025, included in portfolio investments is \$186,071 (2024 – \$188,142) that is restricted for use as per the requirements in Sections 99 and 100 of the *Insurance Act* (Alberta). Endowment principal included in portfolio investments amounts to \$109,920 (2024 – \$105,273) (Note 23).

**Note 11 Portfolio Investments (continued)**

The following are the total net remeasurement gains on portfolio investments:

|                                                                                                    | 2025             | 2024             |
|----------------------------------------------------------------------------------------------------|------------------|------------------|
| Accumulated remeasurement gains                                                                    | \$ 69,702        | \$ 64,073        |
| Restricted unrealized net gains attributable to unexpended deferred operating revenue (Note 16(b)) | 3,251            | 13,617           |
|                                                                                                    | <b>\$ 72,953</b> | <b>\$ 77,690</b> |

**Fair Value Hierarchy**

The relative reliability of data or inputs used to measure the fair value is determined based on the following fair value hierarchy:

- Level 1 – Unadjusted quoted market prices in active markets for identical assets or liabilities;
- Level 2 – Observable or corroborated inputs, other than level 1, such as quoted prices for similar assets or liabilities in inactive markets or market data for substantially the full term of the assets or liabilities; and
- Level 3 – Unobservable inputs that are supported by little or no market activity and that are significant to the fair value of the assets and liabilities.

|                                       | 2025        |                     |                  |                     |
|---------------------------------------|-------------|---------------------|------------------|---------------------|
|                                       | Level 1     | Level 2             | Level 3          | Total               |
| Cash held for investing purposes      | \$ -        | \$ 89,207           | \$ -             | \$ 89,207           |
| Interest bearing securities:          |             |                     |                  |                     |
| Money market securities               | -           | 597,429             | -                | 597,429             |
| Fixed income securities               | -           | 1,386,625           | 58,282           | 1,444,907           |
| Equities:                             |             |                     |                  |                     |
| Canadian equity investments and funds | -           | 162,401             | -                | 162,401             |
| Global equity investments and funds   | -           | 344,898             | -                | 344,898             |
| Real estate pooled funds              | -           | -                   | 38,777           | 38,777              |
|                                       | <b>\$ -</b> | <b>\$ 2,580,560</b> | <b>\$ 97,059</b> | <b>\$ 2,677,619</b> |
| Percent of total                      | 0%          | 96%                 | 4%               | 100%                |

|                                       | 2024             |                     |                  |                     |
|---------------------------------------|------------------|---------------------|------------------|---------------------|
|                                       | Level 1          | Level 2             | Level 3          | Total               |
| Cash held for investing purposes      | \$ -             | \$ 131,690          | \$ -             | \$ 131,690          |
| Interest bearing securities:          |                  |                     |                  |                     |
| Money market securities               | -                | 614,641             | -                | 614,641             |
| Fixed income securities               | -                | 1,252,966           | 51,167           | 1,304,133           |
| Equities:                             |                  |                     |                  |                     |
| Canadian equity investments and funds | 77,908           | 121,801             | -                | 199,709             |
| Global equity investments and funds   | -                | 299,807             | -                | 299,807             |
| Real estate pooled funds              | -                | -                   | 37,712           | 37,712              |
|                                       | <b>\$ 77,908</b> | <b>\$ 2,420,905</b> | <b>\$ 88,879</b> | <b>\$ 2,587,692</b> |
| Percent of total                      | 3%               | 94%                 | 3%               | 100%                |

**Note 11 Portfolio Investments (continued)****Reconciliation of Investments classified as level 3**

|                                                                               | 2025                    |                          |                  |
|-------------------------------------------------------------------------------|-------------------------|--------------------------|------------------|
|                                                                               | Fixed income securities | Real estate pooled funds | Total            |
| Beginning of year                                                             | \$ 51,167               | \$ 37,712                | \$ 88,879        |
| Purchases                                                                     | 1,735                   | -                        | 1,735            |
| Sales                                                                         | (1,126)                 | -                        | (1,126)          |
| Realized loss                                                                 | (27)                    | -                        | (27)             |
| Gain included in the Consolidated Statement of Remeasurement Gains and Losses | 2,012                   | 922                      | 2,934            |
| Transfers in <sup>(i)</sup>                                                   | 4,521                   | 143                      | 4,664            |
| <b>End of year</b>                                                            | <b>\$ 58,282</b>        | <b>\$ 38,777</b>         | <b>\$ 97,059</b> |

|                                                                                      | 2024                    |                          |                  |
|--------------------------------------------------------------------------------------|-------------------------|--------------------------|------------------|
|                                                                                      | Fixed income securities | Real estate pooled funds | Total            |
| Beginning of year                                                                    | \$ 51,325               | \$ 48,690                | \$ 100,015       |
| Purchases                                                                            | 2,015                   | -                        | 2,015            |
| Sales                                                                                | (333)                   | (11,129)                 | (11,462)         |
| Realized (loss) gain                                                                 | (21)                    | 1,684                    | 1,663            |
| Gain (loss) included in the Consolidated Statement of Remeasurement Gains and Losses | 595                     | (1,533)                  | (938)            |
| Transfers out <sup>(i)</sup>                                                         | (2,414)                 | -                        | (2,414)          |
| <b>End of year</b>                                                                   | <b>\$ 51,167</b>        | <b>\$ 37,712</b>         | <b>\$ 88,879</b> |

<sup>(i)</sup> Transfers are attributable to changes in the observability of market data.

**Note 12 Accounts Receivable**

|                                      | 2025                |                                 |                   | 2024              |
|--------------------------------------|---------------------|---------------------------------|-------------------|-------------------|
|                                      | Gross               | Allowance for Doubtful Accounts | Net               | Net (Note 29)     |
| AH operating transfers receivable    | \$ 371,315          | \$ -                            | \$ 371,315        | \$ 129,549        |
| Other capital transfers receivable   | 99,273              | -                               | 99,273            | 105,534           |
| Patient accounts receivable          | 171,921             | 45,782                          | 126,139           | 116,584           |
| Drugs rebates receivable             | 148,516             | -                               | 148,516           | 105,960           |
| AH capital transfers receivable      | 9,755               | -                               | 9,755             | 20,497            |
| Other operating transfers receivable | 107,073             | -                               | 107,073           | 89,050            |
| Other accounts receivable            | 175,817             | 58,715                          | 117,102           | 188,351           |
|                                      | <b>\$ 1,083,670</b> | <b>\$ 104,497</b>               | <b>\$ 979,173</b> | <b>\$ 755,525</b> |

Accounts receivable are unsecured and non-interest bearing. At March 31, 2024, the total allowance for doubtful accounts was \$54,010 of which \$44,350 related to patient accounts receivable.

**Note 13 Contractual Rights**

Contractual rights are rights of AHS to economic resources arising from contracts or agreements that will result in both assets and revenues in the future when the terms of those contracts or agreements are met.

Estimated amounts that will be received or receivable for each of the next five years and thereafter are as follows:

| Year ended March 31 | Operating leases | Other contracts <sup>(i)</sup> | Total      |
|---------------------|------------------|--------------------------------|------------|
| 2026                | \$ 2,424         | \$ 297,602                     | \$ 300,026 |
| 2027                | 2,194            | 133,551                        | 135,745    |
| 2028                | 1,974            | 8,594                          | 10,568     |
| 2029                | 1,283            | 7,819                          | 9,102      |
| 2030                | 1,100            | 7,625                          | 8,725      |
| Thereafter          | 1,914            | 20,344                         | 22,258     |
| March 31, 2025      | \$ 10,889        | \$ 475,535                     | \$ 486,424 |
| March 31, 2024      | \$ 13,740        | \$ 70,585                      | \$ 84,325  |

<sup>(i)</sup> Other contracts include the transition services framework agreements with Recovery Alberta and PCA of \$410,215 (2024 - \$nil) (Note 3(a)).

**Note 14 Accounts Payable and Accrued Liabilities**

|                                                 | 2025         | 2024         |
|-------------------------------------------------|--------------|--------------|
| Payroll payable and related accrued liabilities | \$ 1,326,452 | \$ 816,482   |
| Trade accounts payable and accrued liabilities  | 979,321      | 692,594      |
| Provision for unpaid claims <sup>(a)</sup>      | 158,285      | 167,598      |
| Obligations under capital leases <sup>(b)</sup> | 134,122      | 142,882      |
| Other liabilities                               | 35,059       | 36,399       |
|                                                 | \$ 2,633,239 | \$ 1,855,955 |

As at March 31, 2025, accounts payable and accrued liabilities includes payables related to the purchase of tangible capital assets of \$244,166 (2024 – \$237,885). Of these amounts, \$9,526 (2024 – \$9,166) comprise life lease deposits received from tenants of certain AHS' long term care facilities, and obligations under capital leases of \$134,122 (2024 – \$142,882).

(a) Provision for unpaid claims is an actuarial estimate of liability claims against AHS. It is influenced by factors such as historical trends involving claim payment patterns, loss payments, number of unpaid claims, claims severity and claim frequency patterns.

The provision has been actuarially estimated using the discounted value of claim liabilities using a discount rate of 4.18% (2024 – 4.37%).

(b) Obligations under capital leases include site leases with the University of Calgary, vehicle and equipment leases, site leases for ambulance services and a community care service facility.

The obligations will be settled between 2026 and 2041 and have an implicit interest rate payable ranging from 2.53% to 5.41% (2024 – 2.53% to 5.41%).

**Note 14      Accounts Payable and Accrued Liabilities (continued)**

AHS is committed to making payments for obligations under capital leases as follows:

| Year ended March 31 | Minimum Contract Payments |
|---------------------|---------------------------|
| 2026                | \$ 24,336                 |
| 2027                | 22,344                    |
| 2028                | 18,458                    |
| 2029                | 13,550                    |
| 2030                | 10,707                    |
| Thereafter          | 62,928                    |
|                     | <b>152,323</b>            |
| Less: interest      | (18,201)                  |
|                     | <b>\$ 134,122</b>         |

**Note 15      Employee Future Benefits**

|                                                    | 2025              | 2024              |
|----------------------------------------------------|-------------------|-------------------|
| Accrued vacation pay                               | \$ 621,598        | \$ 680,804        |
| Accumulating non-vesting sick leave <sup>(a)</sup> | 121,192           | 137,552           |
| SERP pension plans                                 | (33)              | 183               |
|                                                    | <b>\$ 742,757</b> | <b>\$ 818,539</b> |

**(a) Accumulating Non-Vesting Sick Leave**

Sick leave benefits are paid by AHS; there are no employee contributions and no assets set aside to support the obligation.

|                                       | 2025              | 2024              |
|---------------------------------------|-------------------|-------------------|
| Funded status – deficit               | \$ 136,098        | \$ 99,365         |
| Unamortized net actuarial (loss) gain | (14,906)          | 38,187            |
| <b>Accrued benefit liability</b>      | <b>\$ 121,192</b> | <b>\$ 137,552</b> |

Key assumptions used in the determination of the accumulating non-vesting sick leave liability are:

|                                                           | 2025       | 2024       |
|-----------------------------------------------------------|------------|------------|
| Estimated average remaining service life                  | 13 years   | 10 years   |
| Draw down rate of accumulated non-vesting sick leave bank | 24.10%     | 18.30%     |
| Discount rate – beginning of year                         | 5.00%      | 5.60%      |
| Discount rate – end of year                               | 3.63%      | 5.00%      |
| Rate of compensation increase per year                    | 2024-25    | 2023-24    |
|                                                           | 2.00%      | 2.25%      |
|                                                           | 2025-26    | 2024-25    |
|                                                           | 3.00%      | 2.00%      |
|                                                           | 2026-27    | 2025-26    |
|                                                           | 3.00%      | 2.00%      |
|                                                           | Thereafter | Thereafter |
|                                                           | 2.75%      | 2.75%      |

**Note 15 Employee Future Benefits (continued)****(b) Local Authorities Pension Plan (LAPP)****(i) AHS Participation in the LAPP**

The majority of AHS employees participate in the LAPP. AHS' employees comprise approximately 42% (2024 - 46%) of the total membership in LAPP. AHS is not responsible for future funding of the plan deficit other than through contribution increases. As AHS is exposed to the risk of contribution rate increases, the following disclosure is provided to explain this risk.

The LAPP provides for a pension of 1.4% for each year of pensionable service based on the average salary of the highest five consecutive years up to the average Canada Pension Plan's Year's Maximum Pensionable Earnings (YMPE), over the same five consecutive year period and 2.0% on the excess, subject to the maximum pension benefit limit allowed under the *Income Tax Act* (Canada). The maximum pensionable service allowable under the plan is 35 years.

**(ii) LAPP Surplus**

The LAPP carried out an actuarial valuation as at December 31, 2023 and these results were then extrapolated to December 31, 2024.

|                                        | December 31, 2024    | December 31, 2023    |
|----------------------------------------|----------------------|----------------------|
| LAPP net assets available for benefits | \$ 70,698,830        | \$ 63,337,859        |
| LAPP pension obligation                | 51,141,682           | 48,281,339           |
| <b>LAPP surplus</b>                    | <b>\$ 19,557,148</b> | <b>\$ 15,056,520</b> |

The 2025 and 2024 LAPP contribution rates are as follows:

| Calendar 2025                                                         |                                                                       | Calendar 2024                                                         |                                                                       |
|-----------------------------------------------------------------------|-----------------------------------------------------------------------|-----------------------------------------------------------------------|-----------------------------------------------------------------------|
| Employer                                                              | Employees                                                             | Employer                                                              | Employees                                                             |
| 8.45% of pensionable earnings up to the YMPE and 11.65% of the excess | 7.45% of pensionable earnings up to the YMPE and 10.65% of the excess | 8.45% of pensionable earnings up to the YMPE and 11.65% of the excess | 7.45% of pensionable earnings up to the YMPE and 10.65% of the excess |

**(c) Pension Expense**

|                                                    | 2025              | 2024              |
|----------------------------------------------------|-------------------|-------------------|
| Local Authorities Pension Plan                     | \$ 607,841        | \$ 521,199        |
| Defined contribution pension plans and group RRSPs | 39,116            | 41,331            |
| Other pension plans                                | 1,645             | 1,727             |
|                                                    | <b>\$ 648,602</b> | <b>\$ 564,257</b> |

**Note 16 Unexpended Deferred Operating Revenue**

(a) Changes in the unexpended deferred operating revenue balance are as follows:

|                                                                                                                                       | 2025              |                                 |                           |                   | 2024              |
|---------------------------------------------------------------------------------------------------------------------------------------|-------------------|---------------------------------|---------------------------|-------------------|-------------------|
|                                                                                                                                       | AH                | Other Government <sup>(i)</sup> | Donors and Non-Government | Total             | Total             |
| Balance, beginning of year                                                                                                            | \$ 225,768        | \$ 111,259                      | \$ 360,895                | \$ 697,922        | \$ 572,628        |
| Adjustment related to Ministry of Seniors, Community, and Social Services <sup>(ii)</sup>                                             | (8,034)           | 8,034                           | -                         | -                 | -                 |
| <b>Balance, beginning of year (reclassified)</b>                                                                                      | <b>217,734</b>    | <b>119,293</b>                  | <b>360,895</b>            | <b>697,922</b>    | <b>572,628</b>    |
| Received or receivable during the year                                                                                                | 1,598,305         | 168,077                         | 164,831                   | 1,931,213         | 2,016,146         |
| Unexpended deferred operating revenue returned                                                                                        | (840)             | (1,595)                         | (739)                     | (3,174)           | (10,322)          |
| Restricted investment income                                                                                                          | 1,567             | 3,467                           | 26,386                    | 31,420            | 16,860            |
| Transferred from unexpended deferred capital revenue <sup>(iii)</sup> (Note 17(a))                                                    | 8,288             | 52,282                          | 480                       | 61,050            | 105,058           |
| Transferred due to restructuring (Note 3(a))                                                                                          | (56,687)          | (66,513)                        | (1,760)                   | (124,960)         | -                 |
| Recognized as revenue                                                                                                                 | (1,640,502)       | (234,567)                       | (147,036)                 | (2,022,105)       | (1,996,431)       |
| Miscellaneous other revenue recognized                                                                                                | (1,567)           | (125)                           | (26,666)                  | (28,358)          | (15,742)          |
|                                                                                                                                       | <b>126,298</b>    | <b>40,319</b>                   | <b>376,391</b>            | <b>543,008</b>    | <b>688,197</b>    |
| Changes in unrealized net gains attributable to portfolio investments related to endowments and unexpended deferred operating revenue | 2,531             | 190                             | (13,087)                  | (10,366)          | 9,725             |
| <b>Balance, end of year</b>                                                                                                           | <b>\$ 128,829</b> | <b>\$ 40,509</b>                | <b>\$ 363,304</b>         | <b>\$ 532,642</b> | <b>\$ 697,922</b> |

- (i) The balance for other government includes \$1,626 (2024 – \$1,879) of unexpended deferred operating revenue received from government entities outside the Province of Alberta. The remaining balance in other government all relates to the Province of Alberta (Note 25).
- (ii) On December 12, 2024, the Premier of Alberta approved the transfer of the continuing care department from the Ministry of Health to the Ministry of Seniors, Community, and Social Services. A reclassification has been made to the consolidated financial statements for the year ended March 31, 2024 to reclassify the related continuing care amount from Alberta Health transfers to other government transfers to conform with 2025 presentation.
- (iii) The transfer is mainly comprised of restricted capital funding that was used for approved expenditures that did not meet the definition of a tangible capital asset.

**Note 16 Unexpended Deferred Operating Revenue (continued)**

- (b) The unexpended deferred operating revenue balance at the end of the year is externally restricted for the following purposes:

|                                                                                                                                            | 2025              |                  |                           |                   | 2024              |
|--------------------------------------------------------------------------------------------------------------------------------------------|-------------------|------------------|---------------------------|-------------------|-------------------|
|                                                                                                                                            | AH                | Other Government | Donors and Non-Government | Total             | Total (Note 29)   |
| Research and education                                                                                                                     | \$ 6,667          | \$ 1,242         | \$ 270,271                | \$ 278,180        | \$ 283,879        |
| Acute care                                                                                                                                 | 49,448            | 331              | 42,922                    | 92,701            | 153,666           |
| Support services                                                                                                                           | 6,670             | 3,410            | 43,593                    | 53,673            | 49,658            |
| Diagnostic and therapeutic services                                                                                                        | 28,341            | 1,507            | 1,264                     | 31,112            | 23,086            |
| Continuing care                                                                                                                            | -                 | 24,356           | 90                        | 24,446            | 20,246            |
| Emergency medical services                                                                                                                 | 22,341            | 1,588            | 348                       | 24,277            | 21,254            |
| Administration                                                                                                                             | 9,508             | 1,133            | 333                       | 10,974            | 8,436             |
| Population and public health                                                                                                               | 1,638             | 1,754            | 3,103                     | 6,495             | 15,121            |
| Community care                                                                                                                             | 285               | 106              | 1,795                     | 2,186             | 100,059           |
| Others individually less than \$10,000                                                                                                     | 2,597             | 2,739            | 11                        | 5,347             | 8,900             |
|                                                                                                                                            | <b>127,495</b>    | <b>38,166</b>    | <b>363,730</b>            | <b>529,391</b>    | <b>684,305</b>    |
| Unrealized net gain (loss) attributable to portfolio investments related to endowments and unexpended deferred operating revenue (Note 11) | 1,334             | 2,343            | (426)                     | 3,251             | 13,617            |
|                                                                                                                                            | <b>\$ 128,829</b> | <b>\$ 40,509</b> | <b>\$ 363,304</b>         | <b>\$ 532,642</b> | <b>\$ 697,922</b> |

**Note 17 Unexpended Deferred Capital Revenue**

- (a) Changes in the unexpended deferred capital revenue balance are as follows:

|                                                                                    | 2025              |                                 |                           |                   | 2024              |
|------------------------------------------------------------------------------------|-------------------|---------------------------------|---------------------------|-------------------|-------------------|
|                                                                                    | AH                | Other Government <sup>(i)</sup> | Donors and Non-Government | Total             | Total             |
| Balance, beginning of year                                                         | \$ 110,800        | \$ 2,524                        | \$ 100,748                | \$ 214,072        | \$ 177,901        |
| Received or receivable during the year                                             | 128,930           | 223,560                         | 82,963                    | 435,453           | 473,542           |
| Used for acquisition of land                                                       | (168)             | -                               | -                         | (168)             | -                 |
| Unexpended deferred capital revenue returned                                       | -                 | -                               | (6,887)                   | (6,887)           | (3,626)           |
| Transferred to expended deferred capital revenue (Note 18)                         | (91,399)          | (171,381)                       | (63,260)                  | (326,040)         | (327,575)         |
| Transferred to unexpended deferred operating revenue <sup>(iii)</sup> (Note 16(a)) | (8,288)           | (52,282)                        | (480)                     | (61,050)          | (105,058)         |
| Revenue recognized on settlement of asset retirement obligations (Note 20)         | (158)             | (945)                           | (376)                     | (1,479)           | (1,112)           |
| <b>Balance, end of year</b>                                                        | <b>\$ 139,717</b> | <b>\$ 1,476</b>                 | <b>\$ 112,708</b>         | <b>\$ 253,901</b> | <b>\$ 214,072</b> |

- (i) The balance for other government includes \$32 (2024 - \$nil) of unexpended deferred capital revenue received from government entities outside the Province of Alberta. The remaining balance in other government all relates to the Province of Alberta (Note 25).
- (ii) The transfer is mainly comprised of restricted capital funding of approved expenditures that did not meet the definition of a tangible capital asset.

**Note 17 Unexpended Deferred Capital Revenue (continued)**

- (a) The unexpended deferred capital revenue balance at the end of the year is externally restricted for the following purposes:

|                             | 2025              |                  |                           |                   | 2024           |
|-----------------------------|-------------------|------------------|---------------------------|-------------------|----------------|
|                             | AH                | Other Government | Donors and Non-Government | Total             | Total          |
| Equipment                   | \$ 17,932         | \$ 111           | \$ 110,999                | \$ 129,042        | \$ 105,298     |
| Information systems         | 5,278             | -                | -                         | 5,278             | 2,952          |
| Facilities and improvements | 116,507           | 1,365            | 1,709                     | 119,581           | 105,822        |
|                             | <b>\$ 139,717</b> | <b>\$ 1,476</b>  | <b>112,708</b>            | <b>\$ 253,901</b> | <b>214,072</b> |

**Note 18 Expended Deferred Capital Revenue**

Changes in the expended deferred capital revenue balance are as follows:

|                                                                   | 2025              |                                 |                           |                     | 2024                |
|-------------------------------------------------------------------|-------------------|---------------------------------|---------------------------|---------------------|---------------------|
|                                                                   | AH                | Other Government <sup>(i)</sup> | Donors and Non-Government | Total               | Total               |
| Balance, beginning of year                                        | \$ 578,509        | \$ 7,876,374                    | \$ 241,548                | \$ 8,696,431        | \$ 8,525,465        |
| Transferred from unexpended deferred capital revenue (Note 17(a)) | 91,399            | 171,381                         | 63,260                    | 326,040             | 327,575             |
| Constructed tangible capital assets on behalf of AHS              | -                 | 258,231                         | -                         | 258,231             | 198,774             |
| Contributed tangible capital assets                               | -                 | -                               | -                         | -                   | 24                  |
| Transferred to PHA's due to restructuring (Note 3(a))             | (1,303)           | (530)                           | (512)                     | (2,345)             | -                   |
| Recognized as revenue                                             | (110,130)         | (222,678)                       | (32,995)                  | (365,803)           | (355,407)           |
| <b>Balance, end of year</b>                                       | <b>\$ 558,475</b> | <b>\$ 8,082,778</b>             | <b>\$ 271,301</b>         | <b>\$ 8,912,554</b> | <b>\$ 8,696,431</b> |

- <sup>(i)</sup> The balance includes \$294 (2024 - \$228) of expended deferred capital revenue received from government entities outside of the Province of Alberta. The remaining balance in other government relates to the Province of Alberta (Note 25).

## Note 19 Debt

|                                | 2025              | 2024              |
|--------------------------------|-------------------|-------------------|
| Debentures <sup>(a)</sup> :    |                   |                   |
| Parkade loan #1                | \$ 6,014          | \$ 9,811          |
| Parkade loan #2                | 8,506             | 11,658            |
| Parkade loan #3                | 15,626            | 19,081            |
| Parkade loan #4                | 78,885            | 88,956            |
| Parkade loan #5                | 20,097            | 22,331            |
| Parkade loan #6                | 16,105            | 17,279            |
| Parkade loan #7                | 36,452            | 38,774            |
| Parkade loan #8                | 147,320           | 149,397           |
| Parkade loan #9                | 19,407            | 20,000            |
| Energy savings initiative loan | 12,911            | 14,887            |
| EMS support vehicle loan       | 14,264            | 23,639            |
|                                | <b>\$ 375,587</b> | <b>\$ 415,813</b> |

- (a) Alberta Treasury Board and Finance (TBF) is responsible for the administration of the Province's lending program.

AHS issued debentures to TBF, a related party, to finance the construction of parkades. AHS has pledged revenue derived directly or indirectly from the operations of all parking facilities being constructed, renovated, owned, and operated by AHS as security for these debentures.

AHS issued a debenture to TBF relating to an energy savings initiative. AHS has pledged the mortgage on the Royal Alexandra Hospital Lands and Alberta Hospital Lands as security for this debenture.

AHS issued a debenture to TBF relating to EMS support vehicles. AHS has pledged the vehicles as security for this debenture.

AHS is in compliance with all performance requirements relating to its debentures as at March 31, 2025.

The maturity dates and interest rates for the outstanding debentures are as follows:

|                                | Maturity Date  | Fixed Interest Rate |
|--------------------------------|----------------|---------------------|
| Parkade loan #1                | September 2026 | 4.4025%             |
| Parkade loan #2                | September 2027 | 4.3870%             |
| Parkade loan #3                | March 2029     | 4.9150%             |
| Parkade loan #4                | September 2031 | 4.9250%             |
| Parkade loan #5                | June 2032      | 4.2280%             |
| Parkade loan #6                | December 2035  | 3.6090%             |
| Parkade loan #7                | March 2038     | 2.6400%             |
| Parkade loan #8                | December 2059  | 3.6010%             |
| Parkade loan #9                | March 2044     | 5.1200%             |
| Energy savings initiative loan | December 2030  | 2.4160%             |
| EMS support vehicle loan       | September 2026 | 1.1500%             |

- (b) As at March 31, 2025, AHS has access to a \$220,000 (2024 – \$220,000) revolving demand facility with a Canadian chartered bank which may be used for operating purposes. Draws on the facility bear interest at the bank's prime rate less 0.50% per annum. As at March 31, 2025, AHS has \$nil (2024 – \$nil) drawn against this facility.

AHS also has access to a \$33,000 (2024 – \$33,000) revolving demand letter of credit facility which may be used to secure AHS' obligations to third parties. At March 31, 2025, AHS has \$3,376 (2024 – \$3,316) in a letter of credit outstanding against this facility. AHS is in compliance with the terms of the agreement relating to the letter of credit as at March 31, 2025.

**Note 19 Debt (continued)**

- (c) AHS is committed to making principal and interest payments with respect to its outstanding debt as follows:

| Year Ended March 31 | Principal         | Interest          | Total             |
|---------------------|-------------------|-------------------|-------------------|
| 2026                | \$ 41,671         | \$ 14,090         | \$ 55,761         |
| 2027                | 36,274            | 12,584            | 48,858            |
| 2028                | 28,904            | 11,236            | 40,140            |
| 2029                | 28,302            | 10,023            | 38,325            |
| 2030                | 25,125            | 8,849             | 33,974            |
| Thereafter          | 215,311           | 99,432            | 314,743           |
|                     | <b>\$ 375,587</b> | <b>\$ 156,214</b> | <b>\$ 531,801</b> |

During the year, the total interest related to debt was \$15,334 (2024 – \$15,788), comprised of capitalized interest of \$1,017 (2024 – \$nil) (Note 21(a)) and interest expense of \$14,317 (2024 – \$15,788). Accrued interest at March 31, 2025 amounted to \$2,520 (2024 – \$2,679).

**Note 20 Asset Retirement Obligations**

|                                                 | 2025              | 2024              |
|-------------------------------------------------|-------------------|-------------------|
| Asset retirement obligations, beginning of year | \$ 539,421        | \$ 583,172        |
| Liability incurred                              | -                 | -                 |
| Liability settled and adjustments               | (5,214)           | (1,978)           |
| Revision in estimates (Note 21)                 | 101,256           | (41,773)          |
| Asset retirement obligations, end of year       | <b>\$ 635,463</b> | <b>\$ 539,421</b> |

AHS has asset retirement obligations to remove hazardous asbestos fibre containing materials from its buildings. Regulations require AHS to handle and dispose of the asbestos in a prescribed manner when it is disturbed, such as when the building undergoes renovations or is demolished. Although timing of the asbestos removal is conditional on the building undergoing renovations or being demolished, regulations create an existing obligation for AHS to remove the asbestos when asset retirement activities occur.

The estimate of the liability is based primarily on asbestos abatement rates calculated using the current costs incurred as part of AHS renovation and demolition projects. Third party engineering reports, building schematics, and professional judgments were used in determining the square meters containing asbestos. A funding source for this obligation has not been determined.

The timing of settlement of asset retirement obligations is currently unknown. For the year ended March 31, 2025, a recovery of \$3,983 (2024 - \$1,978) was recognized, which includes costs incurred by AI on behalf of AHS of \$2,504 (2024 - \$866) (Note 17(a)).

## Note 21 Tangible Capital Assets

| Cost                                  | 2024                 | Additions <sup>(a)</sup> | Transfers/<br>Adjustments <sup>(i)</sup> | Transfer related to<br>restructuring (Note 3) | Disposals/<br>Write-downs | 2025                 |
|---------------------------------------|----------------------|--------------------------|------------------------------------------|-----------------------------------------------|---------------------------|----------------------|
| Facilities and improvements           | \$ 12,983,453        | \$ -                     | \$ 218,486                               | \$ -                                          | \$ -                      | \$ 13,201,939        |
| Work in progress                      | 661,491              | 525,550                  | (379,102)                                | -                                             | (8,371)                   | 799,568              |
| Equipment                             | 3,139,882            | 226,922                  | (867)                                    | (19,484)                                      | (98,264)                  | 3,248,189            |
| Information systems                   | 2,384,398            | 8,671                    | 115,330                                  | -                                             | (70,509)                  | 2,437,890            |
| Building service equipment            | 1,090,744            | -                        | 134,534                                  | -                                             | -                         | 1,225,278            |
| Land <sup>(b)</sup>                   | 121,781              | 168                      | (960)                                    | -                                             | (64)                      | 120,925              |
| Leased facilities and<br>improvements | 373,174              | 453                      | 2,413                                    | -                                             | (183)                     | 375,857              |
| Land improvements                     | 114,042              | -                        | 10,462                                   | -                                             | -                         | 124,504              |
|                                       | <b>\$ 20,868,965</b> | <b>\$ 761,764</b>        | <b>\$ 100,296</b>                        | <b>\$ (19,484)</b>                            | <b>\$ (177,391)</b>       | <b>\$ 21,534,150</b> |

| Accumulated Amortization              | 2024                 | Amortization<br>Expense | Effect of<br>Transfers/<br>Adjustments <sup>(i)</sup> | Transfer related to<br>restructuring (Note 3) | Disposals/<br>Write-downs | 2025                 |
|---------------------------------------|----------------------|-------------------------|-------------------------------------------------------|-----------------------------------------------|---------------------------|----------------------|
| Facilities and improvements           | \$ 5,151,365         | \$ 134,701              | \$ -                                                  | \$ -                                          | \$ -                      | \$ 5,286,066         |
| Work in progress                      | -                    | -                       | -                                                     | -                                             | -                         | -                    |
| Equipment                             | 2,431,494            | 170,955                 | -                                                     | (14,684)                                      | (97,339)                  | 2,490,426            |
| Information systems                   | 1,825,309            | 169,635                 | -                                                     | -                                             | (70,300)                  | 1,924,644            |
| Building service equipment            | 675,892              | 58,671                  | -                                                     | -                                             | -                         | 734,563              |
| Land <sup>(b)</sup>                   | -                    | -                       | -                                                     | -                                             | -                         | -                    |
| Leased facilities and<br>improvements | 234,030              | 16,170                  | -                                                     | -                                             | (143)                     | 250,057              |
| Land improvements                     | 83,220               | 3,907                   | -                                                     | -                                             | -                         | 87,127               |
|                                       | <b>\$ 10,401,310</b> | <b>\$ 554,039</b>       | <b>\$ -</b>                                           | <b>\$ (14,684)</b>                            | <b>\$ (167,782)</b>       | <b>\$ 10,772,883</b> |

| Cost                                  | 2023                 | Additions         | Transfers/<br>Adjustments <sup>(i)</sup> | Disposals/<br>Write-downs | 2024                 |
|---------------------------------------|----------------------|-------------------|------------------------------------------|---------------------------|----------------------|
| Facilities and improvements           | \$ 12,717,651        | \$ -              | \$ 267,024                               | \$ (1,222)                | \$ 12,983,453        |
| Work in progress                      | 665,094              | 506,115           | (509,307)                                | (411)                     | 661,491              |
| Equipment                             | 2,974,510            | 207,650           | (2,001)                                  | (40,277)                  | 3,139,882            |
| Information systems                   | 2,268,704            | 14,038            | 108,723                                  | (7,067)                   | 2,384,398            |
| Building service equipment            | 1,025,874            | -                 | 64,883                                   | (13)                      | 1,090,744            |
| Land <sup>(b)</sup>                   | 121,749              | 32                | -                                        | -                         | 121,781              |
| Leased facilities and<br>improvements | 317,743              | 33,000            | 22,431                                   | -                         | 373,174              |
| Land improvements                     | 107,568              | -                 | 6,474                                    | -                         | 114,042              |
|                                       | <b>\$ 20,198,893</b> | <b>\$ 760,835</b> | <b>\$ (41,773)</b>                       | <b>\$ (48,990)</b>        | <b>\$ 20,868,965</b> |

| Accumulated Amortization              | 2023                | Amortization<br>Expense | Effect of Transfers/<br>Adjustments | Disposals/<br>Write-downs | 2024                 |
|---------------------------------------|---------------------|-------------------------|-------------------------------------|---------------------------|----------------------|
| Facilities and improvements           | \$ 4,994,290        | \$ 158,195              | \$ -                                | \$ (1,120)                | \$ 5,151,365         |
| Work in progress                      | -                   | -                       | -                                   | -                         | -                    |
| Equipment                             | 2,298,163           | 172,250                 | -                                   | (38,919)                  | 2,431,494            |
| Information systems                   | 1,682,243           | 150,060                 | -                                   | (6,994)                   | 1,825,309            |
| Building service equipment            | 626,105             | 49,801                  | -                                   | (14)                      | 675,892              |
| Land <sup>(b)</sup>                   | -                   | -                       | -                                   | -                         | -                    |
| Leased facilities and<br>improvements | 214,316             | 19,714                  | -                                   | -                         | 234,030              |
| Land improvements                     | 80,127              | 3,093                   | -                                   | -                         | 83,220               |
|                                       | <b>\$ 9,895,244</b> | <b>\$ 553,113</b>       | <b>\$ -</b>                         | <b>\$ (47,047)</b>        | <b>\$ 10,401,310</b> |

<sup>(i)</sup> Transfers and adjustments mainly relate to reclassifications between capital asset categories and revisions to asset retirement costs of \$101,256 (2024 - \$41,773) (Note 20).

**Note 21 Tangible Capital Assets (continued)**

|                                    | Net Book Value       |                      |
|------------------------------------|----------------------|----------------------|
|                                    | 2025                 | 2024                 |
| Facilities and improvements        | \$ 7,915,873         | \$ 7,832,088         |
| Work in progress                   | 799,568              | 661,491              |
| Equipment                          | 757,763              | 708,388              |
| Information systems                | 513,246              | 559,089              |
| Building service equipment         | 490,715              | 414,852              |
| Land <sup>(b)</sup>                | 120,925              | 121,781              |
| Leased facilities and improvements | 125,800              | 139,144              |
| Land improvements                  | 37,377               | 30,822               |
|                                    | <b>\$ 10,761,267</b> | <b>\$ 10,467,655</b> |

**(a) Additions**

Additions include tangible capital assets constructed by AI on behalf of AHS of \$258,231 (2024 – \$198,774) (Note 25) and \$nil (2024 – \$24) contributed from other sources. During the year, AHS capitalized interest of \$1,017 (2024 – \$nil) (Note 19(c)) within work in progress. Capital lease additions amounted to \$15,185 (2024 – \$37,753).

**(b) Leased Land**

Land at the following sites have been leased to AHS at nominal values:

| Site                                            | Leased from                               | Lease Expiry      |
|-------------------------------------------------|-------------------------------------------|-------------------|
| Banff Health Unit                               | Banff Mineral Spring Hospital Association | January 2028      |
| Evansburg Community Health Centre               | Yellowhead County                         | April 2031        |
| Bethany Care Centre                             | Red Deer College                          | April 2034        |
| Myrnam Land                                     | Eagle Hill Foundation                     | May 2038          |
| Helipad Land at Two Hills                       | Stella Stefiuk                            | August 2041       |
| McConnell Place North                           | City of Edmonton                          | September 2044    |
| Northeast Community Health Centre               | City of Edmonton                          | February 2047     |
| Jasper Healthcare Centre                        | Parks Canada                              | March 2049        |
| Foothills Medical Centre Parkade                | University of Calgary                     | July 2054         |
| Alberta Children's Hospital                     | University of Calgary                     | December 2103     |
| Kaye Edmonton Clinic (Parcel H)                 | The University of Alberta                 | February 2109     |
| Laneway adjacent to Queen Elizabeth II Hospital | City of Grande Prairie                    | Under negotiation |
| Jasper Residential Property                     | Parks Canada                              | Under negotiation |

**(c) Leased Tangible Capital Assets**

Tangible capital assets acquired through capital leases includes vehicle leases, equipment, information systems and facilities with a cost of \$594,661 (2024 – \$579,537) and accumulated amortization of \$349,068 (2024 – \$286,709).

**(d) Asset Retirement Costs**

Facilities and improvements and Building service equipment, include \$639,561 (2024 - \$538,304) of asset retirement costs and \$344,121 (2024 - \$359,135) of related accumulated amortization.

**Note 22 Inventories of Supplies**

|                               | 2025              | 2024              |
|-------------------------------|-------------------|-------------------|
| Pharmaceuticals               | \$ 118,324        | \$ 106,759        |
| Medical and surgical supplies | 42,830            | 47,395            |
| Other <sup>(i)</sup>          | 28,283            | 43,015            |
|                               | <b>\$ 189,437</b> | <b>\$ 197,169</b> |

- (i) Other is mainly related to staff wearing apparel such as gowns and masks, housekeeping, and other supplies not included under pharmaceuticals and medical and surgical supplies.

During the year, a valuation adjustment of \$588 (2024 – \$78,997) was recognized. The prior year's valuation adjustment was due to reduced demand for inventories purchased to support public health emergencies in previous years.

**Note 23 Accumulated Surplus**

Accumulated surplus is comprised of the following:

|                                                                             | 2025                 |                                                    |                           |                                                                                                      |                     | 2024                |
|-----------------------------------------------------------------------------|----------------------|----------------------------------------------------|---------------------------|------------------------------------------------------------------------------------------------------|---------------------|---------------------|
|                                                                             | Unrestricted Surplus | Invested in Tangible Capital Assets <sup>(a)</sup> | Endowments <sup>(b)</sup> | Internally Restricted Surplus for Insurance Equity Requirements, Foundations and HBTA <sup>(c)</sup> | Total               | Total               |
| Balance, beginning of year                                                  | \$ 379,343           | \$ 696,347                                         | \$ 105,273                | \$ 123,495                                                                                           | \$ 1,304,458        | \$ 1,120,827        |
| Annual operating surplus                                                    | 71,060               | -                                                  | -                         | -                                                                                                    | 71,060              | 183,631             |
| Net investment in tangible capital assets                                   | 6,747                | (6,747)                                            | -                         | -                                                                                                    | -                   | -                   |
| Transfer of insurance equity requirements, foundations and HBTA net surplus | (235,275)            | -                                                  | -                         | 235,275                                                                                              | -                   | -                   |
| Transfer of net surplus related to asset retirement obligations             | (20,229)             | 20,229                                             | -                         | -                                                                                                    | -                   | -                   |
| Transfer of endowment contributions (Note 6)                                | (4,647)              | -                                                  | 4,647                     | -                                                                                                    | -                   | -                   |
| <b>Balance, end of year</b>                                                 | <b>\$ 196,999</b>    | <b>\$ 709,829</b>                                  | <b>\$ 109,920</b>         | <b>\$ 358,770</b>                                                                                    | <b>\$ 1,375,518</b> | <b>\$ 1,304,458</b> |

**Note 23 Accumulated Surplus (continued)****(a) Invested in Tangible Capital Assets**

Invested in tangible capital assets represents the portion of accumulated surpluses that has been invested in the acquisition or construction of AHS' assets. The balance is offset by asset retirement costs recognized to date in accumulated surplus net of related liability settlements.

Reconciliation of invested in tangible capital assets:

|                                                                        | 2025                | 2024                |
|------------------------------------------------------------------------|---------------------|---------------------|
| Tangible capital assets (Note 21)                                      | \$ 10,761,26        | \$ 10,467,655       |
| Net Book Value of Asset Retirement Costs capitalized (Note 21(d))      | (295,440)           | (179,169)           |
| Less funded by:                                                        |                     |                     |
| Expended deferred capital revenue (Note 18)                            | (8,912,55           | (8,696,431)         |
| Debt (Note 19)                                                         | (375,587)           | (415,813)           |
| Unexpended debt                                                        | 15,814              | 32,405              |
| Obligations under capital leases (Note 14)                             | (134,122)           | (142,882)           |
| Life lease deposits (Note 14)                                          | (9,526)             | (9,166)             |
|                                                                        | <b>\$ 1,049,852</b> | <b>\$ 1,056,599</b> |
| Asset retirement costs recognized net of related liability settlements | (340,023)           | (360,252)           |
|                                                                        | <b>\$ 709,829</b>   | <b>\$ 696,347</b>   |

**(b) Endowments**

Endowments represent the portion of accumulated surplus that is restricted and must be maintained in perpetuity. Transfers of endowment contributions from unrestricted surplus include \$4,647 (2024 – \$27,781) of net contributions received in the year (Note 6).

**(c) Internally Restricted Surplus for Insurance Equity Requirements and Foundations and HBTA**

Insurance equity requirements comprise surpluses of \$17,250 (2024 – \$21,042) related to equity of the LPIP mainly relating to legislative requirements per the Insurance Act. Foundations comprise surpluses amounting to \$119,325 (2024 – \$102,453) related to donations received by AHS' Controlled Foundations without external restrictions attached. Health Benefit Trust of Alberta (HBTA) comprise surpluses of \$222,195 (2024 – \$nil) related to equity representing in substance a prepayment of future premiums to the HBTA.

**Note 24 Contractual Obligations, Contingent Liabilities and Other Matters****(a) Contractual Obligations**

Contractual obligations are AHS' obligations to others that will become liabilities in the future when the terms of those contracts or agreements are met.

The estimated aggregate amount payable for the unexpired terms of these contractual obligations are as follows:

| Year ended<br>March 31 | Services <sup>(i)</sup> | Other <sup>(ii)</sup> | Operating<br>Lease | Capital<br>Projects | Total                |
|------------------------|-------------------------|-----------------------|--------------------|---------------------|----------------------|
| 2026                   | \$ 3,550,457            | \$ 476,012            | \$ 64,410          | \$ 255,715          | \$ 4,346,594         |
| 2027                   | 1,414,436               | 213,797               | 58,579             | 51,483              | 1,738,295            |
| 2028                   | 1,648,488               | 292,260               | 50,808             | 24,999              | 2,016,555            |
| 2029                   | 1,090,400               | 77,159                | 42,335             | 21,905              | 1,231,799            |
| 2030                   | 955,709                 | 47,522                | 35,114             | 13,644              | 1,051,989            |
| Thereafter             | 6,439,438               | 51,141                | 141,681            | -                   | 6,632,260            |
| March 31, 2025         | <b>\$ 15,098,928</b>    | <b>\$ 1,157,891</b>   | <b>\$ 392,927</b>  | <b>\$ 367,746</b>   | <b>\$ 17,017,492</b> |
| March 31, 2024         | \$ 16,056,317           | \$ 1,071,034          | \$ 372,841         | \$ 448,478          | \$ 17,948,670        |

(i) Service obligations mainly relate to contracts with third parties for the provision of long-term care services and home care services.

(ii) Other obligations mainly relate to contracts with third parties for maintenance, information technology services, software, equipment, and procurement of medical supplies and food.

**(b) Contingent Liabilities****(i) Legal Claims**

AHS is subject to legal claims during its normal course of business. AHS recognizes a liability when the assessment of a claim indicates that a future event is likely to confirm that a liability has been incurred at the date of the financial statements and the amount of the contingent loss can be reasonably estimated.

Accruals have been made in specific instances where it is likely that losses will be incurred based on a reasonable estimate. As at March 31, 2025, accruals have been recorded as part of the provision for unpaid claims and other liabilities (Note 14). Included in this accrual are claims in which AHS has been jointly named with the Minister. The accrual provided for these claims under the provision for unpaid claims represents AHS' portion of the liability.

AHS has been named in 195 legal claims (2024 – 234 claims) related to conditions in existence at March 31, 2025 where the likelihood of the occurrence of a future event confirming a contingent loss is not determinable. Of these, 158 claims have \$623,540 in specified amounts and 37 claims have no specified amounts (2024 – 201 claims with \$706,650 of specified claims and 33 claims with no specified amounts). The resolution of indeterminable claims may result in a liability, if any, that is different than the claimed amount.

**(ii) Collective Agreements**

AHS currently has 18 (2024 – 1) collective agreements that have expired as at March 31, 2025. Given that negotiations are ongoing or have not commenced, no additional disclosures have been made.

**(c) Other Matters**

Management and external parties are currently conducting various investigations into certain procurement practices. These investigations remain ongoing as of the date of the approval of the financial statements. Management is not aware of any material impacts to the presentation of these financial statements as at the date of approval, as these investigations continue.

**Note 25 Related Parties**

Transactions with related parties are included within these consolidated financial statements, unless otherwise stated.

The Minister appoints the Official Administrator. The viability of AHS' operations depends on transfers from AH. Transactions between AHS and AH are reported and disclosed in the Consolidated Statement of Operations, the Consolidated Statement of Financial Position, and the Notes to the Consolidated Financial Statements, and are therefore excluded from the tables below.

Related parties also include key management personnel of AHS. AHS has defined key management personnel to include those disclosed in Schedules 2A and 2B of these consolidated financial statements, except management reporting to CEO direct reports. Related party transactions with key management personnel primarily consist of compensation related payments and are undertaken on similar terms and conditions to those adopted if the entities were dealing at arm's length.

AHS is a related party with respect to those entities consolidated or included on a modified equity basis in the consolidated financial statements of the Province of Alberta. Entities consolidated or included on a modified equity basis have been grouped with their respective ministry and transactions between AHS and the other ministries, except for provincial health agencies which are disclosed separately, are recorded at their exchange amount as follows:

|                                                       | Revenues <sup>(a)</sup> |                                 | Expenses          |                   |
|-------------------------------------------------------|-------------------------|---------------------------------|-------------------|-------------------|
|                                                       | 2025                    | 2024<br>(Restated)<br>(Note 29) | 2025              | 2024              |
| Alberta Advanced Education <sup>(b)</sup>             | \$ 50,493               | \$ 57,875                       | \$ 172,515        | \$ 184,077        |
| Alberta Infrastructure <sup>(c)</sup>                 | 282,607                 | 312,158                         | 9,677             | 12,064            |
| Alberta Mental Health and<br>Addiction <sup>(d)</sup> | 483,961                 | 121,779                         | -                 | -                 |
| Other ministries <sup>(e)</sup>                       | 106,377                 | 64,122                          | 18,462            | 31,749            |
| Provincial Health Agencies <sup>(f)</sup>             | 159,304                 | -                               | -                 | -                 |
| <b>Total for the year</b>                             | <b>\$ 1,082,742</b>     | <b>\$ 555,934</b>               | <b>\$ 200,654</b> | <b>\$ 227,890</b> |

|                                                       | Receivable from   |                                 | Payable to        |                   |
|-------------------------------------------------------|-------------------|---------------------------------|-------------------|-------------------|
|                                                       | 2025              | 2024<br>(Restated)<br>(Note 29) | 2025              | 2024              |
| Alberta Advanced Education <sup>(b)</sup>             | \$ 10,287         | \$ 11,435                       | \$ 22,822         | \$ 33,399         |
| Alberta Infrastructure <sup>(c)</sup>                 | 63,950            | 69,725                          | 1,955             | 6,144             |
| Alberta Mental Health and<br>Addiction <sup>(d)</sup> | -                 | 28,419                          | -                 | 1,288             |
| Other ministries <sup>(e)</sup>                       | 87,933            | 47,928                          | 378,173           | 419,112           |
| Provincial Health Agencies <sup>(f)</sup>             | -                 | -                               | 293,694           | -                 |
| <b>Balance, end of year</b>                           | <b>\$ 162,170</b> | <b>\$ 157,507</b>               | <b>\$ 696,644</b> | <b>\$ 459,943</b> |

(a) Revenues with Province of Alberta ministries include other government transfers of \$897,739 (2024 – \$523,929), (Note 5), other income of \$184,252 (2024 – \$29,791) (Note 7), and fees and charges of \$751 (2024 – \$2,214). A substantial portion of other income is for the provision of corporate and clinical support service provided to the provincial health agencies under the transition services framework agreement.

(b) The majority of AHS' transactions with Alberta Advanced Education relate to initiatives with the University of Alberta and the University of Calgary. These initiatives include teaching, research, and program delivery. A number of physicians are employed by either AHS or the universities but perform services for both. Due to proximity of locations, some initiatives result in sharing physical space and support services. The transactions reported are a result of funding provided from one to the other and recoveries of shared costs.

**Note 25 Related Parties (continued)**

- (c) The transactions with AI relate to the construction of tangible capital assets on behalf of AHS. These transactions include operating transfers of \$61,424 (2024 – \$98,110) and recognition of expended deferred capital revenue of \$221,183 (2024 – \$214,048) relating to tangible capital assets with stipulations or external restrictions to utilize over their remaining useful lives. Not included in the table above but included in total amounts disclosed in Note 21(a) is tangible capital assets constructed by AI on behalf of AHS of \$258,231 (2024 – \$198,774).
- (d) The transactions with Alberta Mental Health and Addiction, up until the operational date of RA, related to initiatives to support Albertans experiencing addiction and mental health challenges. Subsequent to the operational date, it relates to the corporate and clinical support services being provided by AHS.
- (e) The payable transactions with other ministries include the debt payable to TBF (Note 19(a)).
- (f) The transactions with provincial health agencies relate to the services provided by AHS in accordance with the transition services agreements. Under the Transition Services Framework Agreement, AHS provides corporate and clinical support services including but not limited to nutrition, food, linen & environmental services, capital management, information technology, protective services, pharmacy services, human resources, financial operations, APL services and health information management.

At March 31, 2025, AHS has recorded deferred revenue from other ministries within the Province of Alberta, excluding AH, of \$38,883 (2024 – \$117,414) related to unexpended deferred operating revenue (Note 16(a)), \$1,444 (2024 – \$2,524) related to unexpended deferred capital revenue (Note 17(a)) and \$8,082,484 (2024 – \$7,876,146) related to expended deferred capital revenue (Note 18).

Contingent liabilities in which AHS has been jointly named with other government entities within the Province of Alberta are disclosed in Note 24.

**Note 26 Government Partnerships**

AHS has proportionately consolidated 50% of the results of NACTRC and 33.33% of the results in iRSM. AHS also proportionately consolidated its 50% ownership interests in PCNs up until February 1, 2025. Following the transfer of primary care responsibilities to PCA, effective February 1, 2025, AHS no longer holds any ownership interest in PCN's (Note 2(a)(ii) and Note 3). The following is 100% of the financial position and results of operations for AHS' government partnerships.

|                                                                             | 2025        | 2024        |
|-----------------------------------------------------------------------------|-------------|-------------|
| Financial assets (portfolio investments, accounts receivable, other assets) | \$ 1,526    | \$ 83,417   |
| Liabilities (trade accounts payable, unexpended deferred operating revenue) | 1,526       | 83,417      |
| <b>Accumulated surplus</b>                                                  | <b>\$ -</b> | <b>\$ -</b> |
| Total revenues                                                              | \$ 250,700  | \$ 279,160  |
| Total expenses                                                              | 250,700     | 279,160     |
| <b>Annual surplus</b>                                                       | <b>\$ -</b> | <b>\$ -</b> |

**Note 27**      **Trusts under Administration****(a) Health Benefit Trust of Alberta (HBTA)**

AHS is one of more than 30 participants in the HBTA and has a majority representation on the HBTA governance board. The HBTA is a formal employee life and health trust established under a Trust Agreement effective January 1, 2000. The HBTA provides health and other related employee benefits pursuant to the authorizing Trust Agreement.

The HBTA's balances as at March 31 are as follows:

|                      | 2025              | 2024              |
|----------------------|-------------------|-------------------|
| Financial assets     | \$ 365,504        | \$ 232,932        |
| Liabilities          | 209               | 23,983            |
| Net financial assets | 365,295           | 208,949           |
| Non-financial assets | -                 | -                 |
| <b>Net assets</b>    | <b>\$ 365,295</b> | <b>\$ 208,949</b> |

AHS has included in prepaid expenses \$222,195 (2024 – \$142,539) representing in substance a prepayment of future premiums to the HBTA. For the fiscal year ended March 31, 2025, AHS paid premiums of \$692,534 (2024 – \$632,746) which is approximately 88% (2024 – 98%) of the total premiums received by the HBTA.

**(b) Other Trust Funds**

AHS holds funds in trust for research and development, education, and other programs. These amounts are held and administered on behalf of others in accordance with the terms and conditions embodied in the relevant agreements with no unilateral power to change the conditions set out in the trust indenture (or agreement) and therefore are not reported in these consolidated financial statements. As at March 31, 2025, the balance of funds held in trust by AHS for research and development is \$100 (2024 – \$100).

AHS holds funds in trust from continuing care residents for personal expenses. As at March 31, 2025, the balance of these funds is \$3,024 (2024 – \$2,185). These amounts are not included in the consolidated financial statements.

AHS and a third party trustee administer the SERP in accordance with a retirement compensation arrangement trust agreement. As at March 31, 2025, there are \$24,255 in plan assets (2024 – \$25,176). These amounts are not included in the consolidated financial statements.

**Note 28**      **Segment Disclosure**

The Consolidated Schedule of Segment Disclosures – *Schedule 3* is intended to enable users to better understand the reporting entity and identify the resources allocated to the major activities of AHS.

AHS' revenues, as reported on the Consolidated Statement of Operations, are most informatively presented by source and are not reasonably assignable to the reportable segments. For each reported segment, the expenses are directly or reasonably attributable to the segment.

The segments have been selected based on the presentation that is adopted for the financial reporting, planning and budget processes, and represent the major distinguishable activities of AHS.

Segments include:

**(a) Continuing care**

Continuing care homes are comprised of Type A (formerly long-term care), Type B (formerly designated supportive living), Type C (formerly palliative and hospice care), and psychiatric care in facilities operated by AHS and contracted providers.

**Note 28     Segment Disclosure (continued)****(b) Community care**

Community care includes Type B (formerly designated supportive living), Type C (formerly palliative and hospice care), and community programs including PCN, Family Care Clinics, urgent care centres, community paramedic program, and mental health. This segment excludes community-based dialysis, oncology, and surgical services.

**(c) Home care**

Home care is comprised of home nursing and support.

**(d) Acute care**

Acute care is comprised predominantly of patient care units such as medical, surgical, intensive care, palliative care, obstetrics, pediatrics, mental health, emergency, day/night care, clinics, day surgery, and contracted surgical services. This segment also includes operating and recovery rooms.

**(e) Emergency medical services**

Emergency medical services is comprised of ground ambulance, air ambulance, patient transport, and central dispatch. AHS also supports community paramedic programs, as well as other programs that support the learning, development, quality and safety of emergency medical services professionals.

**(f) Diagnostic and therapeutic services**

Diagnostic and therapeutic services support and provide care for patients through clinical lab (both in the community and acute settings), diagnostic imaging, pharmacy, rehabilitation services such as physiotherapy, occupational therapy, respiratory therapy, and speech language **pathology**.

**(g) Population and public health**

Population and public health is comprised primarily of health promotion, disease and injury prevention, and health protection. This segment also includes immunizations, traveler's health clinics, outbreaks, screening programs, and disease surveillance. This segment excludes activities associated with treatment of communicable diseases.

**(h) Research and education**

Research and education is comprised primarily of costs pertaining to formally organized health research and graduate medical education, primarily funded by donations, and third party contributions.

**(i) Information technology**

Information technology is comprised of costs pertaining to the provision of service and consultation in the design, development, implementation of technology services and systems.

**(j) Support services**

Support services is comprised of building maintenance operations (including utilities), materials management (including purchasing, central warehousing, distribution, and sterilization), housekeeping, patient registration, health records, infection control, food services, and emergency preparedness.

**(k) Administration**

Administration is comprised of human resources, finance, communications, and general administration, as well as a share of administration of certain contracted health service providers. General administration includes senior executives and many functions such as planning and development, quality assurance, patient safety, insurance, privacy, public relations, risk management, internal audit, and legal.

## Note 29 Corresponding Amounts

Certain corresponding amounts have been reclassified to conform with 2025 presentation, with the most significant reclass pertaining to the transfer of the continuing care department. On December 12, 2024, the Premier of Alberta approved the transfer of the continuing care department from the Ministry of Health to the Ministry of Seniors, Community and Social Services. As a result, the statement of operations and the accounts receivable note for the year ending March 31, 2024, were updated to reflect this change. Specifically, \$30,966 was reclassified from AH transfers to other government transfers in the statement of operations, and \$39,000 from AH operating transfers receivable to other operating transfers receivable within accounts receivable (Note 12), to align with the 2025 presentation.

## Note 30 Subsequent Events

On November 8, 2023, the Premier of Alberta announced the restructuring of AHS, resulting in the creation of four new agencies focusing on primary care, acute care, continuing care and mental health and addiction. As of March 31, 2025, two agencies, Recovery Alberta and PCA, are operational (Note 3).

The provincial health agency responsible for acute care, ACA, was established on February 1, 2025 and became operational on April 1, 2025. ACA is accountable to the Ministry of Health. On the effective date of April 1, 2025, AHS transferred all of the powers, duties, responsibilities, and functions of AHS necessary for ACA to fulfil its responsibility for the governance and coordination of acute care health services across Alberta. ACA will continue to engage AHS as a service provider to deliver the acute care services until the Minister of Health transfers these acute care services to the new AHS PHC. The full financial effect of the restructuring, including the full impact on AHS assets, liabilities and operations is currently unknown.

The provincial health agency responsible for the continuing care sector, ALA, was established on April 1, 2025, and is expected to be operational by fall 2025. ALA is accountable to the Alberta Ministry of Seniors, Community and Social Services. The full financial effect of the restructuring, including the impact on AHS assets, liabilities and operations is currently unknown.

The *Alberta Health Services Provincial Health Corporation Regulation, Alberta Regulation 213/2024* came into effect June 1, 2025. As a result of this regulation, the entity Alberta Health Services PHC was established for the purposes of delivering health services in the acute care health services sector. The operational date and the full financial effect, including the impact on the AHS assets, liabilities and operations is currently unknown.

On October 31, 2024, the Government of Alberta enacted the Real Property Governance Act, 2024 (Alberta), as part of its efforts to centralize the management of public property to enhance accountability and transparency. Under this legislation, all freehold real property owned by AHS was transferred to AI effective April 1, 2025. Any debt associated with the freehold real property transferred to AI remains with AHS, and no amendments have been made to the terms and conditions of this debt at this time. The analysis of the full financial effect, including the impact on the AHS assets, liabilities, and operations is currently underway.

On March 10, 2025, the Government of Alberta announced that the management and oversight of emergency health services, including ground, air, dispatch and interfacility transfers, will transfer from AHS to ACA later this year. Additionally, on April 8, 2025, the Government of Alberta announced that the cancer care and organ tissue services will transfer from AHS to ACA, which will be responsible for overseeing cancer services within the province. The full financial effect, including the impact on AHS assets, liabilities, and operations is currently unknown.

The Government of Alberta also announced that a new shared services entity is expected to be established and become operational in 2025. This entity will provide corporate and support services for all four provincial health agencies, and service providers. The operational date and the full financial effect, including the impact on AHS assets, liabilities and operations is currently unknown.

**Note 30      Subsequent Events (continued)**

On May 1, 2025, the Government of Alberta introduced Bill 55, the *Health Statutes Amendment Act, 2025*. The bill proposes additional amendments to the *Provincial Health Agencies Act*, the *Public Health Act*, and other related legislation. As disclosed in public statements from the Government of Alberta, these amendments to the legislation will enable key policy shifts to support refocusing efforts, including updating the oversight and governance for health foundations and reorganize the governance and planning for public health services. The full financial effect, including the impact on AHS assets, liabilities and operations is currently unknown.

On May 16, 2025, the Government of Alberta announced that the Ministry of Health has been divided into two new ministries to improve governance and operational focus across the healthcare system. The newly established Ministry of Hospital and Surgical Health Services, responsible for acute care, including emergency medical services, cancer care, organ and tissue donation, capital planning, procurement, and oversight of ACA. The second newly established Ministry of Primary and Preventative Health Services will oversee primary care, public health, Indigenous health, health workforce and colleges regulation, pharmaceutical and supplementary benefits, diagnostic services, and oversight of PCA. Additionally, the Ministry of Seniors, Community and Social Services has been renamed to Assisted Living and Social Services to reflect its expanded responsibility for ALA.

Effective July 1, 2025, approximately 4,365 positions will be transferred from AHS to ALA and PCA. These include certain positions transitioning to ALA to support seniors health and continuing care as well positions transferring to PCA to support parts of public health, aspects of chronic disease management, and some Indigenous wellness. Medical officers of health will also transition to the Ministry of Primary and Preventative Health Services, with additional responsibilities for public health to be transferred to this Ministry in the future. Additionally, three new provincial health corporations have been established: Emergency Health Services PHC, Cancer Care PHC, and Organ and Tissue Donation and Transplantation PHC. These PHCs are expected to be operational later this year.

**Note 31      Approval of Consolidated Financial Statements**

The consolidated financial statements were approved by the Official Administrator on June 2, 2025 and submitted to the Ministry of Health

## Schedule 1 – Consolidated Schedule of Expenses By Object

For The Year Ended March 31

|                                                                                                                              | 2025                               |                      | 2024                 |
|------------------------------------------------------------------------------------------------------------------------------|------------------------------------|----------------------|----------------------|
|                                                                                                                              | Budget<br>(Note 4)<br>(Schedule 4) | Actual               | Actual               |
| Salaries and benefits                                                                                                        | \$ 9,935,928                       | \$ 10,069,576        | \$ 9,839,465         |
| Contracts with health service providers                                                                                      | 3,748,600                          | 3,434,674            | 3,466,333            |
| Contracts under the Health Facilities Act                                                                                    | 77,000                             | 67,497               | 55,824               |
| Drugs and gases <sup>(i)</sup>                                                                                               | 742,600                            | 768,984              | 738,881              |
| Medical supplies <sup>(i)</sup>                                                                                              | 731,700                            | 870,581              | 815,353              |
| Other contracted services                                                                                                    | 1,664,300                          | 1,679,236            | 1,688,175            |
| Other <sup>(a)</sup>                                                                                                         | 1,682,000                          | 1,575,782            | 1,685,083            |
| Amortization and loss on disposals/write-downs of tangible capital assets (Note 21)                                          | 561,900                            | 563,648              | 555,033              |
| Net effect of restructuring and related transactions (Note 3)                                                                | -                                  | 2,761                | -                    |
|                                                                                                                              | <b>\$ 19,144,028</b>               | <b>\$ 19,032,739</b> | <b>\$ 18,844,147</b> |
| (a) Significant amounts included in Other are:                                                                               |                                    |                      |                      |
| Equipment expense                                                                                                            | \$ 323,300                         | \$ 323,441           | \$ 295,346           |
| Utilities                                                                                                                    | 194,400                            | 173,487              | 230,705              |
| Building and ground expenses                                                                                                 | 174,800                            | 134,365              | 174,780              |
| Housekeeping, laundry and linen, staff wearing apparel, plant maintenance and biomedical engineering supplies <sup>(i)</sup> | 145,000                            | 123,452              | 163,538              |
| Building rent                                                                                                                | 126,000                            | 128,771              | 125,766              |
| Food and dietary supplies                                                                                                    | 105,100                            | 94,662               | 86,029               |
| Fundraising and grants awarded                                                                                               | 55,200                             | 77,881               | 65,734               |
| Insurance and liability claims                                                                                               | 47,800                             | 63,662               | 58,656               |
| Office supplies and courier                                                                                                  | 54,400                             | 62,735               | 64,347               |
| Minor equipment purchases                                                                                                    | 98,100                             | 59,925               | 103,351              |
| Travel                                                                                                                       | 33,000                             | 39,198               | 46,165               |
| Telecommunications                                                                                                           | 37,600                             | 31,790               | 34,250               |
| Licenses, fees and memberships                                                                                               | 24,300                             | 26,717               | 26,139               |
| Education                                                                                                                    | 15,500                             | 10,663               | 14,994               |
| Other                                                                                                                        | 247,500                            | 225,033              | 195,283              |
|                                                                                                                              | <b>\$ 1,682,000</b>                | <b>\$ 1,575,782</b>  | <b>\$ 1,685,083</b>  |

- (i) Includes inventory valuation adjustments of \$588 (2024 – \$78,997) (Note 22). The current year valuation adjustment is included in medical supplies and the prior year valuation adjustments are included in drugs and gases, medical supplies, and housekeeping, laundry and linen, staff wearing apparel, plant maintenance and biomedical engineering supplies.

## Schedule 2 – Schedules of Remuneration and Benefits

For The Year Ended March 31

### SCHEDULE 2A – OFFICIAL ADMINISTRATOR REMUNERATION FOR THE YEAR ENDED MARCH 31, 2025

|                                     | Term               | 2025<br>Remuneration | 2024<br>Remuneration |
|-------------------------------------|--------------------|----------------------|----------------------|
| <b>Official Administrator</b>       |                    |                      |                      |
| Andre Tremblay                      | Since Jan 31, 2025 | \$ -                 | \$ -                 |
| <b>Total Official Administrator</b> |                    | <b>\$ -</b>          | <b>\$ -</b>          |

Andre Tremblay was appointed to the position of Official Administrator effective January 31, 2025 per Ministerial Order 808/2025. The incumbent was not remunerated for this position.

The tenure of the Official Administrator is in lieu of a Board at AHS.

### SCHEDULE 2B – FORMER BOARD REMUNERATION FOR THE YEAR ENDED MARCH 31, 2025

|                                         | Term                         | 2025 Committees   | 2025<br>Remuneration | 2024<br>Remuneration |
|-----------------------------------------|------------------------------|-------------------|----------------------|----------------------|
| <b>Former Board Chair<sup>(f)</sup></b> |                              |                   |                      |                      |
| Angela Fong                             | Sep 23, 2024 to Jan 31, 2025 | FARC, FC, GCHRC   | \$ 75                | \$ -                 |
| Dr. Lyle Oberg                          | Nov 8, 2023 to Sep 22, 2024  | FARC, FC, GCHRC   | 188                  | 155                  |
| <b>Former Board Members</b>             |                              |                   |                      |                      |
| Sandy Edmonstone (Vice Chair)           | Nov 8, 2023 to Jan 31, 2025  | FC (Chair), GCHRC | 43                   | 14                   |
| Cynthia Farmer <sup>(g)</sup>           | Nov 8, 2023 to Jan 31, 2025  | -                 | -                    | -                    |
| Angela Fong                             | Nov 21, 2023 to Sep 22, 2024 | GCHRC (Chair)     | 14                   | 12                   |
| Paul George Haggis                      | Nov 8, 2023 to Jan 31, 2025  | FARC (Chair), FC  | 43                   | 14                   |
| Darren Hedley <sup>(g)</sup>            | Jan 23, 2025 to Jan 31, 2025 | -                 | -                    | -                    |
| Dr. Lyle Oberg                          | Sep 23, 2024 to Jan 31, 2025 | -                 | 26                   | -                    |
| Evan Romanow <sup>(g)</sup>             | Nov 8, 2023 to Jan 31, 2025  | FC                | -                    | -                    |
| Andre Tremblay <sup>(g)</sup>           | Nov 8, 2023 to Jan 8, 2025   | FARC, FC, GCHRC   | -                    | -                    |
| <b>Total Former Board</b>               |                              |                   | <b>\$ 389</b>        | <b>\$ 195</b>        |

The former Board chair and former Board members were remunerated with monthly honoraria. In addition, former Board members received remuneration for attendance at Board and committee meetings up until September 22, 2024, at which time there was a change in the compensation structure.

Board committees were established by the former Board to assist in governing AHS and overseeing the management of AHS' business and affairs.

Committee legend: FARC = Finance, Audit and Risk Committee, FC = Foundations Committee, GCHRC = Governance, Compliance and Human Resources Committee.

**SCHEDULE 2C – FORMER OFFICIAL ADMINISTRATOR / FORMER OFFICIAL ADMINISTRATOR  
COMMITTEE REMUNERATION FOR THE YEAR ENDED MARCH 31, 2025**

|                                                                                      | Term                        | 2025<br>Remuneration | 2024<br>Remuneration |
|--------------------------------------------------------------------------------------|-----------------------------|----------------------|----------------------|
| <b>Former Official Administrator</b>                                                 |                             |                      |                      |
| Dr. John Cowell                                                                      | Nov 17, 2022 to Nov 8, 2023 | \$ -                 | \$ 436               |
| <b>Former Official Administrator Committee Participants</b>                          |                             |                      |                      |
| Tara Lockyer                                                                         | Nov 24, 2022 to Nov 8, 2023 | -                    | 2                    |
| Gregory Turnbull                                                                     | Nov 24, 2022 to Nov 8, 2023 | -                    | 3                    |
| Tyler White                                                                          | Jul 12, 2023 to Nov 8, 2023 | -                    | 1                    |
| Gord Winkel                                                                          | Nov 24, 2022 to Nov 8, 2023 | -                    | 3                    |
| <b>Total Former Official Administrator / Former Official Administrator Committee</b> |                             | <b>\$ -</b>          | <b>\$ 445</b>        |

The tenure of the Official Administrator was in lieu of a Board at AHS.

Dr. John Cowell's last term as Official Administrator ended November 8, 2023 per Ministerial Order 307/2023, and did not serve in the fiscal year ended March 31, 2025.

## SCHEDULE 2D - EXECUTIVE REMUNERATION AND BENEFITS FOR THE YEAR ENDED MARCH 31, 2025

| For the Current Fiscal Year                                                                                                  | 2025               |                               |                                    |                                        |                  |                          |                  |
|------------------------------------------------------------------------------------------------------------------------------|--------------------|-------------------------------|------------------------------------|----------------------------------------|------------------|--------------------------|------------------|
|                                                                                                                              | FTE <sup>(a)</sup> | Base Salary <sup>(b, h)</sup> | Other Cash Benefits <sup>(c)</sup> | Other Non-Cash Benefits <sup>(d)</sup> | Subtotal         | Severance <sup>(e)</sup> | Total            |
| <b>Official Administrator/Formal Board Direct Reports</b>                                                                    |                    |                               |                                    |                                        |                  |                          |                  |
| Andre Tremblay – Interim President and Chief Executive Officer <sup>(l, w)</sup>                                             | 0.22               | \$ 48                         | \$ -                               | \$ 11                                  | \$ 59            | \$ -                     | \$ 59            |
| Athana Mentzelopoulos – President and Chief Executive Officer <sup>(j)</sup>                                                 | 0.78               | 454                           | -                                  | 49                                     | 503              | 583                      | 1,086            |
| Ronda White – Chief Audit Executive <sup>(k)</sup>                                                                           | 0.94               | 314                           | 2                                  | 81                                     | 397              | -                        | 397              |
| <b>CEO Direct Reports</b>                                                                                                    |                    |                               |                                    |                                        |                  |                          |                  |
| Sean Chilton – Senior VP, Clinical Operations <sup>(l, x)</sup>                                                              | 1.00               | 518                           | 3                                  | 106                                    | 627              | -                        | 627              |
| Erin O'Neill – Senior VP, Finance and Shared Services <sup>(m, w)</sup>                                                      | 0.06               | 8                             | -                                  | -                                      | 8                | -                        | 8                |
| Dr. Sid Viner – VP and Medical Director, Clinical Operations <sup>(k)</sup>                                                  | 0.84               | 433                           | 22                                 | 84                                     | 539              | -                        | 539              |
| Dr. Peter Jamieson – VP, Quality and Chief Medical Officer <sup>(n)</sup>                                                    | 0.84               | 420                           | 20                                 | 128                                    | 568              | -                        | 568              |
| Susan McGillivray – Interim VP, People and Health Professions <sup>(k)</sup>                                                 | 0.94               | 381                           | -                                  | 77                                     | 458              | -                        | 458              |
| Karen Horon – VP, Cancer Care Alberta and Clinical Support Services <sup>(k)</sup>                                           | 0.94               | 363                           | 1                                  | 91                                     | 455              | -                        | 455              |
| Brenda Hubley – Acting VP, Cancer Care Alberta and Clinical Support Services <sup>(o)</sup>                                  | 0.24               | 74                            | 8                                  | 17                                     | 99               | -                        | 99               |
| Dr. Mircea Fagarasanu – Interim VP, Provincial Clinical Excellence <sup>(p)</sup>                                            | 0.92               | 250                           | 38                                 | 70                                     | 358              | -                        | 358              |
| Natalie McMurtry – Interim VP, Provincial Clinical Excellence <sup>(q)</sup>                                                 | 0.06               | 18                            | -                                  | 4                                      | 22               | -                        | 22               |
| Thomas Mountain – Interim Chief Program Officer, Addiction and Mental Health and Correctional Health Services <sup>(r)</sup> | 0.17               | 46                            | -                                  | 14                                     | 60               | -                        | 60               |
| Kerry Bales – Chief Program Officer, Addiction and Mental Health and Correctional Health Services <sup>(s)</sup>             | 0.25               | 85                            | -                                  | 27                                     | 112              | -                        | 112              |
| Christine Myatt – Senior Program Officer, Community Engagement and Communications <sup>(t)</sup>                             | 0.90               | 183                           | -                                  | 52                                     | 235              | -                        | 235              |
| Holly Budd – Acting Senior Program Officer, Community Engagement and Communications <sup>(u)</sup>                           | 0.04               | 8                             | 2                                  | 1                                      | 11               | -                        | 11               |
| Michael Lam – Interim VP, Corporate Services and Chief Financial Officer <sup>(k)</sup>                                      | 0.94               | 329                           | 1                                  | 73                                     | 403              | -                        | 403              |
| Andrea Beckwith-Ferraton – Interim General Counsel and Corporate Secretary <sup>(k)</sup>                                    | 0.94               | 228                           | 25                                 | 79                                     | 332              | -                        | 332              |
| Tracy Chalaturnyk – Transition Lead <sup>(v)</sup>                                                                           | 0.92               | 250                           | 38                                 | 52                                     | 340              | -                        | 340              |
| <b>Total Executive</b>                                                                                                       | <b>11.94</b>       | <b>\$ 4,410</b>               | <b>\$ 160</b>                      | <b>\$ 1,016</b>                        | <b>\$ 5,586</b>  | <b>\$ 583</b>            | <b>\$ 6,169</b>  |
|                                                                                                                              |                    |                               |                                    |                                        |                  |                          |                  |
| <b>Management Reporting to CEO Direct Reports</b>                                                                            | <b>51.21</b>       | <b>\$ 13,663</b>              | <b>\$ 451</b>                      | <b>\$ 2,731</b>                        | <b>\$ 16,845</b> | <b>\$ 980</b>            | <b>\$ 17,825</b> |

**SCHEDULE 2D - EXECUTIVE REMUNERATION AND BENEFITS FOR THE YEAR ENDED MARCH 31, 2025 (CONTINUED)**

| For the Prior Fiscal Year                                                                                                            | 2024               |                               |                                    |                                        |                  |                          |                  |
|--------------------------------------------------------------------------------------------------------------------------------------|--------------------|-------------------------------|------------------------------------|----------------------------------------|------------------|--------------------------|------------------|
|                                                                                                                                      | FTE <sup>(a)</sup> | Base Salary <sup>(b, h)</sup> | Other Cash Benefits <sup>(c)</sup> | Other Non-Cash Benefits <sup>(d)</sup> | Subtotal         | Severance <sup>(e)</sup> | Total            |
| <b>Board/Former Official Administrator Direct Reports</b>                                                                            |                    |                               |                                    |                                        |                  |                          |                  |
| Athana Mentzelopoulos – President and Chief Executive Officer                                                                        | 0.31               | \$ 179                        | \$ -                               | \$ 88                                  | \$ 267           | \$ -                     | \$ 267           |
| Sean Chilton – Interim President and Chief Executive Officer                                                                         | 0.06               | 35                            | -                                  | 6                                      | 41               | -                        | 41               |
| Mauro Chies – President and Chief Executive Officer                                                                                  | 0.63               | 374                           | -                                  | 85                                     | 459              | 1,386                    | 1,845            |
| Ronda White – Chief Audit Executive                                                                                                  | 1.00               | 313                           | 2                                  | 73                                     | 388              | -                        | 388              |
| Athana Mentzelopoulos – Official Administrator Advisor/ Provisional Lead, Emergency Medical Services                                 | 0.33               | 119                           | -                                  | 44                                     | 163              | -                        | 163              |
| <b>CEO Direct Reports</b>                                                                                                            |                    |                               |                                    |                                        |                  |                          |                  |
| Sean Chilton – Interim VP and Chief Operating Officer, Clinical Operations/VP, People, Health Professions and Information Technology | 0.94               | 410                           | 19                                 | 102                                    | 531              | -                        | 531              |
| Lori Anderson – Acting VP and Chief Operating Officer, Clinical Operations                                                           | 0.03               | 10                            | 3                                  | 2                                      | 15               | -                        | 15               |
| Deb Gordon – VP and Chief Operating Officer, Clinical Operations                                                                     | 0.52               | 212                           | -                                  | 7                                      | 219              | 970                      | 1,189            |
| Dr. Sid Viner – VP and Medical Director, Clinical Operations                                                                         | 0.90               | 436                           | 22                                 | 102                                    | 560              | -                        | 560              |
| Dr. Peter Jamieson – Interim VP, Quality and Chief Medical Officer                                                                   | 0.33               | 177                           | -                                  | -                                      | 177              | -                        | 177              |
| Dr. Francois Belanger – VP, Quality and Chief Medical Officer                                                                        | 0.63               | 316                           | -                                  | 85                                     | 401              | 1,071                    | 1,472            |
| Susan McGillivray – Interim VP, People and Health Professions                                                                        | 0.60               | 229                           | -                                  | 61                                     | 290              | -                        | 290              |
| Karen Horon – VP, Cancer Care Alberta and Clinical Support Services                                                                  | 1.00               | 331                           | 1                                  | 96                                     | 428              | -                        | 428              |
| Natalie McMurtry – Interim VP, Provincial Clinical Excellence                                                                        | 0.81               | 219                           | -                                  | 48                                     | 267              | -                        | 267              |
| Dr. Braden Manns – Interim VP, Provincial Clinical Excellence                                                                        | 0.19               | 107                           | 2                                  | 7                                      | 116              | -                        | 116              |
| Kerry Bales – Chief Program Officer, Addiction and Mental Health and Correctional Health Services                                    | 1.00               | 328                           | -                                  | 99                                     | 427              | -                        | 427              |
| Holly Budd – Acting Senior Program Officer, Community Engagement and Communications                                                  | 0.33               | 60                            | 12                                 | 18                                     | 90               | -                        | 90               |
| Gail Fredrickson – Interim VP, Community Engagement and Communications                                                               | 0.07               | 18                            | 2                                  | 3                                      | 23               | -                        | 23               |
| Colleen Turner – VP, Community Engagement and Communications                                                                         | 0.60               | 233                           | -                                  | 103                                    | 336              | 792                      | 1,128            |
| Michael Lam – Interim VP, Corporate Services and Chief Financial Officer                                                             | 0.33               | 97                            | 12                                 | 29                                     | 138              | -                        | 138              |
| Colleen Purdy – VP, Corporate Services and Chief Financial Officer                                                                   | 0.63               | 273                           | -                                  | 57                                     | 330              | 504                      | 834              |
| Andrea Beckwith-Ferraton – Interim General Counsel and Corporate Secretary                                                           | 0.37               | 86                            | 10                                 | 23                                     | 119              | -                        | 119              |
| Tina Giesbrecht – General Counsel                                                                                                    | 0.63               | 165                           | 5                                  | 49                                     | 219              | 356                      | 575              |
| Geoffrey Pradella – Chief Strategy Officer                                                                                           | 0.35               | 105                           | -                                  | 26                                     | 131              | 144                      | 275              |
| <b>Total Executive</b>                                                                                                               | <b>12.59</b>       | <b>\$ 4,832</b>               | <b>\$ 90</b>                       | <b>\$ 1,213</b>                        | <b>\$ 6,135</b>  | <b>\$ 5,223</b>          | <b>\$ 11,358</b> |
| <b>Management Reporting to CEO Direct Reports</b>                                                                                    |                    |                               |                                    |                                        |                  |                          |                  |
|                                                                                                                                      | <b>59.68</b>       | <b>\$ 14,938</b>              | <b>\$ 468</b>                      | <b>\$ 2,844</b>                        | <b>\$ 18,250</b> | <b>\$ 388</b>            | <b>\$ 18,638</b> |

## SCHEDULE 2E - EXECUTIVE SUPPLEMENTAL PENSION PLAN AND SUPPLEMENTAL EXECUTIVE RETIREMENT PLAN

Certain employees will receive retirement benefits that supplement the benefits limited under the registered plans for service. The Supplemental Pension Plan (SPP) is a defined contribution plan and the Supplemental Executive Retirement Plan (SERP) is a defined benefit plan. The amounts in this table represent the total SPP and SERP benefits earned by the individual during the fiscal year. The current period benefit costs for SPP and the other costs for SERP included in other non-cash benefits disclosed in Schedule 2D are prorated for the period of time the individual was in their position directly reporting to the Official Administrator/former Board or directly reporting to the President and Chief Executive Officer. Only individuals holding a position directly reporting to the Official Administrator/former Board or President and Chief Executive Officer during the current fiscal year are disclosed.

|                                                                                                                                          | 2025                                                 |                               |       | 2024  |                                                                                      |                                             |                                                                                      |   |
|------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------|-------------------------------|-------|-------|--------------------------------------------------------------------------------------|---------------------------------------------|--------------------------------------------------------------------------------------|---|
|                                                                                                                                          | SPP                                                  | SERP                          |       |       |                                                                                      |                                             |                                                                                      |   |
|                                                                                                                                          | Current<br>Period<br>Benefit<br>Costs <sup>(1)</sup> | Other<br>Costs <sup>(2)</sup> | Total | Total | Account Balance <sup>(3)</sup> or<br>Accrued Benefit<br>Obligation<br>March 31, 2024 | Change<br>During the<br>Year <sup>(4)</sup> | Account Balance <sup>(3)</sup> or<br>Accrued Benefit<br>Obligation<br>March 31, 2025 |   |
| Andre Tremblay - Interim President and<br>Chief<br>Executive Officer <sup>(5)</sup>                                                      | \$ -                                                 | \$ -                          | \$ -  | \$ -  | \$ -                                                                                 | \$ -                                        | \$ -                                                                                 | - |
| Athana Mentzelopoulos - President and<br>Chief<br>Executive Officer <sup>(6)</sup>                                                       | -                                                    | -                             | -     | 14    | 14                                                                                   | (14)                                        | -                                                                                    | - |
| Ronda White - Chief Audit Executive                                                                                                      | 16                                                   | -                             | 16    | 14    | 191                                                                                  | 50                                          | 241                                                                                  |   |
| Sean Chilton – Senior VP, Clinical<br>Operations                                                                                         | 38                                                   | -                             | 38    | 30    | 325                                                                                  | 94                                          | 419                                                                                  |   |
| Erin O'Neill - Senior VP, Finance and<br>Shared<br>Services <sup>(5)</sup>                                                               | -                                                    | -                             | -     | -     | -                                                                                    | -                                           | -                                                                                    | - |
| Dr. Sid Viner - VP and Medical Director,<br>Clinical<br>Operations                                                                       | 36                                                   | -                             | 36    | 31    | 203                                                                                  | 80                                          | 283                                                                                  |   |
| Dr. Peter Jamieson - Interim VP, Quality<br>and Chief<br>Medical Officer                                                                 | 33                                                   | -                             | 33    | -     | -                                                                                    | 33                                          | 33                                                                                   |   |
| Susan McGillivray - Interim VP, People and<br>Health<br>Professions                                                                      | -                                                    | (1)                           | (1)   | (1)   | 149                                                                                  | 8                                           | 157                                                                                  |   |
| SERP                                                                                                                                     | -                                                    | -                             | -     | -     | -                                                                                    | -                                           | -                                                                                    | - |
| SPP                                                                                                                                      | 24                                                   | -                             | 24    | 18    | 168                                                                                  | 27                                          | 195                                                                                  |   |
| Karen Horon - VP, Cancer Care Alberta and<br>Clinical Support Services                                                                   | 22                                                   | -                             | 22    | 16    | 68                                                                                   | 27                                          | 95                                                                                   |   |
| Brenda Hubley – Acting VP, Cancer Care<br>Alberta<br>and Clinical Support Services                                                       | 12                                                   | -                             | 12    | 10    | 60                                                                                   | 22                                          | 82                                                                                   |   |
| Dr. Mircea Fagarasanu - Interim VP,<br>Provincial<br>Clinical Excellence                                                                 | 8                                                    | -                             | 8     | 8     | 28                                                                                   | 14                                          | 42                                                                                   |   |
| Natalie McMurtry - Interim VP, Provincial<br>Clinical<br>Excellence                                                                      | 12                                                   | -                             | 12    | 7     | 13                                                                                   | 15                                          | 28                                                                                   |   |
| Thomas Mountain - Interim Chief Program<br>Officer,<br>Addiction and Mental Health and<br>Correctional<br>Health Services <sup>(7)</sup> | 3                                                    | -                             | 3     | -     | -                                                                                    | -                                           | -                                                                                    | - |
| Kerry Bales - Chief Program Officer,<br>Addiction<br>and Mental Health and Correctional Health<br>Services <sup>(7)</sup>                | 10                                                   | -                             | 10    | 16    | 251                                                                                  | (251)                                       | -                                                                                    | - |
| Christine Myatt - Senior Program Officer,<br>Community Engagement and<br>Communications                                                  | -                                                    | -                             | -     | -     | -                                                                                    | -                                           | -                                                                                    | - |

**SCHEDULE 2E - EXECUTIVE SUPPLEMENTAL PENSION PLAN AND SUPPLEMENTAL  
EXECUTIVE RETIREMENT PLAN (CONTINUED)**

|                                                                                                          | 2025                                                 |                               |       | 2024  |                                                                                      |                                             |                                                                                      |
|----------------------------------------------------------------------------------------------------------|------------------------------------------------------|-------------------------------|-------|-------|--------------------------------------------------------------------------------------|---------------------------------------------|--------------------------------------------------------------------------------------|
|                                                                                                          | SPP                                                  | SERP                          |       |       |                                                                                      |                                             |                                                                                      |
|                                                                                                          | Current<br>Period<br>Benefit<br>Costs <sup>(1)</sup> | Other<br>Costs <sup>(2)</sup> | Total | Total | Account Balance <sup>(3)</sup> or<br>Accrued Benefit<br>Obligation<br>March 31, 2024 | Change<br>During the<br>Year <sup>(4)</sup> | Account Balance <sup>(3)</sup> or<br>Accrued Benefit<br>Obligation<br>March 31, 2025 |
| Holly Budd - Acting Senior Program Officer,<br>Community Engagement and<br>Communications <sup>(8)</sup> | \$ -                                                 | \$ -                          | \$ -  | \$ -  | \$ -                                                                                 | \$ -                                        | \$ -                                                                                 |
| Michael Lam - Interim VP, Corporate<br>Services and<br>Chief Financial Officer                           | 16                                                   | -                             | 16    | 12    | 101                                                                                  | 39                                          | 140                                                                                  |
| Andrea Beckwith-Ferraton - Interim General<br>Counsel and Corporate Secretary                            | 5                                                    | -                             | 5     | 4     | 48                                                                                   | 16                                          | 64                                                                                   |
| Tracy Chalaturnyk - Transition Lead                                                                      | 8                                                    | -                             | 8     | 8     | 124                                                                                  | 30                                          | 154                                                                                  |

- (1) The SPP current period benefit costs are AHS contributions earned in the period.
- (2) Other SERP costs include retirement benefits, interest expense on the obligations, and amortization of actuarial gains and losses, offset by the expected return on the plan's assets. AHS uses the straight-line method to amortize actuarial gains and losses over the expected average remaining service life of the plan members.
- (3) The account balance represents the total cumulative earned contributions to the SPP as well as cumulative investment gains or losses on the contributions.
- (4) Changes in the accrued benefit obligation include current period benefit cost, interest accruing on the obligations and the amortization of any actuarial gains or losses in the period. Changes in the account balance include the current benefit costs and investment gains or losses related to the account.
- (5) The incumbent is on temporary secondment from the Government of Alberta, and therefore not eligible for enrollment in the SPP.
- (6) The SPP had not fully vested at the time of the incumbent's departure, and as a result no current period benefit costs were incurred.
- (7) The incumbent's account balance transitioned to Recovery Alberta effective September 1, 2024.
- (8) The incumbent's pensionable earnings were below the threshold for enrollment in the SPP.

## FOOTNOTES TO THE SCHEDULES OF REMUNERATION AND BENEFITS FOR THE YEAR ENDED MARCH 31, 2025

### Definitions

- a. For this schedule, full time equivalents (FTE) are determined by actual hours earned divided by 2,022.75 annual base hours (2024 – 2,030.50 hours).
- b. Base salary is regular salary and includes all payments earned related to actual hours earned other than those reported as other cash benefits.
- c. Other cash benefits include, as applicable, honoraria, acting pay, membership fees, and lump sum payments. Relocation expenses are excluded from compensation disclosure as they are considered to be recruiting costs to AHS and not part of compensation unless related to severance. Expense reimbursements are also excluded from compensation disclosure except where the expenses meet the definition of the other cash benefits listed above.
- d. Other non-cash benefits include:
  - Employer's current period benefit costs and other costs of supplemental pension plan and supplemental executive retirement plans as defined in Schedule 2E
  - Employer's share of employee benefit contributions and payments made on behalf of employees including pension, health care, dental and vision coverage, out-of-country medical benefits, group life insurance, accidental disability and dismemberment insurance, and long and short-term disability plans
  - Vacation accruals, and
  - Employer's share of the cost of additional benefits including sabbaticals or other special leave with pay.
- e. Severance includes direct or indirect payments to individuals upon termination which are not included in other cash benefits or other non-cash benefits.

### Former Board

- f. The former Board Chairs were Ex-Officio members on all former Board committees.
- g. These individuals did not claim honoraria.

### Executive

- h. Base salary reported for executives are the actual payments earned during the year, and is therefore contingent on the number of AHS' work days in the year. For the year ended March 31, 2025, the number of work days at AHS was 261 (2024 – 260 work days).
- i. Andre Tremblay was appointed to the position of Interim President and Chief Executive Officer effective January 8, 2025. The incumbent was also appointed Official Administrator effective January 31, 2025. The incumbent is on temporary secondment from the Government of Alberta and received remuneration and benefits from AHS commencing February 19, 2025.
- j. Athana Mentzelopoulos held the position of President and Chief Executive Officer until January 8, 2025 at which time she left AHS. The incumbent received salary and other accrued entitlements to the date of departure. The reported severance included 12 months base salary at the rate in effect at the date of departure. In addition, the incumbent received a vacation payout of \$108 for unused accrued vacation at the time of departure; accrued vacation has been recorded in their compensation as a non-cash benefit in the period it was earned. The reported severance is still subject to change.
- k. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 10, 2025.
- l. Sean Chilton held the position of Interim VP and Chief Operating Officer, Clinical Operations until February 26, 2025 at which time he was appointed to Senior Vice President, Clinical Operations.
- m. Erin O'Neill was appointed to the position of Senior Vice President, Finance and Shared Services effective March 10, 2025. The incumbent is on temporary secondment from the Government of Alberta and received the reported remuneration from AHS in addition to her Government of Alberta compensation.
- n. Dr. Peter Jamieson held the position of Interim Vice President, Quality and Chief Medical Officer until April 15, 2024 at which time he was appointed to Vice President, Quality and Chief Medical Officer. The incumbent's remuneration while Interim Vice President, Quality and Chief Medical Officer was as per the terms of a Medical Administrative Services Agreement. The incumbent became an employee of AHS with his appointment as Vice President, Quality and Chief Medical Officer. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 10, 2025.

## FOOTNOTES TO THE SCHEDULES OF REMUNERATION AND BENEFITS FOR THE YEAR ENDED MARCH 31, 2025 (CONTINUED)

- o. Brenda Hubley held the position of Chief Program Officer, Cancer Care Alberta until October 16, 2024 at which time she was appointed to Acting Vice President, Cancer Care Alberta and Clinical Support Services and became a direct report to the President and Chief Executive Officer. This appointment was to provide coverage for a leave of absence by Karen Horon. The incumbent held the position of Acting Vice President, Cancer Care Alberta and Clinical Support Services until January 10, 2025 at which time the incumbent resumed the role of Chief Program Officer, Cancer Care Alberta and was no longer a direct report to the President and Chief Executive Officer.
- p. Dr. Mircea Fagarasanu held the position of Senior Program Officer, Workplace Health and Safety until April 8, 2024 at which time he was appointed to Interim Vice President, Provincial Clinical Excellence and became a direct report to the President and Chief Executive Officer. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 10, 2025.
- q. Natalie McMurtry was seconded to the Government of Alberta for a two-year term effective April 22, 2024. During this secondment, the incumbent is on leave from all duties at AHS and ceases to be a direct report to the President and Chief Executive Officer at AHS.
- r. Thomas Mountain held the position of Lead, Addiction and Mental Health Operations until July 1, 2024 at which time he was appointed to Interim Chief Program Officer, Addiction and Mental Health and Correctional Health Services and became a direct report to the President and Chief Executive Officer. The incumbent's position transitioned to Recovery Alberta effective September 1, 2024 as per Ministerial Order 801/2024.
- s. Kerry Bales held the position of Chief Program Officer, Addiction and Mental Health and Correctional Health Services until June 30, 2024 at which time he left AHS to assume the position of Chief Executive Officer for Recovery Alberta. In addition, the incumbent received a vacation payout of \$95 for unused accrued vacation at the time of departure; accrued vacation has been recorded in their compensation as a non-cash benefit in the period it was earned.
- t. Christine Myatt was appointed to the position of Senior Program Officer, Community Engagement and Communications effective April 16, 2024. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 10, 2025.
- u. Holly Budd held the position of Acting Senior Program Officer, Community Engagement and Communications until April 16, 2024 at which time she resumed the role of Executive Director, Communications and was no longer a direct report to the President and Chief Executive Officer.
- v. Tracy Chalaturnyk was appointed to the temporary role of Transition Lead effective April 8, 2024 and became a direct report to the President and Chief Executive Officer. These duties are in addition to her position as Senior Program Officer, Human Resource Business Partnerships. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 10, 2025.

### Termination Obligations

- w. There is no severance associated with the secondment agreement. Upon termination of the secondment agreement, the incumbent is to return to the incumbent's regular position at the Government of Alberta.
- x. In the case of termination without just cause by AHS, the incumbent shall receive severance pay equal to 24 months' salary and benefits.

## Schedule 3 - Consolidated Schedule of Segment Disclosures

FOR THE YEAR ENDED MARCH 31

|                                         | 2025                  |                                         |                                           |                   |                   |                           |                     |                                                                            |                                         |                      |
|-----------------------------------------|-----------------------|-----------------------------------------|-------------------------------------------|-------------------|-------------------|---------------------------|---------------------|----------------------------------------------------------------------------|-----------------------------------------|----------------------|
|                                         | Salaries and benefits | Contracts with health service providers | Contracts under the Health Facilities Act | Drugs and gases   | Medical supplies  | Other contracted services | Other               | Amortization and loss on disposals/ write-downs of tangible capital assets | Net effect of restructuring transaction | Total                |
| Continuing care                         | \$ 376,905            | \$ 1,044,780                            | \$ -                                      | \$ 7,645          | \$ 6,733          | \$ 27,351                 | \$ 17,655           | \$ 3,632                                                                   | \$ -                                    | \$ 1,484,701         |
| Community care                          | 695,117               | 877,022                                 | -                                         | 5,245             | 5,168             | 51,958                    | 32,138              | 948                                                                        | -                                       | 1,667,596            |
| Home care                               | 406,715               | 370,276                                 | -                                         | 323               | 12,131            | 150,055                   | 19,163              | 617                                                                        | -                                       | 959,280              |
| Acute care                              | 3,476,632             | 494,095                                 | 67,497                                    | 699,820           | 524,880           | 724,403                   | 152,790             | 68,336                                                                     | -                                       | 6,208,453            |
| Emergency medical services              | 382,817               | 249,786                                 | -                                         | 3,470             | 5,732             | 3,416                     | 42,991              | 21,551                                                                     | -                                       | 709,763              |
| Diagnostic and therapeutic services     | 2,001,386             | 128,549                                 | -                                         | 45,014            | 298,453           | 404,704                   | 106,987             | 53,372                                                                     | -                                       | 3,038,465            |
| Population and public health            | 367,386               | 21,726                                  | -                                         | 5,728             | 10,932            | 8,553                     | 16,694              | 678                                                                        | -                                       | 431,697              |
| Research and education                  | 208,308               | 2,345                                   | -                                         | 91                | 547               | 122,274                   | 41,725              | 51                                                                         | -                                       | 375,341              |
| Information technology                  | 376,215               | 1,963                                   | -                                         | -                 | (203)             | 25,746                    | 240,682             | 169,223                                                                    | -                                       | 813,626              |
| Support services                        | 1,363,190             | 200,698                                 | -                                         | 1,648             | 6,120             | 114,122                   | 835,067             | 243,897                                                                    | -                                       | 2,764,742            |
| Administration                          | 414,905               | 43,434                                  | -                                         | -                 | 88                | 46,654                    | 69,890              | 1,343                                                                      | -                                       | 576,314              |
| Net effect of restructuring transaction | -                     | -                                       | -                                         | -                 | -                 | -                         | -                   | -                                                                          | 2,761                                   | 2,761                |
| <b>Total</b>                            | <b>\$ 10,069,576</b>  | <b>\$ 3,434,674</b>                     | <b>\$ 67,497</b>                          | <b>\$ 768,984</b> | <b>\$ 870,581</b> | <b>\$ 1,679,236</b>       | <b>\$ 1,575,782</b> | <b>\$ 563,648</b>                                                          | <b>\$ 2,761</b>                         | <b>\$ 19,032,739</b> |

**SCHEDULE 3 - CONSOLIDATED SCHEDULE OF SEGMENT DISCLOSURES  
(CONTINUED)  
FOR THE YEAR ENDED MARCH 31**

|                              | 2024                  |                                         |                                           |                   |                   |                           |                     |                                                                           |                      |
|------------------------------|-----------------------|-----------------------------------------|-------------------------------------------|-------------------|-------------------|---------------------------|---------------------|---------------------------------------------------------------------------|----------------------|
|                              | Salaries and benefits | Contracts with health service providers | Contracts under the Health Facilities Act | Drugs and gases   | Medical supplies  | Other contracted services | Other               | Amortization and loss on disposals/write-downs of tangible capital assets | Total                |
| Continuing care              | \$ 340,236            | \$ 951,304                              | \$ -                                      | \$ 7,566          | \$ 7,657          | \$ 42,481                 | \$ 23,068           | \$ 3,048                                                                  | \$ 1,375,360         |
| Community care               | 927,558               | 945,293                                 | -                                         | 9,574             | 5,428             | 53,219                    | 41,622              | 934                                                                       | 1,983,628            |
| Home care                    | 370,243               | 358,439                                 | -                                         | 199               | 11,593            | 83,992                    | 18,683              | 560                                                                       | 843,709              |
| Acute care                   | 3,427,032             | 462,215                                 | 55,824                                    | 654,905           | 472,626           | 744,026                   | 202,602             | 65,645                                                                    | 6,084,875            |
| Emergency medical services   | 357,098               | 230,011                                 | -                                         | 2,800             | 5,952             | 3,869                     | 49,822              | 16,402                                                                    | 665,954              |
| Diagnostic and therapeutic   | 1,820,366             | 264,783                                 | -                                         | 35,072            | 256,841           | 368,902                   | 120,658             | 66,162                                                                    | 2,932,784            |
| Population and public health | 351,956               | 24,052                                  | -                                         | 5,634             | 23,861            | 9,609                     | 23,448              | 613                                                                       | 439,173              |
| Research and education       | 194,329               | 3,229                                   | -                                         | 108               | 453               | 119,400                   | 35,066              | 122                                                                       | 352,707              |
| Information technology       | 358,249               | 1,951                                   | -                                         | -                 | (32)              | 42,702                    | 242,264             | 148,641                                                                   | 793,775              |
| Support services             | 1,279,425             | 185,744                                 | -                                         | 22,996            | 30,828            | 164,998                   | 864,210             | 250,171                                                                   | 2,798,372            |
| Administration               | 412,973               | 39,312                                  | -                                         | 27                | 146               | 54,977                    | 63,640              | 2,735                                                                     | 573,810              |
| <b>Total</b>                 | <b>\$ 9,839,465</b>   | <b>\$ 3,466,333</b>                     | <b>\$ 55,824</b>                          | <b>\$ 738,881</b> | <b>\$ 815,353</b> | <b>\$ 1,688,175</b>       | <b>\$ 1,685,083</b> | <b>\$ 555,033</b>                                                         | <b>\$ 18,844,147</b> |

## Schedule 4 – Consolidated Schedule of The Revised Budget

| Reconciliation of the Current Year presentation for the Budget<br>FOR THE YEAR ENDED MARCH 31, 2025 |                                                         |                                                                                 |                                                                         |                                          |
|-----------------------------------------------------------------------------------------------------|---------------------------------------------------------|---------------------------------------------------------------------------------|-------------------------------------------------------------------------|------------------------------------------|
|                                                                                                     | 2025<br>Original<br>Budget as<br>currently<br>presented | Transfer of mental<br>health and addiction<br>operations to<br>Recovery Alberta | Transfer of<br>primary care<br>operations to<br>Primary Care<br>Alberta | 2025<br>Revised<br>Budget <sup>(i)</sup> |
| <b>Revenues:</b>                                                                                    |                                                         |                                                                                 |                                                                         |                                          |
| Alberta Health transfers                                                                            |                                                         |                                                                                 |                                                                         |                                          |
| Base operating                                                                                      | \$ 14,602,924                                           | \$ (22,292)                                                                     | (25,525)                                                                | \$ 14,555,107                            |
| Other operating                                                                                     | 1,653,528                                               | (505)                                                                           | (21,728)                                                                | 1,631,295                                |
| Recognition of expended deferred capital revenue                                                    | 102,100                                                 | -                                                                               | -                                                                       | 102,100                                  |
| Other government transfers                                                                          | 1,652,176                                               | (773,948)                                                                       | (2,249)                                                                 | 875,979                                  |
| Fees and charges                                                                                    | 621,000                                                 | (3,485)                                                                         | (708)                                                                   | 616,807                                  |
| Ancillary operations                                                                                | 92,700                                                  | -                                                                               | -                                                                       | 92,700                                   |
| Donations and non-government contributions                                                          | 170,900                                                 | (1,671)                                                                         | -                                                                       | 169,229                                  |
| Investment and other income <sup>(a)</sup>                                                          | 248,700                                                 | 151,092                                                                         | 1,665                                                                   | 401,457                                  |
| <b>TOTAL REVENUES</b>                                                                               | <b>19,144,028</b>                                       | <b>(650,809)</b>                                                                | <b>(48,545)</b>                                                         | <b>18,444,674</b>                        |
| <b>Expenses:</b>                                                                                    |                                                         |                                                                                 |                                                                         |                                          |
| Continuing care                                                                                     | 1,425,800                                               | (3,909)                                                                         | -                                                                       | 1,421,891                                |
| Community care                                                                                      | 2,132,200                                               | (380,384)                                                                       | (42,218)                                                                | 1,709,598                                |
| Home care                                                                                           | 930,200                                                 | -                                                                               | -                                                                       | 930,200                                  |
| Acute care                                                                                          | 6,081,528                                               | (200,640)                                                                       | (2,088)                                                                 | 5,878,800                                |
| Emergency medical services                                                                          | 735,400                                                 | -                                                                               | -                                                                       | 735,400                                  |
| Diagnostic and therapeutic services                                                                 | 2,956,600                                               | (45,135)                                                                        | (276)                                                                   | 2,911,189                                |
| Population and public health                                                                        | 420,900                                                 | (2,702)                                                                         | (107)                                                                   | 418,091                                  |
| Research and education                                                                              | 378,400                                                 | (1,718)                                                                         | -                                                                       | 376,682                                  |
| Information technology                                                                              | 804,300                                                 | (283)                                                                           | -                                                                       | 804,017                                  |
| Support services                                                                                    | 2,713,800                                               | (8,473)                                                                         | (2,291)                                                                 | 2,703,036                                |
| Administration                                                                                      | 564,900                                                 | (7,565)                                                                         | (1,565)                                                                 | 555,770                                  |
| <b>TOTAL EXPENSES</b>                                                                               | <b>19,144,028</b>                                       | <b>(650,809)</b>                                                                | <b>(48,545)</b>                                                         | <b>18,444,674</b>                        |
| <b>ANNUAL OPERATING SURPLUS</b>                                                                     | <b>-</b>                                                | <b>-</b>                                                                        | <b>-</b>                                                                | <b>-</b>                                 |
| Accumulated surplus, beginning of year                                                              | 1,304,458                                               | -                                                                               | -                                                                       | 1,304,458                                |
| <b>Accumulated surplus, end of year</b>                                                             | <b>\$ 1,304,458</b>                                     | <b>\$ -</b>                                                                     | <b>-</b>                                                                | <b>\$ 1,304,458</b>                      |

(i) The revised budget, resulting from the restructuring of AHS (Note 1), was approved by the former AHS Board on January 27, 2025.

## SCHEDULE 4 – CONSOLIDATED SCHEDULE OF THE REVISED BUDGET (CONTINUED)

| Reconciliation of the Current Year presentation for the Budget<br>FOR THE YEAR ENDED MARCH 31, 2025           |                            |                                                                                 |                                                                         |                                         |                                          |
|---------------------------------------------------------------------------------------------------------------|----------------------------|---------------------------------------------------------------------------------|-------------------------------------------------------------------------|-----------------------------------------|------------------------------------------|
|                                                                                                               | 2025<br>Original<br>Budget | Transfer of mental<br>health and addiction<br>operations to<br>Recovery Alberta | Transfer of<br>primary care<br>operations to<br>Primary Care<br>Alberta | Community<br>Lab Services<br>Correction | 2025<br>Revised<br>Budget <sup>(i)</sup> |
| Salaries and benefits                                                                                         | \$ 9,935,928               | \$ (484,130)                                                                    | (38,794)                                                                | 82,887                                  | \$ 9,495,891                             |
| Contracts with health service providers                                                                       | 3,748,600                  | (137,291)                                                                       | (4,773)                                                                 | (133,744)                               | 3,472,792                                |
| Contracts under the Health Facilities Act                                                                     | 77,000                     | -                                                                               | -                                                                       | -                                       | 77,000                                   |
| Drugs and gases                                                                                               | 742,600                    | (7,557)                                                                         | (27)                                                                    | -                                       | 735,016                                  |
| Medical supplies                                                                                              | 731,700                    | (1,517)                                                                         | (57)                                                                    | 30,940                                  | 761,066                                  |
| Other contracted services                                                                                     | 1,664,300                  | (8,956)                                                                         | (628)                                                                   | 8,071                                   | 1,662,787                                |
| Other                                                                                                         | 1,682,000                  | (11,358)                                                                        | (4,266)                                                                 | 11,846                                  | 1,678,222                                |
| Amortization and loss on disposals/write-downs of tangible capital assets (Note 21)                           | 561,900                    | -                                                                               | -                                                                       | -                                       | 561,900                                  |
|                                                                                                               | <b>\$ 19,144,028</b>       | <b>\$ (650,809)</b>                                                             | <b>(48,545)</b>                                                         | <b>-</b>                                | <b>\$ 18,444,674</b>                     |
| (a) Significant amounts included in Other are:                                                                |                            |                                                                                 |                                                                         |                                         |                                          |
| Equipment expense                                                                                             | \$ 323,300                 | \$ (959)                                                                        | (51)                                                                    | -                                       | \$ 322,290                               |
| Utilities                                                                                                     | 194,400                    | (2)                                                                             | -                                                                       | -                                       | 194,398                                  |
| Building and ground expenses                                                                                  | 174,800                    | (190)                                                                           | -                                                                       | -                                       | 174,610                                  |
| Housekeeping, laundry and linen, staff wearing apparel, plant maintenance and biomedical engineering supplies | 145,000                    | (1,051)                                                                         | (22)                                                                    | -                                       | 143,927                                  |
| Building rent                                                                                                 | 126,000                    | (432)                                                                           | (117)                                                                   | -                                       | 125,451                                  |
| Minor equipment purchases                                                                                     | 98,100                     | (286)                                                                           | (7)                                                                     | -                                       | 97,807                                   |
| Food and dietary supplies                                                                                     | 105,100                    | (692)                                                                           | -                                                                       | -                                       | 104,408                                  |
| Fundraising and grants awarded                                                                                | 55,200                     | -                                                                               | -                                                                       | -                                       | 55,200                                   |
| Office supplies and courier                                                                                   | 54,400                     | (3,164)                                                                         | (35)                                                                    | 11,846                                  | 63,047                                   |
| Insurance and liability claims                                                                                | 47,800                     | (14)                                                                            | (1,032)                                                                 | -                                       | 46,754                                   |
| Travel                                                                                                        | 33,000                     | (1,920)                                                                         | (45)                                                                    | -                                       | 31,035                                   |
| Telecommunications                                                                                            | 37,600                     | (28)                                                                            | (1)                                                                     | -                                       | 37,571                                   |
| Licenses, fees and memberships                                                                                | 24,300                     | (192)                                                                           | (6)                                                                     | -                                       | 24,102                                   |
| Education                                                                                                     | 15,500                     | (284)                                                                           | (10)                                                                    | -                                       | 15,206                                   |
| Other                                                                                                         | 247,500                    | (2,144)                                                                         | (2,940)                                                                 | -                                       | 242,416                                  |
|                                                                                                               | <b>\$ 1,682,000</b>        | <b>\$ (11,358)</b>                                                              | <b>(4,266)</b>                                                          | <b>11,846</b>                           | <b>\$ 1,678,222</b>                      |

(i) The revised budget, resulting from the restructuring of AHS (Note 1), was approved by the former AHS Board on January 27, 2025.

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# Management’s Responsibility for Financial Reporting

The accompanying financial statements are the responsibility of management and have been reviewed and approved by Senior Management. The financial statements were prepared in accordance with Canadian Public Sector Accounting Standards, and of necessity, include some amounts that are based on estimates and judgement.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains a system of internal accounting controls comprising written policies, standards and procedures, a formal authorization structure, and satisfactory processes for reviewing internal controls. This system provides management with reasonable assurance that transactions are in accordance with governing legislation and are properly authorized, reliable financial records are maintained, and assets are adequately safeguarded.

The Health Quality Council of Alberta’s Board of Directors carries out their responsibility for the financial statements through the Audit and Finance Committee. The Committee meets with management and the Auditor General of Alberta to review financial matters and recommends the financial statements to the Health Quality Council of Alberta Board of Directors for approval upon finalization of the audit. The Auditor General of Alberta has open and complete access to the Audit and Finance Committee.

The Auditor General of Alberta provides an independent audit of the financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures, which allow him to report on the fairness of the financial statements prepared by management.

On behalf of the Health Quality Council of Alberta.

[Originally signed by]

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Interim Chief Executive Officer  
Dr. David Zygun  
May 28, 2025

[Originally signed by]

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Director, Financial Services  
Jessica Wing  
May 28, 2025

# Independent Auditor's Report

To the Board of Directors of the Health Quality Council of Alberta



**Auditor  
General**  
OF ALBERTA

## Report on the Financial Statements

### Opinion

I have audited the financial statements of the Health Quality Council of Alberta, which comprise the statement of financial position as at March 31, 2025, and the statements of operations, change in net financial assets, and cash flows for the year then ended, and notes to the financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying financial statements present fairly, in all material respects, the financial position of the Health Quality Council of Alberta as at March 31, 2025, and the results of its operations, its changes in net financial assets, and its cash flows for the year then ended in accordance with Canadian public sector accounting standards.

### Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Financial Statements* section of my report. I am independent of the Health Quality Council of Alberta in accordance with the ethical requirements that are relevant to my audit of the financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

### Other information

Management is responsible for the other information. The other information comprises the information included in the *Annual Report*, but does not include the financial statements and my auditor's report thereon. The *Annual Report* is expected to be made available to me after the date of this auditor's report.

My opinion on the financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

## **Responsibilities of management and those charged with governance for the financial statements**

Management is responsible for the preparation and fair presentation of the financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is responsible for assessing the Health Quality Council of Alberta's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the Health Quality Council of Alberta's financial reporting process.

## **Auditor's responsibilities for the audit of the financial statements**

My objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgment and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Health Quality Council of Alberta's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Health Quality Council of Alberta's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit evidence obtained up to the date of my auditor's report. However, future events or conditions may cause the Health Quality Council of Alberta to cease to continue as a going concern.

- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Originally signed by W. Doug Wylie FCPA, FCMA, ICD.D]  
Auditor General

May 28, 2025  
Edmonton, Alberta

# Statement of Operations

Year ended March 31

(thousands of dollars)

|                                                  | 2025     |          | 2024     |
|--------------------------------------------------|----------|----------|----------|
|                                                  | Budget   | Actual   | Actual   |
|                                                  | (Note 4) |          |          |
| <b>Revenues</b>                                  |          |          |          |
| Alberta Health transfers                         |          |          |          |
| Base operating                                   | \$ 7,711 | \$ 7,711 | \$ 7,672 |
| Restricted operating (Note 6)                    | 366      | 859      | 1,053    |
| Other government transfer (Note 6)               | -        | -        | 125      |
| Interest income                                  | 75       | 91       | 132      |
| Other revenue                                    | -        | 28       | 25       |
|                                                  | 8,152    | 8,689    | 9,007    |
| <b>Expenses (Schedule 1)</b>                     |          |          |          |
| Administration                                   | 1,633    | 1,629    | 1,544    |
| Health system analytics                          | 4,135    | 3,416    | 3,010    |
| Health system improvement                        | 1,780    | 1,705    | 1,629    |
| Communications and engagement                    | 1,547    | 1,358    | 1,391    |
| Ministerial assessment/study (Note 6)            | 366      | 859      | 1,178    |
| Minister priorities                              | -        | 288      | -        |
|                                                  | 9,461    | 9,255    | 8,752    |
| Annual operating (deficit) surplus               | (1,309)  | (566)    | 255      |
| Accumulated operating surplus, beginning of year | 1,589    | 2,206    | 1,951    |
| Accumulated operating surplus, end of year       | \$ 280   | \$ 1,640 | \$ 2,206 |

The accompanying notes and schedules are part of these financial statements.

# Statement of Financial Position

**Year ended March 31**

(thousands of dollars)

|                                                | <b>2025</b>     | <b>2024</b>     |
|------------------------------------------------|-----------------|-----------------|
| <b>Financial Assets</b>                        |                 |                 |
| Cash                                           | \$ 2,157        | \$ 4,346        |
| Accounts receivable                            | 25              | 37              |
|                                                | <u>2,182</u>    | <u>4,383</u>    |
| <b>Liabilities</b>                             |                 |                 |
| Accounts payable and other accrued liabilities | 800             | 1,069           |
| Unspent deferred contributions (Note 6)        | 88              | 1,609           |
| Employee future benefits (Note 7)              | 53              | 43              |
| Deferred lease inducements (Note 8)            | 30              | 16              |
|                                                | <u>971</u>      | <u>2,737</u>    |
| <b>Net Financial Assets</b>                    | <u>1,211</u>    | <u>1,646</u>    |
| <b>Non-Financial Assets</b>                    |                 |                 |
| Tangible capital assets (Note 9)               | 293             | 463             |
| Purchased intangibles                          | 11              | 12              |
| Prepaid expenses                               | 125             | 85              |
|                                                | <u>429</u>      | <u>560</u>      |
| <b>Net Assets</b>                              | <u>1,640</u>    | <u>2,206</u>    |
| <b>Net Assets</b>                              |                 |                 |
| Accumulated operating surplus (Note 11)        | <u>\$ 1,640</u> | <u>\$ 2,206</u> |

Contractual obligations (Note 10)

The accompanying notes and schedules are part of these financial statements.

## Statement of Change in Net Financial Assets

Year ended March 31

(thousands of dollars)

|                                                                  | 2025           |                 | 2024            |
|------------------------------------------------------------------|----------------|-----------------|-----------------|
|                                                                  | Budget         | Actual          | Actual          |
|                                                                  | (Note 4)       |                 |                 |
| Annual operating (deficit) surplus                               | \$ (1,309)     | \$ (566)        | \$ 255          |
| Acquisition of tangible capital assets (Note 9)                  | -              | (13)            | (16)            |
| Acquisition of purchased intangibles                             | -              | -               | (12)            |
| Amortization of tangible capital assets (Note 9)                 | 178            | 183             | 210             |
| Amortization of purchased intangibles                            | -              | 1               | -               |
| Decrease in prepaid expenses                                     | -              | (40)            | 13              |
| <b>(Decrease) / Increase in net financial assets in the year</b> | <b>(1,131)</b> | <b>(435)</b>    | <b>450</b>      |
| <b>Net financial assets, beginning of year</b>                   | <b>1,589</b>   | <b>1,646</b>    | <b>1,196</b>    |
| <b>Net financial assets, end of year</b>                         | <b>\$ 458</b>  | <b>\$ 1,211</b> | <b>\$ 1,646</b> |

The accompanying notes and schedules are part of these financial statements.

# Statement of Cash Flows

Year ended March 31

(thousands of dollars)

|                                                                       | 2025            | 2024            |
|-----------------------------------------------------------------------|-----------------|-----------------|
| <b>Operating Transactions</b>                                         |                 |                 |
| Annual operating (deficit) surplus                                    | \$ (566)        | \$ 255          |
| Non-cash items:                                                       |                 |                 |
| Amortization of tangible capital assets (Note 9)                      | 183             | 210             |
| Amortization of purchased intangibles                                 | 1               | -               |
| Amortization of deferred lease inducements (Note 8)                   | (4)             | (2)             |
| Increase in employee future benefits (Note 7)                         | 10              | 8               |
|                                                                       | (376)           | 471             |
| Decrease in accounts receivable                                       | 12              | 672             |
| (Increase) Decrease in prepaid expenses                               | (40)            | 13              |
| (Decrease) Increase in accounts payable and other accrued liabilities | (269)           | 378             |
| (Decrease) Increase in unspent deferred contributions (Note 6)        | (1,521)         | 222             |
| Increase in deferred lease inducement (Note 8)                        | 18              | 18              |
| Cash (applied to) provided by operating transactions                  | (2,176)         | 1,774           |
| <b>Capital Transactions</b>                                           |                 |                 |
| Acquisition of tangible capital assets (Note 9)                       | (13)            | (16)            |
| Purchase of intangibles                                               | -               | (12)            |
| Cash (applied to) capital transactions                                | (13)            | (28)            |
| <b>(Decrease) Increase in cash</b>                                    | (2,189)         | 1,746           |
| <b>Cash at beginning of year</b>                                      | 4,346           | 2,600           |
| <b>Cash at end of year</b>                                            | <u>\$ 2,157</u> | <u>\$ 4,346</u> |

The accompanying notes and schedules are part of these financial statements.

# Notes to the Financial Statements

March 31, 2025

(in thousands)

## Note 1 AUTHORITY

The Health Quality Council of Alberta (HQCA) is a government not-for-profit organization formed under the *Health Quality Council of Alberta Act*.

Pursuant to the Act, the HQCA has a mandate to promote and improve patient safety, person-centered care and health service quality on a province-wide basis. The HQCA is exempt from income taxes under the *Income Tax Act*.

## Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES

These financial statements are prepared in accordance with Canadian Public Sector Accounting Standards (PSAS).

### (a) Reporting Entity

The financial statements reflect the assets, liabilities, revenues and expenses of the HQCA.

### (b) Basis of Financial Reporting

#### Revenues

All revenues are reported on the accrual basis of accounting.

Revenues from transactions with performance obligations are recognized as the performance obligations are satisfied by providing the promised services to the payors. Course fees are recognized in the year that courses are delivered.

Cash received for which services have not been provided by year end is recognized as unearned revenue.

Revenues from transactions with no performance obligations are recognized at their realizable value when the HQCA has the authority to claim or retain an inflow of economic resources and identifies a past transaction or event that gives rise to an asset.

#### Government transfers

Transfers from all governments are referred to as government transfers.

Government transfers and the associated externally restricted investment income are recognized as deferred contributions when the stipulations together with the HQCA's actions and communications as to the use of the transfer, create a liability. These transfers are recognized as revenue as the stipulations are met and, when applicable, the HQCA complies with its communicated use of these transfers.

All other government transfers, without stipulations for the use of the transfer, are recognized as revenue when the transfer is authorized and the HQCA meets the eligibility criteria (if any).

#### Expenses

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed.

Grants and transfers are recognized as expenses when the transfer is authorized and eligibility criteria have been met by the recipient.

# Notes to the Financial Statements

March 31, 2025

(in thousands)

## Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

### (b) Basis of Financial Reporting (Cont'd)

#### Valuation of Financial Assets and Liabilities

The HQCA's financial assets and liabilities are generally measured as follows:

| <u>Financial Statement Component</u>           | <u>Measurement</u>                     |
|------------------------------------------------|----------------------------------------|
| Cash                                           | Cost                                   |
| Accounts receivable                            | Lower of cost or net recoverable value |
| Accounts payable and other accrued liabilities | Cost                                   |

The HQCA does not hold equities traded in an active market, nor engage in derivative contracts or foreign currency transactions. The HQCA is not exposed to remeasurement gains or losses and, consequently, a statement of remeasurement gains and losses is not presented.

#### Financial Assets

Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations.

Financial assets are the HQCA's financial claims on external organizations and individuals as well as cash at the year end.

#### *Cash*

Cash comprises cash on hand and demand deposits.

#### *Accounts Receivable*

Accounts receivable are recognized at the lower of cost or net recoverable value. A valuation allowance is recognized when recovery is uncertain.

#### Liabilities

Liabilities represent present obligations of the HQCA to external organizations and individuals arising from past transactions or events occurring before the year end, the settlement of which is expected to result in the future sacrifice of economic benefits. They are recognized when there is an appropriate basis of measurement and management can reasonably estimate the amounts.

#### *Deferred Lease Inducements*

Deferred lease inducements represent amounts received for leasehold improvements and the value of a rent-free period. Lease inducements are deferred and amortized on a straight-line basis over

# Notes to the Financial Statements

March 31, 2025  
(in thousands)

## Note 2      SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

(b)    Basis of Financial Reporting (Cont'd)  
the term of the related lease and the amortization is recognized as a reduction of rent expense for the year.

### *Employee Future Benefits*

The HQCA Board has approved a defined contribution Supplementary Executive Retirement Plan (SERP) for certain members of its executive staff. The SERP supplements the benefit under the HQCA registered plan that is limited by the *Income Tax Act* (Canada). The HQCA contributes a certain percentage of an eligible employee's pensionable earnings in excess of the limits of the *Income Tax Act* (Canada). This plan provides participants with an account balance at retirement based on the contributions made to the plan and investment income earned on the contributions based on investment decisions made by the participants.

### *Non-Financial Assets*

Non-financial assets are acquired, constructed, or developed assets that do not normally provide resources to discharge existing liabilities, but instead:

- (a) are normally employed to deliver government services;
  - (b) may be consumed in the normal course of operations; and
  - (c) are not for sale in the normal course of operations.
- Non-financial assets are limited to tangible capital assets, purchased intangibles and prepaid expenses.

### *Tangible Capital Assets*

Tangible capital assets are recognized at cost less amortization, which includes amounts that are directly related to the acquisition, design, construction, development, improvement or betterment of the assets. Cost includes overhead directly attributable to construction and development, as well as interest costs that are directly attributable to the acquisition or construction of the asset.

The cost, less residual value, of the tangible capital assets, excluding work-in-progress, is amortized on a straight-line basis over their estimated useful lives as follows:

|                                |                        |
|--------------------------------|------------------------|
| Computer hardware and software | 5 years                |
| Office equipment               | 10 years               |
| Leasehold improvements         | Over term of the lease |

Tangible capital assets are written down when conditions indicate that they no longer contribute to HQCA's ability to provide services, or when the value of future economic benefits associated with the tangible capital assets are less than their book value. The net write-downs are accounted for as expenses in the Statement of Operations

# Notes to the Financial Statements

**March 31, 2025**

(in thousands)

## **Note 2      SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)**

### **(b)    Basis of Financial Reporting (Cont'd)**

#### *Purchased Intangible Assets*

Purchased Intangibles are identifiable non-monetary economic resources without physical substance acquired through an arms-length exchange transaction. Purchased Intangibles are recognized at cost less amortization, and tested regularly for impairment. The cost, less any residual value, of purchased intangible assets is amortized on a straight-line basis over its useful life in a manner appropriate to its nature and use, which is normally the shortest of the technological, commercial, and legal life.

#### *Prepaid Expenses*

Prepaid expenses are recognized at cost and amortized based on the terms of the agreement.

#### **Funds and Reserves**

Certain amounts, as approved by the Board of Directors, are set aside in accumulated operating surplus for future operating and capital purposes. Transfers to/from funds and reserves are an adjustment to the respective fund when approved.

#### **Measurement Uncertainty**

Measurement uncertainty exists when there is a variance between the recognized or disclosed amount and another reasonably possible amount. The amounts recognized for amortization of tangible capital assets are based on estimates of the useful life of the related assets. Actual results could differ from estimates.

## **Note 3      FUTURE CHANGES IN ACCOUNTING STANDARDS**

On April 1, 2026, the HQCA will adopt the following new conceptual framework and accounting standard approved by the Public Sector Accounting Board:

### **The Conceptual Framework for Financial Reporting in the Public Sector**

The Conceptual Framework is the foundation for public sector financial reporting standard setting. It replaces the conceptual aspects of Section PS 1000 Financial Statement Concepts and Section PS 1100 Financial Statement Objectives. The conceptual framework highlights considerations fundamental for the consistent application of accounting issues in the absence of specific standards.

## Notes to the Financial Statements

**March 31, 2025**

(in thousands)

### **Note 3 FUTURE CHANGES IN ACCOUNTING STANDARDS (CONT'D)**

#### **PS 1202 Financial Statement Presentation**

Section PS 1202 sets out general and specific requirements for the presentation of information in general purpose financial statements. The financial statement presentation principles are based on the concepts within the Conceptual Framework.

Management is currently assessing the impact of the conceptual framework and the standard on the financial statements.

### **Note 4 BUDGET**

The HQCA's 2024-2025 operating budget was approved by the Board of Directors on January 24, 2024 and submitted to the Ministry of Health.

### **Note 5 FINANCIAL RISK MANAGEMENT**

The HQCA has the following financial instruments: cash, accounts receivable, accounts payable and other accrued liabilities.

The HQCA has exposure to the following risks from its use of financial instruments: interest rate risk, liquidity risk, price risk and credit risk.

#### **(a) Interest rate risk**

The HQCA is exposed to the interest rate associated with cash held in the bank. The interest rate risk is minimal.

#### **(b) Liquidity risk**

Liquidity risk is the risk that the HQCA will encounter difficulty in meeting obligations associated with financial liabilities. The HQCA enters into transactions to purchase goods and services on credit. Liquidity risk is measured by reviewing the HQCA's future net cash flows for the possibility of negative net cash flow. The HQCA manages the liquidity risk resulting from its accounts payable obligations by maintaining adequate cash resources.

#### **(c) Credit risk**

The HQCA is exposed to credit risk from potential non-payment of accounts receivable. During the fiscal year most of the HQCA's receivables are from provincial agencies; therefore the credit risk is minimized.

# Notes to the Financial Statements

**March 31, 2025**

(in thousands)

## Note 6 UNSPENT DEFERRED CONTRIBUTIONS

|                                           | 2025           |        |          | 2024     |
|-------------------------------------------|----------------|--------|----------|----------|
|                                           | Alberta Health | Other  | Total    | Total    |
| Balance at beginning of the year          | \$ 947         | \$ 662 | \$ 1,609 | \$ 1,387 |
| Cash (disbursed) received during the year | -              | (662)  | (662)    | 1,400    |
| Amounts recognized as revenue             | (859)          | -      | (859)    | (1,178)  |
| Balance at end of year                    | \$ 88          | \$ -   | \$ 88    | \$ 1,609 |

The unspent deferred contribution from Alberta Health at the end of the year is externally restricted to support the Pandemic Data Review task force to provide recommendations to the Minister. The balance in other relates to an externally restricted grant from the Ministry of Mental Health and Addiction that was transferred to the Canadian Centre of Recovery Excellence. Total unspent deferred contribution and interest transferred was \$688 (Schedule 3).

## Note 7 EMPLOYEE FUTURE BENEFITS

The HQCA participates in the Local Authorities Pension Plan (LAPP), a multi-employer defined benefit pension plan.

The HQCA accounts for this multi-employer pension plan on a defined contribution basis. The HQCA is not responsible for future funding of the plan deficit other than through contribution increases. Pension expense recorded in the financial statements is equivalent to the HQCA's annual contributions of \$397 for the year ended March 31, 2025 (2024 - \$388).

At December 31, 2024, the Local Authorities Pension Plan reported a surplus of \$19,557,148 (2023 – surplus of \$15,056,661).

The Supplementary Executive Retirement Plan (SERP) payable at year ended March 31, 2025 is \$53 (2024 - \$43). The current year contribution related to this plan is \$7 (2024 - \$8). No payment has been made to plan members at retirement in the current year.

## Note 8 DEFERRED LEASE INDUCEMENTS

The HQCA received a lease inducement in the form of free rent relating to a lease renewal of the premises effective April 1, 2024. This amount will be amortized on a straight-line basis over the term of the related lease and the amortization is recognized as a reduction of rent expense.

|                                       | 2025  | 2024  |
|---------------------------------------|-------|-------|
| Lease inducements - rent free periods | \$ 36 | \$ 18 |
| Less accumulated amortization         | (6)   | (2)   |
|                                       | \$ 30 | \$ 16 |

## Notes to the Financial Statements

**March 31, 2025**

(in thousands)

### Note 9 TANGIBLE CAPITAL ASSETS

|                                            | 2025                |                                    |                           |            | 2024           |
|--------------------------------------------|---------------------|------------------------------------|---------------------------|------------|----------------|
|                                            | Office<br>Equipment | Computer<br>Hardware &<br>Software | Leasehold<br>improvements | Total      | Total          |
| <b>Estimated useful life</b>               | 10 years            | 5 years                            | Over term of the<br>lease |            |                |
| <b>Historical Cost</b>                     |                     |                                    |                           |            |                |
| Beginning of year                          | \$ 402              | \$ 1,349                           | \$ 1,013                  | \$ 2,764   | \$ 2,803       |
| Additions                                  | -                   | 13                                 | -                         | 13         | 16             |
| Disposals, including write-downs           | -                   | -                                  | -                         | -          | (55)           |
|                                            | 402                 | 1,362                              | 1,013                     | 2,777      | 2,764          |
| <b>Accumulated Amortization</b>            |                     |                                    |                           |            |                |
| Beginning of year                          | 363                 | 925                                | 1,013                     | 2,301      | 2,146          |
| Amortization expense                       | 28                  | 155                                | -                         | 183        | 210            |
| Effect of disposals, including write-downs | -                   | -                                  | -                         | -          | (55)           |
|                                            | 391                 | 1,080                              | 1,013                     | 2,484      | 2,301          |
| <br>Net book value at March 31, 2025       | <br>\$ 11           | <br>\$ 282                         | <br>\$ -                  | <br>\$ 293 |                |
| <br><br>Net book value at March 31, 2024   | <br><br>\$ 39       | <br><br>\$ 424                     | <br><br>\$ -              |            | <br><br>\$ 463 |

## Notes to the Financial Statements

**March 31, 2025**

(in thousands)

### Note 10 CONTRACTUAL OBLIGATIONS

Contractual obligations are obligations of the HQCA to others that will become liabilities in the future when the terms of those contracts or agreements are met.

Estimated payment requirements for each of the next five years and thereafter are as follows:

| <b>Year ended<br/>March 31</b> | <b>Operating<br/>Lease</b> |
|--------------------------------|----------------------------|
| 2025 – 26                      | \$ 335                     |
| 2026 – 27                      | 335                        |
| 2027 – 28                      | 347                        |
| 2028 – 29                      | 358                        |
| Thereafter                     | 1,557                      |
|                                | <u>\$ 2,932</u>            |

### Note 11 ACCUMULATED OPERATING SURPLUS

Accumulated operating surplus is comprised of the following:

|                                                            | <b>2025</b>                                                                                              |                                                            |                                               |              | <b>2024</b>  |  |
|------------------------------------------------------------|----------------------------------------------------------------------------------------------------------|------------------------------------------------------------|-----------------------------------------------|--------------|--------------|--|
|                                                            | <b>Investment in<br/>Tangible<br/>Capital<br/>Assets and<br/>Purchased<br/>Intangibles<sup>(a)</sup></b> | <b>Internally<br/>Restricted<br/>Surplus<sup>(b)</sup></b> | <b>Unrestricted<br/>Surplus<br/>(Deficit)</b> | <b>Total</b> | <b>Total</b> |  |
| Balance, April 1                                           | \$ 475                                                                                                   | \$ 1,731                                                   | \$ -                                          | \$ 2,206     | \$ 1,951     |  |
| Annual operating (deficit)<br>surplus                      | -                                                                                                        | -                                                          | (566)                                         | (566)        | 255          |  |
| Net investments in capital<br>assets and intangible assets | (171)                                                                                                    | -                                                          | 171                                           | -            | -            |  |
| Transfers, prior year restricted                           | -                                                                                                        | (1,731)                                                    | 1,731                                         | -            | -            |  |
| Transfers, current year<br>restricted                      | -                                                                                                        | 1,336                                                      | (1,336)                                       | -            | -            |  |
| Balance, March 31                                          | \$ 304                                                                                                   | \$ 1,336                                                   | \$ -                                          | \$ 1,640     | \$ 2,206     |  |

# Notes to the Financial Statements

March 31, 2025

(in thousands)

## Note 11 ACCUMULATED OPERATING SURPLUS (CONT'D)

- (a) Investment in tangible capital assets and intangible assets represents the net book value of internally funded tangible capital assets and intangibles assets. These assets are restricted and are not available for any other purpose.
- (b) The internally restricted surplus represents amounts set aside by the Board for future purposes. Those amounts are not available for other purposes without the approval of the Board. Internally restricted surplus based on the annual work plan is summarized as follows:

|                                                      | 2025     | 2024     |
|------------------------------------------------------|----------|----------|
| Assessing patient/ family/ provider experience       | \$ 469   | \$ 1,252 |
| Supporting Modernizing Alberta's Primary Care System | 425      | 110      |
| Monitoring                                           | 133      | 300      |
| External reviews                                     | 105      | -        |
| Health system improvement resources and activities   | 204      | 69       |
|                                                      | \$ 1,336 | \$ 1,731 |

## Note 12 COMPARATIVE FIGURES

Certain 2024 figures have been reclassified, where necessary, to conform to the 2025 presentation.

## Note 13 APPROVAL OF THE FINANCIAL STATEMENTS

The financial statements were approved by the HQCA Board of Directors on May 28, 2025.

## Schedule 1 – Expenses – Detailed by Object

### Year ended March 31

(thousands of dollars)

|                                                  | 2025     |          | 2024     |  |
|--------------------------------------------------|----------|----------|----------|--|
|                                                  | Budget   | Actual   | Actual   |  |
|                                                  | (Note 4) |          |          |  |
| Salaries and benefits                            | \$ 5,437 | \$ 5,328 | \$ 5,104 |  |
| Supplies, services and other                     | 3,846    | 3,743    | 3,438    |  |
| Amortization of tangible capital assets (Note 9) | 178      | 183      | 210      |  |
| Amortization of intangible capital assets        | -        | 1        | -        |  |
|                                                  | \$ 9,461 | \$ 9,255 | \$ 8,752 |  |

## Schedule 2 – Salary and Benefits Disclosure

**Year ended March 31**

(thousands of dollars)

|                                   | 2025                          |                                       |                                               | 2024          |               |
|-----------------------------------|-------------------------------|---------------------------------------|-----------------------------------------------|---------------|---------------|
|                                   | Base<br>Salary <sup>(1)</sup> | Other Cash<br>Benefits <sup>(2)</sup> | Other Non-<br>Cash<br>Benefits <sup>(3)</sup> | Total         | Total         |
| <b>Board of Directors-Chair</b>   | \$ -                          | \$ 29                                 | \$ -                                          | \$ 29         | \$ 20         |
| <b>Board of Directors-Members</b> | -                             | 50                                    | -                                             | 50            | 28            |
| <b>Chief Executive Officer</b>    | 264                           | 6                                     | 28                                            | 298           | 298           |
|                                   | <b>\$ 264</b>                 | <b>\$ 85</b>                          | <b>\$ 28</b>                                  | <b>\$ 377</b> | <b>\$ 346</b> |

(1) Base salary includes pensionable base pay.

(2) Other cash benefits include honoraria for board members, vehicle allowance. There were no vacation payouts or bonuses paid in 2025.

(3) Other non-cash benefits include: employer's portion of all employee benefits and contributions or payments made on behalf of employees, including pension, Supplementary Executive Retirement Plan, health care benefits, dental coverage, vision coverage, out of country medical benefits, group life insurance, accidental disability and dismemberment insurance, employee assistance program, Canadian Pension Plan, Employment Insurance and fair market value parking.

## Schedule 3 – Related Party Transactions

### Year ended March 31

(thousands of dollars)

Related parties are those entities consolidated or accounted for on a modified equity basis in the Government of Alberta's Consolidated Financial Statements. Related parties also include key management personnel and close family members of those individuals in the HQCA. The HQCA and its employees paid or collected certain taxes and fees set by regulation for premiums, licenses, and other charges. These amounts were incurred in the normal course of business, reflect charges applicable to all users, and have been excluded from this schedule.

The HQCA had the following transactions with related parties recorded in the Statement of Operations and the Statement of Financial Position at the amount of consideration agreed upon between the related parties.

|                                              | 2025            | 2024            |
|----------------------------------------------|-----------------|-----------------|
| <b>Revenues</b>                              |                 |                 |
| Grants                                       | \$ 8,570        | \$ 8,850        |
| Other                                        | -               | 26              |
|                                              | <u>\$ 8,570</u> | <u>\$ 8,876</u> |
| <b>Expenses</b>                              |                 |                 |
| Other services                               | \$ 41           | \$ 207          |
|                                              | <u>\$ 41</u>    | <u>\$ 207</u>   |
| <b>Payable to related parties</b>            | <u>\$ 104</u>   | <u>\$ 1,614</u> |
| <b>Amount transferred to related parties</b> | <u>\$ 688</u>   | <u>\$ -</u>     |

Financial Information

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## Department of Health

**Statement of Credit or Recovery (unaudited)**  
**Year ended March 31, 2025**  
*(in thousands)*

|                                              |    | 2025                   |                                 |                     |                                                                                    |
|----------------------------------------------|----|------------------------|---------------------------------|---------------------|------------------------------------------------------------------------------------|
|                                              |    | Authorized<br>Spending | Actual<br>Revenue<br>Recognized | Unearned<br>Revenue | Total Amount<br>Received /<br>Receivable<br>(Shortfall) /<br>Excess <sup>(1)</sup> |
| <b>Expense Amounts</b>                       |    |                        |                                 |                     |                                                                                    |
| Ministry Support Services                    |    |                        |                                 |                     |                                                                                    |
| Strategic Corporate Support <sup>(a)</sup>   | \$ | 2,024                  | \$ -                            | \$ -                | \$ 1,617 \$ (407)                                                                  |
| Population and Public Health                 |    |                        |                                 |                     |                                                                                    |
| Research and Support Programs <sup>(b)</sup> |    | 1,000                  | -                               | -                   | - (1,000)                                                                          |
|                                              | \$ | 3,024                  | \$ -                            | \$ -                | \$ 1,617 \$ (1,407)                                                                |

(a) The Department receives revenue from the Department of Mental Health and Addiction to recover the cost of providing shared corporate services.

(b) The Department receives revenue from Health Canada to enhance existing health services to persons with chronic Hepatitis C virus infection.

(1) Shortfall is deducted from current year's corresponding funding authority.

## Department of Health

**Lapse/Encumbrance (unaudited)**  
**Year ended March 31, 2025**  
*(in thousands)*

| <b>Expense Vote by Program</b>                                | <b>Voted</b>                   | <b>Supplementary</b>           | <b>Adjustments</b>    | <b>Voted</b>      | <b>Voted</b>                  | <b>Expended/</b>    |
|---------------------------------------------------------------|--------------------------------|--------------------------------|-----------------------|-------------------|-------------------------------|---------------------|
| <b>Operating Expense</b>                                      | <b>Estimate <sup>(1)</sup></b> | <b>Estimate <sup>(2)</sup></b> | <b><sup>(3)</sup></b> | <b>Estimate</b>   | <b>Actuals <sup>(4)</sup></b> | <b>(Unexpended)</b> |
| <b>1 Ministry Support Services</b>                            |                                |                                |                       |                   |                               |                     |
| 1.1 Minister's Office                                         | \$ 1,225                       | \$ -                           | \$ -                  | \$ 1,225          | \$ 1,203                      | \$ (22)             |
| 1.2 Deputy Minister's Office                                  | 1,973                          | -                              | -                     | 1,973             | 2,564                         | 591                 |
| 1.3 Strategic Corporate Support<br>and Policy Development     | 57,721                         | -                              | -                     | 57,721            | 96,589                        | 38,868              |
| 1.4 Health Advocates' Office                                  | 3,569                          | -                              | -                     | 3,569             | 2,673                         | (896)               |
|                                                               | <u>64,488</u>                  | <u>-</u>                       | <u>-</u>              | <u>64,488</u>     | <u>103,029</u>                | <u>38,541</u>       |
| <b>2 Health Entities (previously Alberta Health Services)</b> |                                |                                |                       |                   |                               |                     |
| 2.1 Continuing Care                                           | 1,263,803                      | -                              | -                     | 1,263,803         | 1,278,768                     | 14,965              |
| 2.2 Community Care                                            | 1,827,081                      | -                              | -                     | 1,827,081         | 1,849,241                     | 22,160              |
| 2.3 Home Care                                                 | 859,820                        | -                              | -                     | 859,820           | 903,627                       | 43,807              |
| 2.4 Acute Care                                                | 4,446,918                      | -                              | (137,502)             | 4,309,416         | 4,125,210                     | (184,206)           |
| 2.5 Emergency Medical Services                                | 617,797                        | -                              | -                     | 617,797           | 589,095                       | (28,702)            |
| 2.6 Diagnostic and Therapeutic Services                       | 2,433,051                      | -                              | -                     | 2,433,051         | 2,483,152                     | 50,101              |
| 2.7 Population and Public Health                              | 386,570                        | -                              | -                     | 386,570           | 389,735                       | 3,165               |
| 2.8 Health Workforce<br>Education and Research                | 83,328                         | -                              | -                     | 83,328            | 84,568                        | 1,240               |
| 2.9 Information Technology                                    | 454,702                        | -                              | -                     | 454,702           | 457,054                       | 2,352               |
| 2.10 Administration and Support Services                      | 2,387,752                      | -                              | -                     | 2,387,752         | 2,290,617                     | (97,135)            |
| 2.12 Health Quality Council of Alberta                        | 9,976                          | -                              | -                     | 9,976             | 7,711                         | (2,265)             |
|                                                               | <u>14,770,798</u>              | <u>-</u>                       | <u>(137,502)</u>      | <u>14,633,296</u> | <u>14,458,778</u>             | <u>(174,518)</u>    |
| <b>3 Physician Compensation and Development</b>               |                                |                                |                       |                   |                               |                     |
| 3.1 Program Support                                           | 10,425                         | -                              | -                     | 10,425            | 11,078                        | 653                 |
| 3.2 Physician Services                                        | 5,760,110                      | -                              | -                     | 5,760,110         | 5,830,800                     | 70,690              |
| 3.3 Physician Education and Recruitment                       | 452,490                        | -                              | -                     | 452,490           | 375,765                       | (76,725)            |
|                                                               | <u>6,223,025</u>               | <u>-</u>                       | <u>-</u>              | <u>6,223,025</u>  | <u>6,217,643</u>              | <u>(5,382)</u>      |

## Department of Health

**Lapse/Encumbrance (unaudited) (continued)**  
**Year ended March 31, 2025**  
*(in thousands)*

| Expense Vote by Program                                               | Voted<br>Estimate <sup>(1)</sup> | Supplementary<br>Estimate <sup>(2)</sup> | Adjustments<br><sup>(3)</sup> | Adjusted<br>Voted<br>Estimate | Voted<br>Actuals <sup>(4)</sup> | Over<br>Expended/<br>(Unexpended) |
|-----------------------------------------------------------------------|----------------------------------|------------------------------------------|-------------------------------|-------------------------------|---------------------------------|-----------------------------------|
| <b>Operating Expense</b>                                              |                                  |                                          |                               |                               |                                 |                                   |
| <b>4 Drugs and Supplemental Health Benefits</b>                       |                                  |                                          |                               |                               |                                 |                                   |
| 4.1 Program Support                                                   | \$ 71,827                        | \$ -                                     | \$ -                          | \$ 71,827                     | \$ 64,698                       | \$ (7,129)                        |
| 4.2 Outpatient Drugs                                                  | 480,000                          | -                                        | -                             | 480,000                       | 473,859                         | (6,141)                           |
| 4.3 Seniors Drug, Dental, Optical<br>and Supplemental Health Benefits | 882,441                          | -                                        | -                             | 882,441                       | 961,608                         | 79,167                            |
| 4.4 Non-Group Drug<br>and Supplemental Health Benefits                | 214,325                          | -                                        | -                             | 214,325                       | 245,330                         | 31,005                            |
| 4.5 Assured Income for the Severely<br>Handicapped Health Benefit     | 260,902                          | -                                        | -                             | 260,902                       | 269,673                         | 8,771                             |
| 4.6 Child Health Benefit                                              | 25,897                           | -                                        | -                             | 25,897                        | 21,634                          | (4,263)                           |
| 4.7 Adult Health Benefit                                              | 229,045                          | -                                        | -                             | 229,045                       | 245,010                         | 15,965                            |
| 4.8 Alberta Aids to Daily Living                                      | 219,750                          | -                                        | -                             | 219,750                       | 211,628                         | (8,122)                           |
| 4.9 Pharmaceutical Innovation and Management                          | 157,980                          | -                                        | -                             | 157,980                       | 198,974                         | 40,994                            |
|                                                                       | <u>2,542,167</u>                 | <u>-</u>                                 | <u>-</u>                      | <u>2,542,167</u>              | <u>2,692,414</u>                | <u>150,247</u>                    |
| <b>5 Primary Health Care</b>                                          |                                  |                                          |                               |                               |                                 |                                   |
| 5.1 Program Support                                                   | 10,806                           | -                                        | -                             | 10,806                        | 9,111                           | (1,695)                           |
| 5.2 Primary Health Care                                               | 464,443                          | 26,200                                   | -                             | 490,643                       | 406,185                         | (84,458)                          |
|                                                                       | <u>475,249</u>                   | <u>26,200</u>                            | <u>-</u>                      | <u>501,449</u>                | <u>415,296</u>                  | <u>(86,153)</u>                   |
| <b>6 Population and Public Health</b>                                 |                                  |                                          |                               |                               |                                 |                                   |
| 6.1 Program Support                                                   | 30,783                           | -                                        | -                             | 30,783                        | 25,661                          | (5,122)                           |
| 6.2 Immunization Support                                              | 4,121                            | -                                        | -                             | 4,121                         | 5,186                           | 1,065                             |
| 6.3 Community-Based Health Services                                   | 182,347                          | -                                        | -                             | 182,347                       | 148,650                         | (33,697)                          |
| 6.4 Children's Health Supports                                        | 15,000                           | -                                        | -                             | 15,000                        | -                               | (15,000)                          |
| 6.5 Health Innovation                                                 | 29,860                           | -                                        | -                             | 29,860                        | 18,135                          | (11,725)                          |
|                                                                       | <u>262,111</u>                   | <u>-</u>                                 | <u>-</u>                      | <u>262,111</u>                | <u>197,632</u>                  | <u>(64,479)</u>                   |
| <b>7 Continuing Care</b>                                              |                                  |                                          |                               |                               |                                 |                                   |
| 7.1 Program Support                                                   | 862                              | -                                        | -                             | 862                           | 609                             | (253)                             |
| 7.2 Continuing Care Programs                                          | -                                | -                                        | -                             | -                             | -                               | -                                 |
| 7.3 Accommodation Standards and Licensing                             | 10,167                           | -                                        | -                             | 10,167                        | 7,270                           | (2,897)                           |
| 7.4 Palliative Care                                                   | 5,000                            | -                                        | -                             | 5,000                         | 3,996                           | (1,004)                           |
|                                                                       | <u>16,029</u>                    | <u>-</u>                                 | <u>-</u>                      | <u>16,029</u>                 | <u>11,875</u>                   | <u>(4,154)</u>                    |

## Department of Health

**Lapse/Encumbrance (unaudited) (continued)**  
**Year ended March 31, 2025**  
*(in thousands)*

| Expense Vote by Program                                                         | Voted<br>Estimate <sup>(1)</sup> | Supplementary<br>Estimate <sup>(2)</sup> | Adjustments<br><sup>(3)</sup> | Adjusted<br>Voted<br>Estimate | Voted<br>Actuals <sup>(4)</sup> | Over<br>Expended/<br>(Unexpended) |
|---------------------------------------------------------------------------------|----------------------------------|------------------------------------------|-------------------------------|-------------------------------|---------------------------------|-----------------------------------|
| <b>Operating Expense</b>                                                        |                                  |                                          |                               |                               |                                 |                                   |
| <b>8 Other Health Services</b>                                                  |                                  |                                          |                               |                               |                                 |                                   |
| 8.1 Program Support                                                             | \$ 16,940                        | \$ -                                     | \$ -                          | \$ 16,940                     | \$ 8,630                        | \$ (8,310)                        |
| 8.2 Allied Health Services                                                      | 160,448                          | -                                        | -                             | 160,448                       | 181,426                         | 20,978                            |
| 8.3 Out-of-Province Health Care Services                                        | 138,702                          | -                                        | -                             | 138,702                       | 150,661                         | 11,959                            |
| 8.4 Cancer Research<br>and Prevention Investment                                | 25,852                           | -                                        | -                             | 25,852                        | 24,560                          | (1,292)                           |
| 8.5 Human Tissue and Blood Services                                             | 268,232                          | -                                        | -                             | 268,232                       | 289,114                         | 20,882                            |
|                                                                                 | 610,174                          | -                                        | -                             | 610,174                       | 654,391                         | 44,217                            |
| <b>9 Information Technology</b>                                                 |                                  |                                          |                               |                               |                                 |                                   |
| 9.1 Program Support                                                             | 25,008                           | -                                        | -                             | 25,008                        | 27,234                          | 2,226                             |
| 9.2 Development and Operations                                                  | 154,006                          | -                                        | -                             | 154,006                       | 118,786                         | (35,220)                          |
|                                                                                 | 179,014                          | -                                        | -                             | 179,014                       | 146,020                         | (32,994)                          |
| <b>Capital Payments to Related Parties</b>                                      |                                  |                                          |                               |                               |                                 |                                   |
| <b>2 Health Entities (previously Alberta Health Services)</b>                   |                                  |                                          |                               |                               |                                 |                                   |
| 2.4 Acute Care                                                                  | 1,225                            | -                                        | -                             | 1,225                         | -                               | (1,225)                           |
| <b>10 Infrastructure Support</b>                                                |                                  |                                          |                               |                               |                                 |                                   |
| 10.2 External Information<br>Systems Development                                | 5,748                            | -                                        | -                             | 5,748                         | 3,005                           | (2,743)                           |
| 10.3 Medical Equipment Replacement<br>and Upgrade Program                       | 30,000                           | -                                        | -                             | 30,000                        | 30,000                          | -                                 |
| 10.4 Alberta Surgical Initiative<br>Capital Program                             | 4,135                            | -                                        | -                             | 4,135                         | 16,225                          | 12,090                            |
| 10.5 Rural Alberta Health<br>Facilities Capital Program                         | 40,000                           | -                                        | -                             | 40,000                        | 40,000                          | -                                 |
| 10.6 National Association of Pharmacy<br>Regulatory Authorities Capital Program | 11,000                           | -                                        | -                             | 11,000                        | 11,000                          | -                                 |
| 10.7 Emergency Medical Services<br>Vehicles Capital Program                     | 15,000                           | -                                        | -                             | 15,000                        | 15,000                          | -                                 |
| 10.8 Beaverlodge Health Centre Replacement                                      | -                                | -                                        | -                             | -                             | 6,000                           | 6,000                             |
|                                                                                 | 105,883                          | -                                        | -                             | 105,883                       | 121,230                         | 15,347                            |
| <b>Total</b>                                                                    | \$ 25,250,163                    | \$ 26,200                                | \$ (137,502)                  | \$ 25,138,861                 | \$ 25,018,308                   | \$ (120,553)                      |
| <b>Credit or Recovery (Shortfall)</b>                                           | -                                | -                                        | (1,407)                       | (1,407)                       | -                               | 1,407                             |
|                                                                                 | \$ 25,250,163                    | \$ 26,200                                | \$ (138,909)                  | \$ 25,137,454                 | \$ 25,018,308                   | \$ (119,146)                      |
| <b>(Lapse)/Encumbrance</b>                                                      |                                  |                                          |                               |                               |                                 | \$ (119,146)                      |

## Department of Health

**Lapse/Encumbrance (unaudited) (continued)**  
**Year ended March 31, 2025**  
*(in thousands)*

| Expense Vote by Program                                       | Voted<br>Estimate <sup>(1)</sup> | Supplementary<br>Estimate <sup>(2)</sup> | Adjustments<br><sup>(3)</sup> | Adjusted<br>Voted<br>Estimate | Voted<br>Actuals <sup>(4)</sup> | Over<br>Expended/<br>(Unexpended) |
|---------------------------------------------------------------|----------------------------------|------------------------------------------|-------------------------------|-------------------------------|---------------------------------|-----------------------------------|
| <b>Capital Investment Vote by Program</b>                     |                                  |                                          |                               |                               |                                 |                                   |
| <b>Department Capital Acquisitions</b>                        |                                  |                                          |                               |                               |                                 |                                   |
| <b>1 Ministry Support Services</b>                            |                                  |                                          |                               |                               |                                 |                                   |
| 1.3 Strategic Corporate Support<br>and Policy Development     | \$ -                             | \$ -                                     | \$ -                          | \$ -                          | \$ 3,165                        | \$ 3,165                          |
| <b>6 Population and Public Health</b>                         |                                  |                                          |                               |                               |                                 |                                   |
| 6.2 Immunization Support                                      | -                                | -                                        | -                             | -                             | 32                              | 32                                |
| <b>7 Continuing Care</b>                                      |                                  |                                          |                               |                               |                                 |                                   |
| 7.1 Program Support                                           | 635                              | -                                        | -                             | 635                           | -                               | (635)                             |
| <b>9 Information Technology</b>                               |                                  |                                          |                               |                               |                                 |                                   |
| 9.2 Development and Operations                                | 22,480                           | -                                        | -                             | 22,480                        | 10,489                          | (11,991)                          |
| <b>10 Infrastructure Support</b>                              |                                  |                                          |                               |                               |                                 |                                   |
| 10.8 Beaverlodge Health Centre Replacement                    | 6,700                            | -                                        | -                             | 6,700                         | -                               | (6,700)                           |
| <b>Total</b>                                                  | <b>\$ 29,815</b>                 | <b>\$ -</b>                              | <b>\$ -</b>                   | <b>\$ 29,815</b>              | <b>\$ 13,686</b>                | <b>\$ (16,129)</b>                |
| <b>(Lapse)/Encumbrance</b>                                    |                                  |                                          |                               |                               |                                 | <b>\$ (16,129)</b>                |
| <b>Financial Transactions Vote by Program</b>                 |                                  |                                          |                               |                               |                                 |                                   |
| <b>Inventory Acquisition</b>                                  |                                  |                                          |                               |                               |                                 |                                   |
| <b>4 Drugs and Supplemental Health Benefits</b>               |                                  |                                          |                               |                               |                                 |                                   |
| 4.2 Outpatient Drugs                                          | \$ 9,000                         | \$ -                                     | \$ -                          | \$ 9,000                      | \$ 7,310                        | \$ (1,690)                        |
| <b>6 Population and Public Health</b>                         |                                  |                                          |                               |                               |                                 |                                   |
| 6.2 Immunization Support                                      | 150,196                          | -                                        | -                             | 150,196                       | 105,774                         | (44,422)                          |
| <b>Total</b>                                                  | <b>\$ 159,196</b>                | <b>\$ -</b>                              | <b>\$ -</b>                   | <b>\$ 159,196</b>             | <b>\$ 113,084</b>               | <b>\$ (46,112)</b>                |
| <b>(Lapse)/Encumbrance</b>                                    |                                  |                                          |                               |                               |                                 | <b>\$ (46,112)</b>                |
| <b>Contingency Vote by Program</b>                            |                                  |                                          |                               |                               |                                 |                                   |
| <b>2 Health Entities (previously Alberta Health Services)</b> |                                  |                                          |                               |                               |                                 |                                   |
| 2.4 Acute Care                                                | \$ -                             | \$ -                                     | \$ 522,058                    | \$ 522,058                    | \$ 522,058                      | \$ -                              |
| <b>3 Physician Compensation and Development</b>               |                                  |                                          |                               |                               |                                 |                                   |
| 3.2 Physician Services                                        | -                                | -                                        | 448,083                       | 448,083                       | 448,083                         | -                                 |
| <b>Total</b>                                                  | <b>\$ -</b>                      | <b>\$ -</b>                              | <b>\$ 970,141</b>             | <b>\$ 970,141</b>             | <b>\$ 970,141</b>               | <b>\$ -</b>                       |
| <b>(Lapse)/Encumbrance</b>                                    |                                  |                                          |                               |                               |                                 | <b>\$ -</b>                       |

Department of Health

Lapse/Encumbrance (unaudited) (continued)  
Year ended March 31, 2025

- (1) As per "Expense Vote by Program", "Capital Investment Vote by Program" and "Financial Transactions Vote by Program" pages 110 to 112 of 2024-25 Government Estimates. Effective December 12, 2024, the responsibility for the administration of portion of Strategic Corporate Support and Policy Development, Program Support and Continuing Care Programs, and Continuing Care Beds were transferred to the Minister of Seniors, Community and Social Services (Order in Council 384/2024). The voted operating expense has been restated by (\$422,392).
- (2) Per the Supplementary Supply Estimates approved on March 20, 2025.
- (3) Adjustments include encumbrances, capital carry over amounts, transfers between votes, credit or recovery increases approved by Treasury Board, and credit or recovery shortfalls. An encumbrance is incurred when, on a vote by vote basis, the total of actual disbursements in the prior year exceed the total adjusted estimate. All calculated encumbrances from the prior year are reflected as an adjustment to reduce the corresponding voted estimate in the current year.

Adjustments also include supply vote transfers of \$970,141 for "Contingency" as approved by the Lieutenant Governor in Council under the direction of the Minister of Finance (Order in Council 058/2025). The Contingency supply vote consists of a provisional funding authority transferable to any ministry. Upon approval by the Lieutenant Governor in Council, the President of Treasury Board and Minister of Finance may either spend or transfer all or a portion of this supply vote to another Minister for public emergencies, disasters or unanticipated costs.

- (4) Actuals exclude non-voted amounts such as amortization, inventory consumption, and valuation adjustments as no cash disbursement is required (non-cash amounts), or because the Legislative Assembly has already provided the funding authority pursuant to a statute other than an appropriation act.

## Department of Health

### Statement of Remissions, Compromises and Write-offs (unaudited)

|                                     | 2025            | 2024            |
|-------------------------------------|-----------------|-----------------|
| Compromises:                        |                 |                 |
| Fee for service recoveries          | \$ 286          | \$ -            |
| Write-Offs:                         |                 |                 |
| Medical Claim Recoveries            | 1,973           | 2,900           |
| Pharmaceuticals and Health Benefits | -               | 3,220           |
| Other Receivables                   | 445             | 498             |
| Total <sup>(1)</sup>                | <u>\$ 2,704</u> | <u>\$ 6,618</u> |

<sup>(1)</sup> There were no remissions during the year.

The above statement has been prepared pursuant to Section 23 of the *Financial Administration Act*. The statement includes all remissions, compromises and write-offs made or approved during the fiscal year.

### Payments Based on Agreements (unaudited)

The following has been prepared pursuant to Section 25(3) of the *Financial Administration Act*.

The Department entered into agreements, under the Alberta Health Care Insurance Plan, with other Provincial and Territorial Governments to provide health services on their behalf. The Department pays service providers for services rendered under the agreements and recovers the amount paid. Service providers include Alberta Health Services and physicians. The Department merely provides a service for another party and assumes no liability beyond that of completely discharging the role of being a conduit for funding. Costs based on these agreements are incurred by the Department under authority in Section 25 of the *Financial Administration Act*. Accounts receivable includes \$48,737 (2024 - \$56,750) relating to payments based on agreements.

Amounts paid based on agreements are as follows:

|                                              | 2025              | 2024              |
|----------------------------------------------|-------------------|-------------------|
| Other Provincial and Territorial Governments | <u>\$ 344,586</u> | <u>\$ 344,034</u> |

## Ministry of Health

### Trust Funds under Administration (unaudited) Year ended March 31, 2025 (in thousands)

Trust funds under administration are funds consisting of public money over which the Legislature has no power of appropriation. The Ministry has no equity in the funds and administers them for the purposes of various trusts and therefore they are not included in the consolidated financial statements.

As at March 31, 2025, the balance of funds held in trust for Health Benefit Trust of Alberta was \$365,295 (2024 - \$208,949) and for research and development, education, and other programs was \$3,124 (2024 - \$2,285).

The Ministry and a third-party trustee administer the Supplemental Executive Retirement Plan in accordance with a retirement compensation arrangement trust agreement. As at March 31, 2025, there are \$24,255 in plan assets (2024 - \$25,176).

## Annual Report Extracts and Other Statutory Reports

### ***Public Interest Disclosure (Whistleblower Protection) Act***

Section 32 of the *Public Interest Disclosure (Whistleblower Protection) Act* reads:

- 32(1) Every chief officer must prepare a report annually on all disclosures that have been made to the designated officer of the department, public entity or office of the Legislature for which the chief officer is responsible.
- (2) The report under subsection (1) must include the following information:
- (a) the number of disclosures received by the designated officer, the number of disclosures acted on and the number of disclosures not acted on by the designated officer;
  - (b) the number of investigations commenced by the designated officer as a result of disclosures;
  - (c) in the case of an investigation that results in a finding of wrongdoing, a description of the wrongdoing and any recommendations made or corrective measures taken in relation to the wrongdoing or the reasons why no corrective measure was taken.
- (3) The report under subsection (1) must be included in the annual report of the department, public entity or office of the Legislature if the annual report is made publicly available.

There were no disclosures of wrongdoing filed with my office, for your department, between April 1, 2024 and March 31, 2025